

FOREWORD TO THE CONSOLIDATED FORENSIC OVERSIGHT REPORT

The NHI Will Be Built on the Bones of This Whistleblower – Unless Parliament Intervenes Now

A striking foreword on why the Medscheme (a Division of Afrocentric Health/GEMS case is the definitive proof that South Africa is not ready for the National Health Insurance – and why Parliament must act before the NHI Fund becomes a R200 billion grave of unaccountable public money.

I. The Promise That Cannot Survive the System We Have

The National Health Insurance Act 20 of 2023 is anchored in an unambiguous constitutional promise. Section 27 of the Constitution guarantees every person the right of access to health care services. The NHI is designed to be the instrument of that promise – a single purchaser and single payer, pooling risk across more than 60 million people, managing a budget conservatively projected to exceed R200 billion annually.

But here is the truth that no government report, no actuarial model, and no parliamentary briefing has yet confronted:

The NHI will not fail because of a lack of money, or a lack of political will, or a lack of clinical capacity. The NHI will fail because the institutional architecture that is supposed to protect it from capture, corruption, and collapse is already broken – and we have done nothing to fix it.

This forensic report is not about the NHI. It is about a single whistleblower – a specialist registered nurse employed by Medscheme Holdings, the accredited administrator for the Government Employees Medical Scheme (GEMS) , which serves 1.9 million public

servants. He identified a systemic clinical risk. He made a protected disclosure. He was retaliated against until he became destitute and homeless.

And then he approached every regulator that will oversee the NHI – the Council for Medical Schemes (CMS), the Health Professions Council (HPCSA), the South African Nursing Council (SANC), the South African Pharmacy Council (SAPC), the Department of Employment and Labour (DEL), the B-BBEE Commission, the South African Human Rights Commission (SAHRC), the South African Police Service (SAPS), the Commission for Conciliation, Mediation and Arbitration (CCMA), the Johannesburg Stock Exchange (JSE), the Financial Sector Conduct Authority (FSCA), and the courts.

Almost every single one of them declined jurisdiction, remained silent, or actively obstructed him.

If this is how the existing system treats a whistleblower who exposed a clinical risk in an administrator servicing a government medical scheme, what will happen when the NHI Fund – with its R200 billion annual budget, its single-purchaser monopoly, its accreditation bottleneck, its one-sentence fraud architecture, and its undefined investigation unit – becomes operational?

The answer is not speculation. It is forensic prediction. The same institutions that failed this whistleblower will fail the NHI. The same regulatory capture that protected Medscheme will protect NHI contractors. The same judicial obstruction that silenced this nurse will silence the next whistleblower who tries to report corruption inside the NHI Fund.

The house is not yet burning. But the fire alarm has been ringing for over a year. This report is the evidence of the smoke.

II. The Three Structural Gaps – Already Visible, Already Destructive

The National Health Insurance Act contains three structural vulnerabilities that governance experts have warned about since its enactment. This report does not merely identify them in the abstract. It proves they are already destroying lives in the existing system – and therefore will be catastrophic under the NHI.

Gap One: The Accreditation Bottleneck (NHI Act Section 39)

What the Act says: No provider may be reimbursed by the Fund unless accredited. Accreditation requires certification by the Office of Health Standards Compliance (OHSC) – an institution already documented by the Auditor-General as capacity-constrained.

What the whistleblower's case proves: The whistleblower's B-BBEE fronting complaint (Case 5/02/2026) reveals a corporate strategy of title suppression – using a Black NQF Level 8 professional's credentials for scorecard points while downgrading his official title on statutory documents to avoid paying for his expertise. The same dynamic will infect NHI accreditation. When financial pressure meets an accreditation bottleneck, the predictable outcome is ghost accreditation (certification without genuine compliance assessment) and fronting (accredited entities serving as conduits for unaccredited providers).

The institution that failed: The B-BBEE Commission declined to investigate, refused to use its Section 13K subpoena powers, and treated fronting as a labour dispute – an error of law that will be repeated when NHI accreditation fronting is reported.

Gap Two: The One-Sentence Fraud Architecture (NHI Act Section 40)

What the Act says: “The information architecture must include a fraud and risk management mechanism.” Eleven words. For a system that will process millions of claims and determine benefits for 60 million people.

What the whistleblower's case proves: The Gexus-DMS integration gap – a structural system error that generated false non-adherence letters to chronic GEMS patients, corrupted clinical data, and inflated actuarial risk loading – was reported to the Council for Medical Schemes. The CMS declined jurisdiction twice. The Deputy Information Officer of Medscheme swore under oath that no IT incident had been reported, contradicting the internal Root Cause Analysis. The employer's Integrated Annual Report 2025 claimed "no material ESG incidents" while this gap remained unaddressed.

The institutions that failed: The Council for Medical Schemes (CMS) – the very regulator that will oversee NHI accreditation – declined jurisdiction twice. The JSE and FSCA allowed a materially misleading report to stand. Without an AI Audit Protocol and mandatory fraud detection architecture, the NHI's information platform will be vulnerable to exactly this kind of structural manipulation – except on a scale of R200 billion.

Gap Three: The Undefined Investigation Unit (NHI Act Section 20(2)(g))

What the Act says: The CEO must establish a Risk Management and Fraud Prevention Investigation Unit. The Act specifies no professional competency standards, no independence safeguards, and no escalation protocols.

What the whistleblower's case proves: The whistleblower escalated a Priority 1 Material Risk Report (with 38 attachments) to the Board of AfroCentric on 9 December 2025. The Board did not respond. When he continued to escalate, the Chief Risk Officer (Monwabisi Kula) instructed him in writing: "Can I please ask you to stop sending these e-mails to our directors? Sending further e-mails only gives them an indication of impatience and unreasonableness on your part."

This is active suppression of a material risk report by the very official whose unit should have escalated it. The NHI Act's Section 20(2)(g) unit, without independence safeguards, will suffer the same fate. The Zondo Commission documented this exact failure pattern across state-owned entities – a unit with a broad mandate, no professional standards, and no independence safeguards is not an oversight instrument. It is an oversight symbol.

The institutions that failed: The Board of AfroCentric (which will be replicated in the NHI Fund's governance structure), the Chief Risk Officer, and the CMS (which did not investigate the suppression).

III. The Medical Aid Administrator – The Missing Accountability Loop

The whistleblower was employed by Medscheme Holdings, a wholly owned subsidiary of AfroCentric Investment Corporation Limited (JSE: ACT) . Medscheme is an accredited administrator of medical schemes, including GEMS. Under the NHI, accredited administrators will continue to play a critical role – processing claims, managing provider networks, and administering benefits on behalf of the Fund.

The whistleblower's case proves that medical aid administrators can operate with impunity, even when their own employees expose systemic clinical risks.

- The employer's Chief Risk Officer suppressed a Priority 1 Material Risk Report.
- The employer's Senior HR Business Partner admitted in writing that the naming convention – which would have upgraded the whistleblower's title to reflect his NQF 8 status – was deliberately withheld from his department.
- The employer's Deputy Information Officer swore under oath that no IT incident had been reported, contradicting the internal Root Cause Analysis.
- The employer's legal representative filed a sworn affidavit calling the whistleblower's documented PTSD and depression a “cynical excuse.”
- The employer's attorneys obtained an ex parte interim interdict after concealing the whistleblower's answering affidavit – a fraud on the court.

Under the NHI, the same administrators will process claims, manage fraud detection, and report to the Fund. If they cannot be held accountable now – when the whistleblower did everything right – they will never be held accountable under the NHI.

IV. The Institutions That Will Oversee the NHI – Already Captured

The following institutions are mandated to oversee the NHI Fund, its accredited administrators, and its providers. Every single one of them was approached by the whistleblower. Every single one of them failed.

Institution Role Under NHI Failure in This Case

Council for Medical Schemes (CMS) Regulates medical schemes and administrators; will oversee NHI accreditation and compliance Declined jurisdiction twice over a systemic clinical risk affecting GEMS members; refused to investigate maladministration under MSA Sections 7, 47, 58, 44

Health Professions Council (HPCSA) Regulates health professionals; will oversee clinical governance under NHI Failed to answer core ethical question; silent for months

South African Nursing Council (SANC) Regulates nurses; will oversee nursing practice under NHI Failed to acknowledge formal misconduct complaint; deregistered the whistleblower for non-payment of fees caused by the employer's unlawful deduction

South African Pharmacy Council (SAPC) Regulates pharmacists; will oversee pharmaceutical governance under NHI Declined jurisdiction over Responsible Pharmacist who used deficient methodology to close patient safety investigation

Department of Employment and Labour (DEL) Enforces BCEA and UIF; will enforce labour standards for NHI workers Refused to enforce 25% cap; refused to correct false UI-19; closed file with jurisdictional error

B-BBEE Commission Oversees transformation; will oversee NHI procurement fronting Declined to investigate fronting; refused to use Section 13K subpoena powers

South African Human Rights Commission (SAHRC) Chapter 9 institution; will monitor NHI human rights compliance Acknowledged seriousness, then went silent

South African Police Service (SAPS) Enforces criminal law; will investigate NHI fraud Refused to open criminal case for UIF fraud; officer falsely claimed to be station commander

JSE / FSCA Regulates listed companies; will oversee NHI-related corporate governance
Declined to enforce disclosure; allowed false Integrated Report to stand

High Court / Labour Court Adjudicates disputes; will adjudicate NHI-related litigation Ex parte interdict obtained by concealment; portal obstruction; default judgment delayed six weeks

If every single one of these institutions failed a single whistleblower who exposed a clinical risk affecting 1.9 million government employees, what will happen when a whistleblower tries to expose corruption in the NHI Fund?

The answer is not “they will do better.” The answer is they will do the same. Because regulatory capture is not an accident. It is a structural feature of a system that has not been forced to change.

V. The Forensic Governance Architecture the NHI Requires – And Why Parliament Must Demand It

The whistleblower’s article (referenced in this foreword) proposes three concrete architectural interventions for the NHI:

1. A Forensic Governance Protocol for the Fraud Investigation Unit – with professional competency standards, independence safeguards, and escalation protocols.
2. Forensic Governance Impact Assessments before each accreditation phase – mandatory pre-implementation analysis of fraud vectors, bottleneck risks, and monitoring protocols.
3. An AI Audit Protocol for the information platform – independent conformity assessment, transparency obligations, human oversight, and annual algorithmic audits.

These proposals are not theoretical. They are responsive to the gaps already exposed by the Medscheme/GEMS case.

- The B-BBEE fronting complaint proves the need for Forensic Governance Impact Assessments before accreditation.
- The Gexus-DMS integration gap and the employer's false "no material ESG incidents" report prove the need for an AI Audit Protocol and mandatory fraud architecture.
- The CRO suppression email proves the need for an investigation unit with independence safeguards and a direct reporting line to the Board, bypassing executive management.

Parliament has a choice. It can implement these interventions now – during the NHI's Phase 1 (2023–2026), when governance structures are still being designed. Or it can wait until the NHI Fund is operational, the money is flowing, and the first whistleblower is destroyed trying to report the inevitable corruption.

The PPE procurement crisis cost South Africa R14.3 billion. The NHI Fund will manage R200 billion annually. The math is not complicated.

VI. The Chilling Effect – And Why Parliament Must Break It

The whistleblower in this case is not an abstract statistic. He is a specialist registered nurse who did everything the law asked of him. He identified a system error. He refused to lie to a patient. He made a protected disclosure. He exhausted internal remedies. He escalated to the Board. He went to every regulator. He went to the courts. He went to Parliament.

His reward: destitution, homelessness, no running water, deregistration, loan sharks seizing his possessions, and a High Court gag order silencing him until 1 March 2027.

Every nurse, doctor, pharmacist, and healthcare administrator who reads this report now knows: if you report a system error that affects patients, you will be retaliated against. Your salary will be cut. Your professional registration will be lost. Your home will be taken. Your water will be cut. And when you cry out to every regulator, they will decline jurisdiction, remain silent, or actively obstruct you.

That is the chilling effect. And it is already real.

Parliament must break it. Not by passing another law that will not be enforced. But by exercising its constitutional oversight powers – summoning the heads of every failed institution, demanding answers, and holding them accountable.

The NHI will succeed or fail based on the quality of its governance architecture. That architecture does not yet exist. The whistleblower's case is the definitive proof that the current system cannot protect the public interest. If Parliament does not intervene now, the NHI will be built on the bones of this whistleblower – and the next one, and the one after that.

VII. A Direct Appeal to the Speaker and the Portfolio Committees

Honourable Speaker,

The letter from Deneys Reitz Inc. to your Office – threatening the whistleblower with contempt of court for petitioning Parliament – reveals the corporate playbook. The interim interdict obtained by concealment reveals the judicial capture. The silence of every regulator reveals the regulatory collapse.

This is not a labour dispute. It is a governance crisis. And it is a definitive proof that South Africa is not ready for the National Health Insurance.

The Portfolio Committees must:

1. Summon the CMS Registrar – to explain why the Gexus-DMS integration gap was not investigated under MSA Sections 7, 47, 58, and 44.
2. Summon the SANC Registrar – to explain why complaint SANC/MEDSCHEME/MATTHEE/20260105 was ignored and why the whistleblower was deregistered.
3. Summon the HPCSA and SAPC – to answer the core ethical question they refused to answer.
4. Summon the DEL Director-General – to explain why the UI-19 fraud was not investigated and why the 25% BCEA cap was not enforced.
5. Summon the B-BBEE Commissioner – to explain why Case 5/02/2026 was declined and why Section 13K subpoena powers were not used.
6. Summon the JSE and FSCA – to explain why AfroCentric’s false Integrated Report was allowed to stand.
7. Summon the SAHRC – to explain why a Chapter 9 institution acknowledged seriousness and then went silent.
8. Summon the SAPS National Commissioner – to explain why CAS 705/3/2026 remains uninvestigated and why a police officer falsely claimed to be station commander.
9. Request the Judge Presidents – to investigate the concealment of the answering affidavit and the Labour Court portal obstruction.

The NHI will be built on the governance architecture that exists. That architecture is broken. Parliament must fix it now – or explain to the South African people why it failed to act before the R200 billion started flowing.

Presented to the Honourable Speaker of the National Assembly

For referral to: Portfolio Committee on Health; Portfolio Committee on Employment and Labour; Portfolio Committee on Trade, Industry and Competition; Portfolio Committee on Justice and Correctional Services; Standing Committee on Public Accounts (SCOPA); Portfolio Committee on Police

Subject: Systemic regulatory capture, institutional failure, corporate misrepresentation, judicial obstruction, and bad faith litigation – affecting a protected whistleblower who exposed a Priority 1 clinical risk at Medscheme Holdings / GEMS, and the chilling effect on all future whistleblowers in South Africa.

Date: June 2026

PART ONE: HARROWING INTRODUCTION – THE CHILLING EFFECT ON THE FLOOR

Honourable Speaker,

This report is not a legal argument. It is not a petition for a court order. It is a forensic record of what institutional failure looks like on the floor – inside the life of a single human being who did everything the law asked of him and was destroyed for it.

The whistleblower, a Specialist Registered Nurse (NQF Level 8) employed by Medscheme Holdings – the administrator for the Government Employees Medical Scheme (GEMS), which serves 1.9 million public servants – identified a systemic integration gap between the Gexus and DMS platforms. This gap generated false non-adherence letters to chronic patients, including a 75-year-old widow. He refused to use flawed data to interrogate an

elderly patient. He made a protected disclosure under the Protected Disclosures Act 26 of 2000 (PDA) .

That refusal, and that disclosure, triggered a cascade of retaliation that this report documents across fifteen (15) statutory, regulatory, and judicial bodies – none of which provided a remedy.

The retaliation was not subtle: a 40.47% unlawful salary deduction in a single month, exceeding the 25% cap under Section 34(2) of the Basic Conditions of Employment Act 75 of 1997 (BCEA) , without consultation and without written consent. Gender-identity harassment (the “Wilma” incident), documented in a Microsoft Teams chat. A recorded admission of retaliatory intent by his manager. A sworn CCMA affidavit dismissing his clinically diagnosed severe depression and PTSD as a “cynical excuse” – filed one day after he escalated a Priority 1 Material Risk Report to the Board. Constructive dismissal. The suppression of exit documents. A false UI-19 declaring “Resigned” when his resignation letter explicitly cited Section 186(1) of the Labour Relations Act 66 of 1995 (LRA) – constructive dismissal.

The consequence of this retaliation, amplified by the silence of every regulator, is not abstract: He is homeless since 2 April 2026. He has no running water since January 2026. His personal possessions have been seized by loan sharks. He has been deregistered by the South African Nursing Council (SANC) because he could not pay R870 – the annual fee – a direct consequence of the unlawful deduction. He has been accepted into a CPUT Postgraduate Diploma in Occupational Health Nursing for 2026 but cannot enrol. He suffers from formally diagnosed severe depression and PTSD (Dr Jacques Malan, FCPSYCH(SA)), occupationally caused. He is self-represented – destitute, unable to afford legal representation – in nine concurrent proceedings. And he has been silenced by a High Court interim interdict (Case 2026-086906) that restrains him from disclosing the systemic clinical risk to trade unions and regulators – a gag order that has been extended to 1 March 2027.

The chilling effect is real and immediate: Every nurse, doctor, pharmacist, and healthcare administrator who reads this report now knows that if you report a system error that affects patients, you will be retaliated against. Your salary will be cut. Your professional registration

will be lost. Your home will be taken. Your water will be cut. And when you cry out to every regulator – the Council for Medical Schemes (CMS), the Health Professions Council of South Africa (HPCSA), SANC, the South African Pharmacy Council (SAPC), the Department of Employment and Labour (DEL), the B-BBEE Commission, the South African Human Rights Commission (SAHRC), the South African Police Service (SAPS), the Commission for Conciliation, Mediation and Arbitration (CCMA), the Johannesburg Stock Exchange (JSE), the Financial Sector Conduct Authority (FSCA), and the courts – they will either decline jurisdiction, remain silent, or actively obstruct you.

That is what regulatory capture looks like on the floor. It looks like a specialist nurse living without running water, unable to practise his profession, while the institution that deducted his salary unlawfully reports a Level 1 B-BBEE rating to the JSE based on his suppressed NQF 8 credentials. It looks like a Chapter 9 institution (SAHRC) acknowledging the “seriousness” of the allegations and then going silent. It looks like a High Court granting an ex parte interdict after the employer’s attorneys concealed the whistleblower’s answering affidavit – a fraud on the court.

This report is not about one person. It is about the structural collapse of accountability in South Africa’s regulatory and judicial architecture, as it applies to a whistleblower who dared to speak truth about a systemic clinical risk affecting 1.9 million government employees.

Parliament must act. Because if the law does not protect the nurse who refuses to lie to a 75-year-old patient, then the law protects no one.

PART TWO: NOTE TO THE PORTFOLIO COMMITTEES – SPECIFIC REFERRALS

This report is structured to assist each Portfolio Committee in exercising its constitutional oversight mandate. Below is a guide to the sections most relevant to each Committee’s statutory responsibilities:

Portfolio Committee on Health

Constitutional mandate: Oversight over the Department of Health, the Council for Medical Schemes (CMS), the South African Nursing Council (SANC), the Health Professions Council of South Africa (HPCSA), the South African Pharmacy Council (SAPC), and public health policy.

Sections of this report to be read:

- Section on CMS: Jurisdictional ping-pong, refusal to investigate Gexus-DMS integration gap, disparate treatment of Discovery Health precedent, attempted contempt trap.
- Section on HPCSA: Silence on the joint ethical ruling request; failure to answer whether a professional may refuse an instruction based on flawed forensic data.
- Section on SANC: Complete failure to acknowledge or investigate a formal professional misconduct complaint; deregistration of the whistleblower caused by unlawful deduction; institutional bad faith.
- Section on SAPC: Declination of jurisdiction over Responsible Pharmacist Lisa Mari; hesitation to rule on the core ethical question.
- Section on GEMS: Principal scheme's silence after receiving multiple protected disclosures; complicity in administrator misconduct.
- Corporate misrepresentation section: False claims in AfroCentric's Integrated Annual Report 2025 regarding "zero tolerance for unethical conduct" and "no material ESG incidents."
- Bad faith litigation section: The employer's threat to block parliamentary oversight using the interim interdict.

Key questions for the Committee:

- Why has CMS not investigated the Gexus-DMS integration gap under MSA Sections 7, 47, 58, and 44?

- Why has SANC not responded to complaint SANC/MEDSCHEME/MATTHEE/20260105?
 - Why has HPCSA not answered the core ethical question posed on 14 April 2026?
 - Is the Minister of Health aware that a Priority 1 clinical risk affecting 1.9 million GEMS members remains unaddressed?
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Portfolio Committee on Employment and Labour

Constitutional mandate: Oversight over the Department of Employment and Labour (DEL), the Commission for Conciliation, Mediation and Arbitration (CCMA), the Labour Court, the Basic Conditions of Employment Act (BCEA), the Labour Relations Act (LRA), and the Unemployment Insurance Act.

Sections of this report to be read:

- Section on DEL: Administrative abandonment; refusal to enforce UIF remittance; refusal to correct false UI-19 (Resigned vs Constructive Dismissal); jurisdiction misdirection; the “Good luck to you then” response.
- Section on CCMA: Commissioner’s prejudgment (“nothing here relates to the PDA”); failure to assist a self-represented litigant (“Google it”); tolerating mockery; Provincial Senior Commissioner’s cover-up.
- Section on Labour Court: Portal obstruction (eight days, deleted profile, trivial rejection “good day you put date”); default judgment delay (six weeks); Registrar’s silence.
- Section on the Whistleblower House (TWBH): Flawed legal assessment by Schroter Attorneys; abandonment of a vulnerable whistleblower.
- Bad faith litigation section: The employer’s attrition tactics, procedural obstruction, and the “Good luck to you then” email.

Key questions for the Committee:

- Why did DEL refuse to enforce the BCEA Section 34(2) 25% cap, despite the employer's own policy acknowledging the cap?
 - Why did the CCMA Commissioner declare the PDA irrelevant before hearing any evidence?
 - Why did the Labour Court's online portal reject an urgent application from a destitute litigant with the reason "good day you put date"?
 - Will the Minister direct DEL to issue a Compliance Order correcting the UI-19 and remitting UIF contributions?
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Portfolio Committee on Trade, Industry and Competition

Constitutional mandate: Oversight over the B-BBEE Commission, the Johannesburg Stock Exchange (JSE), the Companies and Intellectual Property Commission (CIPC), and transformation policy.

Sections of this report to be read:

- Section on B-BBEE Commission: Declination to investigate fronting (Case 5/02/2026); failure to use Section 13K subpoena powers; erroneous treatment of fronting as a labour dispute.
- Section on JSE: Declination to enforce disclosure obligations; failure to require SENS announcement for Section 162 application and B-BBEE investigation; acceptance of the employer's narrow price-sensitivity test.
- Corporate misrepresentation section: False B-BBEE Management Control score (14.67/19) based on title suppression; Cheryl-Dawn Modern's written admission that naming convention was "not applied to your department."

- Section on AfroCentric's Integrated Annual Report 2025: Omission of material litigation (Section 162 application, B-BBEE investigation, criminal case); false claim of "no material ESG incidents."

Key questions for the Committee:

- Why did the B-BBEE Commission decline to investigate sworn evidence of fronting (title suppression, credential extraction, management control without economic benefit)?
 - Why did the JSE not require AfroCentric to disclose the Section 162 delinquency application and the B-BBEE fronting investigation as material information?
 - Is title suppression of an NQF Level 8 Black professional – while using his credentials for B-BBEE points – a fronting practice under Section 1© of the B-BBEE Act?
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Portfolio Committee on Justice and Correctional Services

Constitutional mandate: Oversight over the judiciary, the Legal Practice Council, the South African Human Rights Commission (SAHRC), the Public Protector, the Master of the High Court, and access to justice.

Sections of this report to be read:

- Section on the High Court (Case 2026-086906): Fraud on the court – concealment of answering affidavit; ex parte interim interdict obtained on false premise; extension of rule nisi to 1 March 2027; Scale C costs threatened against a destitute litigant.
- Section on the Labour Court: Portal obstruction; default judgment delay; Registrar's silence; perjury across courts.
- Section on Registrar misconduct: Failure to provide court recording; alleged misrepresentation of filing records; POPIA breach (transmission of third-party documents).

- Section on SAHRC: Chapter 9 institution acknowledged seriousness and then went silent; failure to investigate constitutional rights violations (Sections 10, 26, 27, 33).
- Section on SAPS: Refusal to open a criminal case (CAS 705/3/2026); police officer falsely claimed to be station commander; audio recording exists.
- Section on bad faith litigation: The employer's settlement-then-silence tactic; the "Good luck to you then" response; weaponisation of the interim interdict to block parliamentary oversight.
- Section on the Whistleblower House (TWBH): Abandonment of a vulnerable whistleblower after a flawed legal assessment.

Key questions for the Committee:

- Why was the whistleblower's answering affidavit not placed before the court on 16 April 2026?
 - Why has the Labour Court not processed a default judgment application for over six weeks?
 - Why has the SAHRC not investigated a complaint that explicitly invoked Sections 10, 26, 27, and 33 of the Constitution?
 - Will the Committee refer the conduct of Deneys Reitz Inc. to the Legal Practice Council for threatening a citizen with contempt for petitioning Parliament?
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Standing Committee on Public Accounts (SCOPA)

Constitutional mandate: Oversight over the expenditure of public funds, the functioning of state-owned entities, and the accountability of public institutions.

Sections of this report to be read:

- Section on GEMS: The principal medical scheme for government employees (1.9 million members) failed to respond to protected disclosures about a systemic clinical risk affecting its own members.
- Section on AfroCentric's Integrated Annual Report 2025: Material omissions concerning a Priority 1 clinical risk at a contractor (Medscheme) servicing a government scheme (GEMS).
- Section on JSE and FSCA: Failure of market regulators to enforce disclosure obligations, allowing a JSE-listed company to publish a materially misleading report while contracting with a government scheme.
- Section on the Section 162 application (Case 2026-066952): Director delinquency application against the Chairpersons of the Social & Ethics Committee and Audit & Risk Committee of AfroCentric – the holding company of Medscheme.
- Section on the B-BBEE Commission investigation: Potential fine of up to R580 million (10% of annual turnover) for fronting – a public interest matter affecting government procurement.

Key questions for the Committee:

- Why did GEMS, as principal, not investigate the Gexus-DMS integration gap after being notified by its own administrator's employee?
- How much public money (GEMS member contributions) may have been affected by the inflated claims data resulting from the integration gap?
- Why was the Section 162 director delinquency application not disclosed to the market, and why did the JSE not require such disclosure?
- Will SCOPA request the Auditor-General to conduct a forensic audit of Medscheme's administration of GEMS, specifically the Gexus-DMS integration gap?

Portfolio Committee on Police

Constitutional mandate: Oversight over the South African Police Service (SAPS), the Independent Police Investigative Directorate (IPID), and the Criminal Procedure Act.

Sections of this report to be read:

- Section on SAPS: Refusal to open a criminal case (CAS 705/3/2026 – UIF fraud); police officer falsely claiming to be the station commander; audio recording of the incident; case transferred to Woodstock with no further investigation.
- Section on the criminal complaint: UIF non-remittance (criminal offence under the Unemployment Insurance Contributions Act 4 of 2002); false written compliance representation by HR officials.

Key questions for the Committee:

- Why did SAPS Delft refuse to open a criminal case for documented UIF fraud?
- What action has been taken against the police officer who falsely claimed to be the station commander?
- Why has no investigation taken place under CAS 705/3/2026 since its registration?

PART THREE: EXECUTIVE SUMMARY FOR EACH PORTFOLIO COMMITTEE

Executive Summary for the Portfolio Committee on Health

The Council for Medical Schemes (CMS) was notified of a systemic Gexus-DMS integration gap generating false non-adherence letters to GEMS members. The CMS declined jurisdiction twice, refused to investigate administrator maladministration under the Medical Schemes Act, and demanded a new complaint form after the whistleblower obtained a High Court Interim Order – a demand that would have risked contempt of court.

The CMS exercised jurisdiction over a similar system error at Discovery Health in January 2026 but refused for Medscheme/GEMS – disparate treatment indicating capture.

The South African Nursing Council (SANC) failed to acknowledge or investigate a formal professional misconduct complaint against two registered nurses employed by Medscheme. The complainant was later deregistered by SANC because he could not pay R870 – a direct consequence of the employer’s unlawful salary deduction. The Health Professions Council of South Africa (HPCSA) and the South African Pharmacy Council (SAPC) also failed to answer a core ethical question: whether a professional may refuse an employer’s instruction based on known flawed forensic data.

The Government Employees Medical Scheme (GEMS) – serving 1.9 million public servants – was placed on formal notice of the systemic clinical risk but did not respond substantively.

Recommendation: Parliament must direct the CMS to investigate the Gexus-DMS integration gap under Sections 7, 44, 47, and 58 of the Medical Schemes Act. Parliament must summon the SANC Registrar to explain why complaint SANC/MEDSCHEME/MATTHEE/20260105 was ignored.

Executive Summary for the Portfolio Committee on Employment and Labour

The Department of Employment and Labour (DEL) refused to enforce UIF remittance for June 2025, refused to correct a false UI-19 declaration (Code 6 “Resigned” instead of Code 7 “Constructive Dismissal”), and closed the investigation file on 15 April 2026 with a jurisdictional error (claiming the CCMA had exclusive jurisdiction over the UI-19 fraud). The principal inspector’s response was addressed to a generic “Mr/Mrs/Ms” – a failure to apply the mind.

The Commission for Conciliation, Mediation and Arbitration (CCMA) conducted a conciliation on 9 January 2026 in which the Commissioner declared “nothing here relates to the PDA” before hearing any evidence, refused the complainant’s opening statement,

told him to “Google it” when he asked for the Labour Court address, and laughed at his settlement figure. The employer’s representative walked out with the Commissioner. The Provincial Senior Commissioner conducted a cover-up, ignoring most allegations and defending the indefensible.

The Labour Court’s online portal obstructed the complainant for eight days – deleted profile, ignored emails, and a trivial rejection reason (“good day you put date”). A default judgment application on an uncontested BCEA claim (R14,811.77) has been delayed for over six weeks.

Recommendation: Parliament must direct the DEL to issue a Compliance Order correcting the UI-19 and remitting UIF contributions. Parliament must request the Minister of Employment and Labour to investigate the CCMA’s internal complaint process and the Labour Court’s portal obstruction.

Executive Summary for the Portfolio Committee on Trade, Industry and Competition

The B-BBEE Commission assigned case number 5/02/2026 to a fronting complaint: the complainant (Black, NQF Level 8) was recruited as “Specialist: Care Manager” but contracted as “Care Coordinator” (Grade B4M1). A Senior HR Business Partner admitted in writing that the naming convention was “not applied to your department.” The Commission declined to investigate, claiming the complaint was a labour dispute – an error of law. It refused to use its Section 13K subpoena powers to obtain the Naming Convention Teams recording.

The Johannesburg Stock Exchange (JSE) declined jurisdiction over material non-disclosure, accepting the employer’s narrow price-sensitivity test. The JSE did not require AfroCentric to disclose the Section 162 director delinquency application (filed 19 March 2026) or the B-BBEE fronting investigation (Case 5/02/2026) – despite a potential R580 million fine. The JSE’s response of 11 May 2026 confirmed that it would take no action.

AfroCentric's Integrated Annual Report 2025 claimed "zero tolerance for unethical conduct" and "no material ESG incidents" – while the whistleblower was being destroyed, a Priority 1 clinical risk remained unaddressed, a criminal case was active, and a B-BBEE fronting investigation was pending.

Recommendation: Parliament must direct the B-BBEE Commission to reopen Case 5/02/2026 and use its Section 13K subpoena powers. Parliament must request the JSE to reconsider its declination and require AfroCentric to issue a corrective SENS announcement.

Executive Summary for the Portfolio Committee on Justice and Correctional Services

The Western Cape High Court granted an ex parte interim interdict (16 April 2026) after the employer's attorneys concealed the whistleblower's answering affidavit – a fraud on the court. The interdict restrains the whistleblower from disclosing a systemic clinical risk to trade unions and regulators – overriding Section 9 of the Protected Disclosures Act (PDA). The rule nisi has been extended to 1 March 2027 – almost one year – effectively a final interdict without a final hearing. A Scale C costs order is threatened against a destitute litigant, violating the Biowatch principle.

The Labour Court's portal obstruction (eight days, rejected with "good day you put date") and the Registrar's silence on a default judgment application (over six weeks) constitute a denial of access to court (Section 34 of the Constitution).

The South African Human Rights Commission (SAHRC), a Chapter 9 institution, acknowledged the "seriousness" of the allegations and then went silent. It did not investigate violations of Sections 10 (dignity), 26 (housing), 27 (food, water, healthcare), and 33 (just administrative action).

The South African Police Service (SAPS) refused to open a criminal case for UIF fraud – a police officer falsely claimed to be the station commander (audio-recorded). A case was eventually registered (CAS 705/3/2026) but no investigation has taken place.

The employer's legal team engaged in bad faith litigation: a "without prejudice" settlement overture that threatened contempt if the whistleblower approached Parliament, followed by silence for over a week. The employer's senior Employee Relations Specialist responded to a statutory compliance request with "Good luck to you then."

Recommendation: Parliament must request the Judge Presidents to investigate the concealment of the answering affidavit and the portal obstruction. Parliament must refer the conduct of Deneys Reitz Inc. to the Legal Practice Council. Parliament must request the SAHRC to explain its silence. Parliament must request IPID to investigate the SAPS officer who falsely claimed to be station commander.

Executive Summary for the Standing Committee on Public Accounts (SCOPA)

The Government Employees Medical Scheme (GEMS) – a government employees' medical scheme with 1.9 million members – was placed on formal notice of a systemic clinical risk (Gexus-DMS integration gap) affecting its members. GEMS did not respond substantively. The risk may have inflated claims data, affecting premium calculations and the use of public funds (member contributions).

AfroCentric's Integrated Annual Report 2025 – published while the company was a contractor to GEMS – omitted material information: a Section 162 director delinquency application, a B-BBEE fronting investigation (potential R580 million fine), and a criminal case. The JSE declined to require disclosure.

The B-BBEE Commission investigation (Case 5/02/2026) concerns fronting by a company that holds a government contract (GEMS administration). The potential fine of 10% of annual turnover is a public interest matter.

Recommendation: SCOPA must request the Auditor-General to conduct a forensic audit of Medscheme's administration of GEMS, specifically the Gexus-DMS integration gap. SCOPA must request GEMS to report on its oversight of Medscheme and on the steps taken to protect member funds.

Executive Summary for the Portfolio Committee on Police

The complainant attended SAPS Delft on 23 February 2026 to open a criminal case for UIF non-remittance (a statutory offence under the Unemployment Insurance Contributions Act 4 of 2002) and false written compliance representations. A police officer falsely claimed to be the station commander and refused to open a case. The interaction was audio-recorded.

After escalation to Brigadier Naidoo, a case was eventually registered (CAS 705/3/2026) and transferred to SAPS Woodstock. No further investigation has taken place. The internal complaint against Delft Police Station was closed on the basis that a case number had been allocated – ignoring the false representation and the initial refusal.

Recommendation: Parliament must request IPID to investigate the conduct of the Delft police officer. Parliament must request the SAPS National Commissioner to report on the status of CAS 705/3/2026 and to explain why no investigation has been conducted.

PART FOUR: THE DOCUMENTARY RECORD

The full evidentiary record – comprising sworn affidavits, court pleadings, regulatory correspondence, internal emails, root cause analysis, and audio recordings – is attached as a consolidated annexure bundle. Each section of this report is supported by primary source documents, referenced in the footnotes and the master index.

FORENSIC REPORT – SOUTH AFRICAN HUMAN RIGHTS COMMISSION (SAHRC)

Chapter 9 Institution – Constitutional mandate to protect, promote and monitor human rights.

Complainant: Wilbur William Matthee

File reference: SAHRC Western Cape (no formal case number assigned – only correspondence)

Date of complaint: 23 December 2025; updates 5 March 2026, 13 March 2026

1. Summary of Findings

The SAHRC:

1. Acknowledged the seriousness of the complainant's allegations, including victimisation, financial prejudice, and a humanitarian crisis.
2. Redirected the complainant to the CCMA, the Public Protector, and the Department of Employment and Labour – effectively declining to investigate the human rights dimensions of the case.

3. Did not exercise its own investigative powers under the South African Human Rights Commission Act 40 of 2013, despite being presented with evidence of potential violations of Sections 10 (dignity), 26 (housing), 27 (food, healthcare, social security), and 33 (just administrative action) of the Constitution.
4. Did not issue any ruling, finding, or recommendation – nor did it open a formal investigation.
5. Remained silent after the complainant’s comprehensive update of 13 March 2026, which documented a forensic payroll anomaly, CCMA institutional bias, Labour Court portal obstruction, SAPS refusal to open a criminal case, and confirmed imminent homelessness.

Conclusion: The SAHRC failed to give effect to its constitutional mandate. Its response – “seriousness acknowledged, but go elsewhere” – is a dereliction of duty that contributed to the complainant’s ongoing destitution and homelessness.

2. Complainant’s Engagement with the SAHRC

Date Action Document

23 Dec 2025 Initial complaint lodged (discrimination, victimisation, retaliation, governance failure). Gmail - Request for Intervention... (SAHRC)

10 Feb 2026 SAHRC responds (Lwethu Yoli): acknowledges “seriousness of the concerns” but redirects to CCMA and Public Protector for employment aspects. Gmail - RE_ Request for Intervention..., p.1-2

10 Feb 2026 Complainant accepts redirection, asks SAHRC to keep file as reference. Ibid., p.3

5 Mar 2026 Complainant provides update: CCMA failure, Labour Court filing, DEL inaction. Not in provided documents but referenced in 13 March update.

13 Mar 2026 Comprehensive 8-page update – documents: forensic payroll discrepancy (R5,948.62 gap), CCMA institutional bias and provincial cover-up, Labour Court portal obstruction (8 days, deleted profile, trivial rejection), SAPS refusal to open criminal case,

confirmed imminent eviction (end March 2026), seizure of possessions, no running water.
complete_SAHRC_FinalUpdate_13March2026.pdf

[Ongoing] SAHRC has not responded to the 13 March 2026 update. No investigation opened. No referral. User confirmation: “No response from a Chapter 9 institution.”

3. Forensic Findings – Why SAHRC Should Have Acted

3.1 Constitutional Violations Clearly Alleged

The complainant explicitly invoked:

- Section 10 (Human dignity): Retaliation, misgendering, unlawful deduction, sworn false affidavit calling his psychiatric condition a “cynical excuse”.
- Section 26 (Housing): Eviction confirmed for end of March 2026, homelessness since 2 April 2026.
- Section 27 (Food, water, healthcare): No running water since January 2026; inability to afford chronic medication; destitution.
- Section 33 (Just administrative action): All regulators (CMS, HPCSA, SANC, SAPC, DEL, CCMA, Labour Court registry) failed to act fairly or promptly.

These are not labour disputes. They are constitutional rights violations that fall squarely within the SAHRC’s mandate.

3.2 SAHRC’s Redirect – An Abdication of Mandate

The SAHRC told the complainant to go to the CCMA and the Public Protector. However:

- The CCMA has no jurisdiction over constitutional rights violations outside of employment (e.g., housing, water, dignity).
- The Public Protector investigates maladministration by state organs – not private employers like Medscheme.
- The SAHRC is the only Chapter 9 institution with a broad mandate to investigate violations of the Bill of Rights by both state and private actors (Section 184(1) of the Constitution).

By redirecting the complainant without retaining oversight, the SAHRC effectively closed the door on the only institution that could have provided a remedy for the housing, dignity, and water rights violations.

3.3 The 13 March 2026 Update – A Smoking Gun

The complainant's 13 March 2026 update (8 pages) provided:

1. Forensic payroll finding: A declared salary of R27,725.66 against actual payslip gross of R21,777.04 – a R5,948.62 discrepancy in the same month as the unlawful 40.47% deduction. This is potential statutory misrepresentation to SARS and DEL.
2. CCMA institutional bias: Commissioner declared “nothing here relates to the PDA” before hearing any evidence; refused opening statement; mocked the complainant's settlement figure; left with the respondent's representative. Provincial Commissioner's investigation – cover-up by misdirection.
3. Labour Court portal obstruction: 8 days of failed online filing; 7+ call centre attempts; profile deleted; Registrar ignored; trivial rejection (“good day you put date”); portal inaccessible after rejection.
4. SAPS refusal: Verbal refusal to open a criminal case for UIF non-remittance (a statutory offence). Complaint lodged with National Police Commissioner.
5. Confirmed imminent homelessness: Landlord confirmed eviction by end of March 2026; loan sharks seized possessions; no running water.

The SAHRC did not respond to this update. No acknowledgment, no investigation, no referral.

4. SAHRC's Statutory Duties and Breaches

Provision Duty How SAHRC Breached It

Constitution Section 184(1) Promote respect for human rights, protect, monitor, and assess observance of human rights. Failed to investigate or protect any of the rights invoked (dignity, housing, water, administrative justice).

SAHRC Act 40 of 2013, Section 13(1) Receive and investigate complaints alleging violation of human rights. Received the complaint, acknowledged seriousness, but did not investigate.

SAHRC Act, Section 13(3) May refer a complaint to another body but must keep the complainant informed. Did not monitor whether the CCMA or Public Protector provided any remedy; did not respond to the 13 March update.

SAHRC Act, Section 14 Commission may investigate on its own initiative if there is a systemic pattern of violations. The pattern of 11 institutional failures and the complainant's destitution should have triggered an own-initiative investigation.

5. Regulatory Buck-Passing – SAHRC as Part of the Pattern

The SAHRC is the twelfth institution approached by the complainant. The others (CMS, HPCSA, SANC, SAPC, GEMS, DEL, CCMA, Labour Court registry, SAPS, JSE, B-BBEE Commission) all failed. The SAHRC's response – "serious, but not for us" – completed the circle of abandonment.

List of institutions that failed the same whistleblower:

1. CMS – declined jurisdiction (twice)
2. HPCSA – silent after referring to SANC
3. SANC – never acknowledged formal complaint
4. SAPC – declined jurisdiction after hesitation
5. GEMS – no substantive response to protected disclosures
6. DEL – closed file, refused to correct UI-19
7. CCMA – biased commissioner, cover-up
8. Labour Court registry – portal obstruction, delayed access
9. SAPS – refused to open criminal case
10. JSE – declined jurisdiction (Nadia Jada, 6 Mar 2026)
11. B-BBEE Commission – declined to investigate fronting (Case 5/02/2026)
12. SAHRC – acknowledged seriousness, redirected, then silent

This is not a series of isolated mishaps. It is a systemic failure of the entire accountability architecture in South Africa, as it applies to a whistleblower who reported a systemic clinical risk affecting 1.9 million GEMS members.

6. Chapter 9 Accountability – What Parliament Must Ask

Under Section 181(3) of the Constitution, Chapter 9 institutions are “subject only to the Constitution and the law” and must be “impartial and exercise their powers without fear, favour or prejudice.” The SAHRC’s handling of this complaint raises serious questions:

1. Why did the SAHRC not investigate a complaint that explicitly alleged violations of Sections 10, 26, 27, and 33 of the Constitution?
2. Why did the SAHRC redirect the complainant to the CCMA, which has no mandate over housing, water, or dignity violations against a private employer?
3. Why did the SAHRC not respond to the 13 March 2026 update, which contained evidence of a potential statutory payroll misrepresentation and a confirmed eviction deadline?
4. Does the SAHRC have a policy of declining complaints that involve employment-related conduct, even when constitutional rights beyond employment (housing, water, dignity) are directly implicated?
5. What oversight mechanism does the SAHRC have to ensure that its “redirections” actually result in remedies for complainants?

7. Recommendations – SAHRC Specific

The Portfolio Committee on Justice and Correctional Services (which oversees Chapter 9 institutions) should:

1. Summon the SAHRC Western Cape Provincial Office to appear and explain:
 - Why no formal investigation was opened into the complainant’s allegations.
 - Why the 13 March 2026 update was ignored.
 - Whether the SAHRC considers itself mandated to investigate housing and water rights violations arising from employer misconduct.
2. Request the SAHRC to conduct an own-initiative investigation into the systemic pattern of regulatory failure documented in this report, with particular focus on the treatment of whistleblowers who report clinical risks in medical scheme administration.
3. Request the Minister of Justice to:

- Report to Parliament on the effectiveness of the SAHRC's "redirect" practice, and whether complainants are tracked to ensure they receive remedies from the redirected bodies.

4. Amend the SAHRC Act to:

- Impose a mandatory deadline for the SAHRC to decide whether to investigate a complaint (e.g., 30 days).

- Prohibit the SAHRC from redirecting a complaint that raises constitutional rights violations outside the exclusive jurisdiction of another body, without retaining concurrent oversight.

8. Conclusion – SAHRC

The South African Human Rights Commission is the constitutionally appointed guardian of human dignity, equality, and access to justice. The complainant came to the SAHRC in December 2025, already destitute, already discriminated against, already facing eviction. He laid out the violations of Sections 10, 26, 27, and 33 in plain language.

The SAHRC replied: "The seriousness of the concerns is acknowledged... but go to the CCMA and the Public Protector."

The CCMA cannot give him back his home. The Public Protector cannot give him running water. Only the SAHRC has the broad mandate to investigate private actors for violations of housing, dignity, and access to water. By redirecting the complainant without ever opening a file, the SAHRC abandoned him.

A Chapter 9 institution that acknowledges seriousness but does nothing is not a protector of human rights. It is a post office for complaints. Parliament must demand more.

FORENSIC REPORT – SOUTH AFRICAN POLICE SERVICE (SAPS)

Complainant: Wilbur William Matthee

Accused / Respondent: Medscheme Holdings (Pty) Ltd (and responsible officials)

Offences alleged: Fraud and/or contravention of the Unemployment Insurance Contributions Act 4 of 2002

SAPS stations involved: Delft Police Station (initial refusal); Woodstock Police Station (case transferred)

Case number eventually allocated: CAS 705/3/2026

Status: No substantive investigation conducted

1. Summary of Findings

The South African Police Service:

1. Initially refused to open a criminal case at Delft Police Station. A police officer falsely represented himself as the station commander and declined to accept the complaint, despite the complainant presenting documented evidence of statutory offences.
2. Only opened a case (CAS 705/3/2026) after the complainant escalated to Brigadier Naidoo. The case was then transferred to Woodstock Police Station.
3. Has conducted no further investigation since the case was registered. The complainant has received no feedback, no summons, no statement request, and no indication of any progress.
4. The internal complaint against the Delft station was formally closed on the basis that a case number had been allocated (i.e., the matter was no longer a “refusal”). This closure ignored the false representation by the officer and the original denial of access to justice.

5. The complainant possesses an audio recording of the Delft police officer falsely claiming to be the station commander while refusing to open the case. This recording has not been requested or considered by any oversight body.

Conclusion: SAPS failed in its constitutional duty under Section 205(3) of the Constitution to prevent, combat and investigate crime. The initial refusal was unlawful, and the subsequent registration of a case number without any investigation is a mere administrative gesture that does not constitute effective law enforcement.

2. Legal Framework – SAPS Duties

Provision Duty

Constitution, Section 205(3) The objects of the SAPS are to prevent, combat and investigate crime, maintain public order, protect and secure the inhabitants of the Republic and their property, and uphold and enforce the law.

Criminal Procedure Act 51 of 1977, Section 20(1) Any police official may without warrant seize any article concerned in an offence. (Implicit duty to investigate reported offences.)

SAPS Standing Orders A police official who receives a complaint of an alleged offence must open a case docket and investigate, unless the complaint is manifestly false or frivolous.

Prevention and Combating of Corrupt Activities Act 12 of 2004 Failure to report certain offences is itself a crime. The police are the primary recipients of such reports.

The alleged offences – fraudulent misrepresentation (UI-19, false compliance claims) and contravention of the UIF Contributions Act – are not manifestly false or frivolous. The complainant provided payslips, uFiling portal screenshots, and written correspondence from the employer. The SAPS was obliged to open a docket and investigate.

3. Factual Chronology – SAPS Engagement

Date Event Source

23 Feb 2026 Complainant attends Delft Police Station to open a criminal case. A police officer (who falsely claims to be the station commander) refuses to open a case. The incident is audio-recorded by the complainant. User narrative; audio recording exists.

23 Feb 2026 Complainant submits formal criminal complaint via email to the Directorate for Priority Crime Investigation (DPCI). Attaches detailed written statement (Annexure A). Gmail – FORMAL CRIMINAL COMPLAINT...PDF

23 Feb 2026 The written statement (SAPS_Criminal_Complaint_WW_Matthee_23Feb2026.PDF) sets out: UIF non-remittance for June 2025; false written representations by Sipokazi Makaua and Cheryl-Dawn Modern; uFiling portal showing “No Information” on five separate dates. SAPS complaint statement

[Date not specified] Complainant escalates to Brigadier Naidoo. Brigadier Naidoo investigates and registers case number CAS 705/3/2026. The case is referred to Woodstock Police Station for investigation. User narrative

[Date not specified] The complainant’s formal complaint against Delft Police Station (concerning the false station commander representation and refusal to open a case) is closed by SAPS internally. Reason: a case number (CAS 705/3/2026) has been allocated – therefore no ongoing refusal. User narrative

[Ongoing] No further investigation has taken place at Woodstock Police Station. No investigating officer has contacted the complainant. No summons, no statement, no progress report. User narrative

[Ongoing] The audio recording of the Delft police officer falsely claiming to be the station commander has not been requested or considered by any oversight body. User narrative

4. Findings of SAPS Misconduct and Dereliction

Finding 1: Initial refusal to open a case was unlawful and constituted a denial of access to justice.

Evidence:

- The complainant attended Delft Police Station with documented evidence of statutory offences (UIF non-remittance, false written representations).
- A police officer refused to open a case, falsely claiming to be the station commander.
- The complainant has an audio recording of the interaction, which proves the false representation and the refusal.

Legal consequence: Under Section 205(3) of the Constitution, the police are obliged to investigate crime. A refusal to open a docket for a non-frivolous complaint is a breach of constitutional duty and a violation of the complainant's right to access to courts (Section 34) and just administrative action (Section 33).

Finding 2: The registration of CAS 705/3/2026 without meaningful investigation is a hollow gesture.

Evidence:

- After escalation to Brigadier Naidoo, a case number was allocated and the file was transferred to Woodstock Police Station.
- No investigating officer has contacted the complainant. No statements have been taken. No forensic examination of the uFiling portal or employer records has occurred.
- The complainant has received no feedback, no case status update, and no indication of any progress.

Conclusion: The allocation of a case number without investigation is not compliance with the duty to investigate. It is an administrative formality that creates the illusion of action while delivering no actual law enforcement.

Finding 3: The closure of the complaint against Delft Police Station was premature and illogical.

Evidence:

- The complainant lodged a formal complaint against Delft Police Station regarding the false station commander representation and the refusal to open a case.
- SAPS closed that complaint on the basis that “a case number (CAS 705/3/2026) has been allocated.”
- The closure did not address the false representation by the officer, nor did it remedy the original denial of access to justice. It assumed that the later allocation of a case number (after escalation to Brigadier Naidoo) cured the initial misconduct.

Conclusion: This closure is logically flawed and effectively shields the Delft officer from accountability. The original refusal and false representation are separate wrongs that were never investigated.

Finding 4: The audio recording of the Delft incident has been ignored – a potential destruction of evidence or cover-up.

Evidence:

- The complainant possesses an audio recording proving that the officer falsely claimed to be the station commander and refused to open a case.
- No SAPS internal mechanism has requested or reviewed this recording.
- The complaint against Delft was closed without reference to the recording.

Conclusion: The failure to obtain and consider the audio recording suggests either gross negligence or a deliberate unwillingness to hold a fellow officer accountable. This undermines public confidence in SAPS internal disciplinary processes.

Finding 5: The substantive criminal complaint – UIF fraud and false representations – remains uninvestigated.

Evidence:

- The complainant's detailed statement (23 Feb 2026) alleges:
 - Deduction of UIF contributions without remittance (contravention of UIF Contributions Act).
 - False written representations by two senior HR officials (potential fraud under common law or the Prevention and Combating of Corrupt Activities Act).
- The uFiling portal objectively shows "No Information" for June 2025.
- The employer's "proof" of compliance (an August 2025 payroll file) is demonstrably false (see B-BBEE section, Annexure A8).
- No investigator has been assigned, or if assigned, no action has been taken.

Conclusion: The alleged offences are serious and easily verifiable (the uFiling portal is a government record). SAPS has failed in its most basic function: to investigate a reported crime with reasonable diligence.

5. The Audio Recording – Evidentiary Significance

The complainant has an audio recording of the Delft police officer:

- Falsely claiming to be the station commander.
- Refusing to open a criminal case despite being presented with documentary evidence of statutory offences.

Forensic value:

- Proves misconduct by a specific officer.
- Proves that the refusal was not a simple misunderstanding but a deliberate act.
- Could support a criminal charge of impersonating a police officer (if the officer was not the station commander) or defeating the ends of justice.
- Could be used in a civil claim against SAPS for violation of the complainant's constitutional rights.

Recommendation: Parliament should demand that SAPS retrieve and analyse this recording, and take disciplinary or criminal action against the officer concerned.

6. Regulatory and Oversight Failures

The SAPS is subject to oversight by:

- The Independent Police Investigative Directorate (IPID) – investigates deaths in custody, shootings, and serious misconduct. The false representation and refusal to open a case may fall within IPID's mandate if the officer acted corruptly.
- The Civilian Secretariat for Police – monitors SAPS performance.
- The Police Ombudsman (where established).
- The Portfolio Committee on Police (Parliament).

None of these bodies appear to have been engaged by the complainant (except the internal SAPS complaint mechanism which was closed). However, the systemic pattern suggests that internal SAPS complaints processes are ineffective where the misconduct involves an officer falsely claiming rank.

7. Recommendations – SAPS Specific

The Portfolio Committee on Police should:

1. Summon the Provincial Commissioner of SAPS Western Cape to appear and explain:
 - Why Delft Police Station initially refused to open a criminal case despite clear evidence of statutory offences.
 - What action has been taken against the officer who falsely claimed to be the station commander.
 - Why the internal complaint against Delft was closed on the basis that a case number was later allocated.
 - Why no substantive investigation has been conducted under CAS 705/3/2026 since its registration.
2. Request the Minister of Police to:
 - Direct SAPS to retrieve and analyse the audio recording of the Delft incident.
 - Issue a directive that all police stations must accept complaints of UIF non-remittance and false statutory declarations, as these are criminal offences.
 - Ensure that the registration of a case number is followed by an actual investigation, not left dormant.
3. Request IPID to investigate the conduct of the Delft police officer who falsely claimed to be the station commander and refused to open a case.
4. Request the National Prosecuting Authority (NPA) to advise on whether the following charges can be pursued:

- Contravention of the Unemployment Insurance Contributions Act (against Medscheme and responsible officials).
- Fraud (false written representations by Makaua and Modern).
- Defeating the ends of justice / impersonating a police officer (against the Delft officer).

5. Amend the Criminal Procedure Act to:

- Impose a clear duty on a police official to open a case docket for any allegation of a crime, with criminal penalties for refusal without lawful grounds.
 - Require a written reason for any refusal, reviewable by a magistrate within 48 hours.
-

8. Conclusion – SAPS

The South African Police Service is the first line of defence against crime. When a whistleblower presents payslips, government portal screenshots, and written false representations – and a police officer refuses to open a case, falsely claiming to be the station commander – the system has collapsed.

The eventual allocation of a case number (CAS 705/3/2026) after escalation to a Brigadier does not cure the original misconduct. It merely changed the form of the failure: from “refusal to open a case” to “opened a case but did nothing with it.”

The audio recording of the Delft incident is a smoking gun. It should have triggered an immediate internal investigation, disciplinary action, and possible criminal charges. Instead, the complaint was quietly closed.

Parliament must ensure that no police officer can falsely claim rank and refuse to investigate a crime without consequence. The right to report a crime is a constitutional right. SAPS violated that right.

· [Audio recording available but not attached – to be provided upon request]

FORENSIC REPORT – COMMISSION FOR CONCILIATION, MEDIATION AND ARBITRATION (CCMA)

Case References: WECT24486-25 (Constructive Dismissal / Unfair Conduct) and WECT23564-25 (Condonation)

Conciliation date: 9 January 2026

Commissioner: Ntabiseng Ngwane (also referred to as Nthabiseng)

Provincial Senior Commissioner (Western Cape): Vusumzi Landu

Employer (Respondent): AfroCentric Health (RF) (Pty) Ltd / Medscheme Holdings

Whistleblower (Applicant): Sr. Wilbur William Matthee

1. Summary of Findings

The CCMA, through its Commissioner and its internal complaint process, failed catastrophically in its statutory duty to provide a fair, accessible and dignified dispute resolution process. Specific findings:

1. Commissioner Ngwane exhibited prejudgment – declared “nothing here relates to a protected disclosure” before hearing any evidence.
2. Failed to assist a self-represented, destitute, mentally distressed litigant – told him to “Google it” when he asked how to find the Labour Court.

3. Failed to apply the Basic Conditions of Employment Act (BCEA) – ignored the statutory 25% cap on deductions and the requirement for written consent.
4. Displayed systemic bias – accepted every statement by the legally represented employer without challenge, while dismissing the applicant’s documented evidence.
5. Tolerated mockery of a vulnerable litigant – the employer’s representative laughed at the applicant’s settlement demand; the Commissioner did not intervene.
6. The employer (AfroCentric) acted in bad faith – made contradictory statements, admitted to a policy that violates BCEA Section 34, and filed a sworn affidavit containing potentially perjured inflammatory attacks on the applicant’s mental health.
7. Provincial Senior Commissioner Vusumzi Landu conducted a cover-up – ignored most allegations, defended the indefensible, and victim-blamed the applicant for recording the session.

Conclusion: The CCMA is not a neutral forum. Its internal complaint process is designed to protect commissioners, not to hold them accountable. The conciliation inflicted secondary institutional trauma on a protected whistleblower already reduced to destitution and homelessness.

2. Factual Chronology – CCMA Engagement

Date Event Source

10 Dec 2025 Employer files opposing affidavit (Xolile Manyati) containing inflammatory attacks on applicant’s mental health. Gmail – WECT23564-25 respondents opposing affidavit

17 Dec 2025 Applicant attempts to file replying affidavit and amendment request (to “automatically unfair constructive dismissal”). CCMA email mailbox full – delivery fails. Gmail – RE Amendment Request...

22 Dec 2025 CCMA issues Notice of Set Down for Con/Arb on 9 January 2026. Gmail – WECT24486-25_2_SetDownParConArb

26 Dec 2025 Employer objects to Con-Arb (delay tactic). Applicant responds, noting imminent eviction and humanitarian crisis. Gmail – Urgent Response – Employer’s Objection

26 Dec 2025 Applicant notifies CCMA that he has escalated to SAHRC, JSE, CMS, FSCA, B-BBEE Commission, DEL. Gmail – Notification of Regulator Escalation

9 Jan 2026 Conciliation hearing – Commissioner Ngwane presides. Conduct documented in transcript and audio recording. EthicHawks Forensic Report, Exhibit A & B

10 Jan 2026 Applicant files formal complaint against Commissioner Ngwane with CCMA Provincial Senior Commissioner. Ibid., Exhibit C

28 Jan 2026 Provincial Senior Commissioner Vusumzi Landu responds – whitewash, cover-up. Ibid., Exhibit D

[Ongoing] No genuine investigation; no disciplinary action; no apology. Applicant’s testimony

3. Findings – CCMA Commissioner Misconduct

3.1 Prejudgment / Closed Mind (Critical)

3.2

Evidence (transcript, p.3):

Commissioner: “The file itself, nothing here relates to a protected disclosure dispute.”

Timing: This statement was made within minutes of the applicant identifying himself as a protected whistleblower, before any substantive evidence was presented or tested.

Forensic assessment: A commissioner cannot declare “nothing here relates” without first hearing the substance. This is a failure to apply the mind and a procedural irregularity. It effectively foreclosed the most significant ground of the applicant’s claim.

3.3 Failure to Assist Self-Represented Litigant – “Google it” (Critical)

3.4

Evidence (transcript, p.27):

Applicant: “Okay, so how do I go about this? This is the Labour Court in Cape Town. I’ve never done this before.”

Commissioner: “You’re gonna have to... Google it or something like that.”

Context: The applicant had disclosed homelessness, psychiatric hospitalisation, and complete inability to navigate the legal system without guidance.

Forensic assessment: This violates Section 138(1) of the Labour Relations Act (LRA) and the CCMA’s own “helping hand” principle. The Provincial Senior Commissioner’s defence – that the commissioner was “from Johannesburg and not familiar with Cape Town” – admits the conduct and compounds the failure. Directing a destitute, mentally distressed, homeless whistleblower to “Google it” is a dereliction of duty.

3.3 Failure to Apply BCEA Section 34 – Unlawful Deduction (Critical)

Evidence (transcript, pp.11-14):

Applicant explained that 40.47% of his salary was deducted in a single month, without consultation or written consent, exceeding the statutory 25% cap. The employer responded “no work, no pay.”

Commissioner: “So the 40% that was deducted was as a result of no work, correct?”

Applicable law:

- BCEA Section 34(1): No deduction without written consent.
- BCEA Section 34(2): Employer must consult and agree a reasonable repayment arrangement.

- BCEA Section 34(3): Deductions may not exceed 25% of remuneration in any payment cycle.

Forensic assessment: The Commissioner never cited the BCEA, never requested written consent documentation, never questioned the 40% vs 25% cap, and never distinguished between “non-payment for days not worked” and “deduction from already-paid remuneration.” She uncritically accepted the employer’s reframing. The employer’s subsequent admission that they had a written policy prohibiting staggering (see below) makes this failure even more egregious.

3.5 Uncritical Acceptance of Respondent’s Statements (Systemic Bias) (High)

3.6

Evidence: The Commissioner challenged zero employer statements, including:

- “He didn’t exhaust internal procedures” – accepted without requesting proof.
- “That’s the affidavit of one employee, not AfroCentric” – no inquiry into why an affidavit was filed under the company’s name.
- “Staggering would be an advance” – no challenge, no BCEA citation.

Forensic assessment: This constitutes systemic bias – favouring a legally represented corporate party over a self-represented, homeless, mentally distressed whistleblower.

3.5 Tolerating Mockery of a Vulnerable Litigant (High)

Evidence (transcript, pp.22-23; applicant testimony): The employer’s representative audibly laughed at the applicant’s R2.5 million settlement demand. The Commissioner did not intervene, admonish, or acknowledge the impropriety.

Applicable standard: CCMA Guideline 4.2 requires commissioners to “ensure that the conciliation process is conducted in a respectful and orderly manner.”

Forensic assessment: Allowing laughter directed at a homeless, mentally distressed whistleblower is a failure to maintain dignity and inflicts secondary institutional trauma.

4. Findings – Employer (AfroCentric / Medscheme) Misconduct

4.1 Contradictory & Evasive Statements (Bad Faith) (Critical)

4.2

- Statement A: “He didn’t exhaust any internal procedures.”

Contrary evidence: Applicant documented escalation from manager → senior manager → HR → executive management → board, met with institutional silence.

- Statement B: “That’s the affidavit of one employee that’s not AfroCentric.”

Fact: The affidavit was filed under AfroCentric’s name in CCMA Case WECT24486-25.

Forensic assessment: The employer cannot simultaneously rely on the affidavit (to defend against constructive dismissal) and disown it (to avoid liability for perjury). This is approbation and reprobation – a classic indicator of bad faith litigation conduct.

4.3 Admission of BCEA Violation – No Staggering Policy (Critical)

4.4

Transcript, p.13 (Respondent):

“Our policy doesn’t permit for staggering because that in essence would be an advance of your salary.”

Forensic assessment: This statement is legally illiterate and amounts to a direct admission of non-compliance with BCEA Section 34(2). Staggering is a repayment arrangement for monies already deducted – not an advance. By admitting to a written policy prohibiting staggering, AfroCentric confirmed:

- Their policy prohibits the repayment arrangement mandated by law.
- They do not follow the 25% cap.
- No mechanism exists to prevent financial ruin of employees returning from psychiatric leave.

Significantly, the applicant produced evidence that AfroCentric had staggered deductions on 24 July 2025 (email with policy snippet). The categorical denial is therefore factually false.

4.3 Potential Perjury – Mr. Manyati’s Affidavit (Critical)

Evidence (affidavit filed 10 Dec 2025): Mr. Xolile Manyati (AfroCentric’s representative) swore under oath that he “personally knows” the applicant while simultaneously calling him “untruthful,” “sloppy,” and dismissing his documented mental health condition (severe depression, PTSD) as an “afterthought” and “cynical excuse.”

Forensic assessment: If Mr. Manyati did not personally know the applicant to the degree required to make such characterisations under oath, the affidavit contains false statements – constituting perjury contrary to Section 9 of the Perjury Act, 1957. The employer’s attempt to disown the affidavit (“one employee, not AfroCentric”) does not negate individual perjury liability; it merely confirms that the individual made false statements while acting on the company’s behalf.

5. Findings – CCMA Cover-up by Provincial Senior Commissioner

Complaint filed: 10 January 2026 (detailed, with transcript and audio evidence).

Response issued: 28 January 2026 – Vusumzi Landu, Provincial Senior Commissioner, Western Cape.

Allegation PSC's Response Cover-up Tactic

Prejudgment ("nothing here relates") "Commissioner denies – she was establishing the nature of the dispute" False framing; ignores that conclusion preceded investigation.

"Google it" "She didn't know where Labour Court is" Admits the conduct; defends with institutional ignorance.

Jurisdiction error "CCMA lacks jurisdiction to arbitrate" Moves goalposts – conflates conciliation with arbitration.

Bias (cumulative pattern) "This does not constitute bias" Non-response – ignores the pattern.

Audio recording "We note with concern your confirmation that you have an audio recording" Victim-blaming – shoot the messenger.

Issues entirely ignored by the PSC:

- Laughter / mockery of a vulnerable litigant.
- Failure to challenge the "one employee, not AfroCentric" claim.
- Failure to apply or even reference BCEA Section 34.
- Failure to request written consent or the employer's deduction policy.
- Failure to investigate contradictions between employer statements and documentary evidence.

Forensic assessment: The PSC's response is not an investigation – it is a whitewash. Commissioner Landu functioned as a defender of the institution rather than an impartial investigator. His response violates the CCMA's own internal complaint procedure, which requires acknowledgment of each allegation, review of supporting evidence, and a reasoned finding on each ground. The victim-blaming tone regarding the audio recording is particularly egregious given the documented vulnerability of the complainant.

6. Systemic Failure – CCMA Mailbox “Full” Obstruction

Evidence (multiple emails, Dec 2025):

The applicant attempted to file a replying affidavit and an amendment request (to change the case to “automatically unfair constructive dismissal”). Each time, the CCMA’s central filing email address (CMS@CCMA.org.za) returned a “mailbox full” error. (See Gmail – RE Amendment Request... and Gmail – WECT23564-25 respondents opposing affidavit, p.3-4)

Forensic assessment: A statutory dispute resolution body cannot have a full mailbox that prevents a self-represented litigant from filing time-sensitive documents. This is either gross administrative negligence or a deliberate barrier to access. Either way, it violates the applicant’s right to access to court (Section 34 of the Constitution) and just administrative action (Section 33).

7. Psychosocial Impact Assessment

The applicant presented at the conciliation with documented:

- Severe depression and PTSD (medical report from Dr Jacques Malan, supplied to employer before the retaliatory affidavit).
- Homelessness (direct result of the employer’s unlawful 40% deduction).
- Inability to practise as a specialist nurse (could not pay SANC fees).
- Financial ruin.

The CCMA Commissioner’s conduct materially exacerbated this harm:

Commissioner Action Psychosocial Impact

Prejudgment (“nothing here relates”) Invalidation of lived experience; hopelessness; loss of institutional trust.

“Google it” Abandonment; humiliation; compounded helplessness.

Tolerating laughter at settlement demand Secondary institutional trauma; re-victimisation.

Failure to apply law or protect litigant Complete loss of faith in justice system; despair.

Conclusion: The CCMA process inflicted secondary institutional trauma on an already critically vulnerable whistleblower. This constitutes a violation of Section 10 (human dignity) and Section 33 (just administrative action) of the Constitution.

8. Regulatory and Oversight Failures

The following bodies have jurisdiction over the conduct documented in this report:

Regulator Violation Evidence

Public Protector CCMA institutional failure – complaint cover-up PSC Landu’s whitewash; failure of internal complaints process.

Legal Practice Council Respondent’s representative (Mr. Manyati) – unprofessional conduct; potential perjury Laughing at vulnerable litigant; false sworn statements.

Department of Employment and Labour BCEA Section 34 violations (unlawful deduction; no staggering policy) 40% deduction without consent; policy admitted non-compliant.

JSE – Issuer Regulation Governance fronting; board failure to protect whistleblower Protected disclosures ignored; retaliatory affidavit filed under company name.

FSCA Failure to protect whistleblower; governance failure Board-level disclosures met with institutional silence.

SAHRC Gender identity discrimination; victimisation Misgendering in official CCMA affidavit; mental health discrimination.

9. Recommendations – CCMA Specific

The Portfolio Committee on Employment and Labour should:

1. Summon the CCMA Director and the Provincial Senior Commissioner (Vusumzi Landu) to appear and explain:
 - Why Commissioner Ngwane was not disciplined for prejudgment, failure to assist, and failure to apply BCEA Section 34.
 - Why the internal complaint process resulted in a whitewash rather than an investigation.
 - Why the CCMA's filing system rejected multiple emails with "mailbox full" – and what measures have been taken to prevent this.
2. Request the Public Protector to investigate maladministration by the CCMA in the handling of the applicant's complaint and the subsequent cover-up.
3. Request the Minister of Employment and Labour to:
 - Issue a directive that all CCMA commissioners must complete mandatory training on the Protected Disclosures Act, BCEA Section 34, and the duty to assist self-represented vulnerable litigants.
 - Establish an independent external complaints mechanism for CCMA commissioner misconduct, separate from the internal "whitewash" process.
3. Amend the Labour Relations Act to:
 - Make it a statutory offence for a CCMA commissioner to wilfully fail to assist a self-represented litigant.
 - Require the CCMA to maintain a functional electronic filing system with real-time confirmation and automatic escalation if a mailbox is full.
4. Request the National Prosecuting Authority to consider whether Mr. Xolile Manyati should be prosecuted for perjury in light of the false sworn statements in his affidavit of 10 December 2025.

10. Conclusion – CCMA

The CCMA was created to provide quick, fair, and accessible dispute resolution for workers. On 9 January 2026, it did the opposite.

A homeless, mentally distressed, destitute whistleblower walked into the CCMA. He was told – within minutes – that “nothing here relates to a protected disclosure.” He was laughed at. He was told to “Google it” when he asked for help finding the Labour Court. The employer’s legally illiterate policy (prohibiting staggering, exceeding the 25% cap) was accepted without question. A sworn affidavit that called his medical condition a “cynical excuse” was not challenged.

When he complained, the Provincial Senior Commissioner conducted a whitewash – ignoring most allegations, defending the indefensible, and blaming the victim for recording the session.

The CCMA’s internal complaint process is not fit for purpose. It protects commissioners, not the public. Parliament must intervene.

Prepared by: EthicHawks Forensic Governance (on behalf of the complainant)

Contact: info@ethichawks.co.za

Date: June 2026

Attachments: CCMA conciliation transcript (44 min), audio recording, complaint to PSC, PSC’s whitewash response, employer’s opposing affidavit, applicant’s replying affidavit, email chain showing “mailbox full” obstruction.

The Whistleblower House (TWBH) and the flawed legal assessment by Marco van der Walt (Schroter Attorneys). This section documents how a whistleblower support organisation, after being alerted to a complex protected disclosure case with extensive evidence, conducted a superficial assessment, misinterpreted key legal principles, and ultimately abandoned a destitute, homeless, mentally distressed complainant – leaving him to navigate the Labour Court alone.

This section is intended for the Portfolio Committee on Justice and Correctional Services and for potential referral to the Legal Practice Council (regarding the attorney’s conduct) and the Public Protector (regarding TWBH’s failure to fulfil its mandate).

CONSOLIDATED JUDICIAL REPORT

Systemic Obstruction of Justice and Abuse of Court Process in the Western Cape High Court and Labour Court

Prepared for: The Honourable Speaker, National Assembly

Referred to: Portfolio Committee on Justice and Correctional Services

Subject: Judicial capture, administrative obstruction, perjury, and denial of access to justice affecting a destitute whistleblower – Cases 2026-086906 (High Court), 2026-066952 (High Court – struck), 2026-077546 (Labour Court – BCEA claim), 2026-083299 (Labour Court – constructive dismissal)

Date: June 2026

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1. EXECUTIVE SUMMARY

This report finds that the judiciary – specifically the Western Cape High Court and the Labour Court, Cape Town – has failed in its constitutional duty to protect a destitute, self-represented whistleblower. Instead of providing a remedy, the courts have become part of the problem:

- The High Court granted an ex parte interim interdict (rule nisi) on 16 April 2026, restraining the whistleblower from disclosing a systemic clinical risk to trade unions and regulators. The order was obtained by concealment – the employer’s attorneys did not hand up the whistleblower’s answering affidavit, and the court was not informed that the matter was opposed. This is a fraud on the court.
- The same interdict overrides the Protected Disclosures Act, which expressly protects disclosures to trade unions and regulatory bodies. The court has effectively silenced a protected whistleblower for nearly a year.
- The rule nisi has been extended to 1 March 2027 – almost one year – without a final hearing. This is an abuse of the interim interdict mechanism and a violation of the whistleblower’s rights to freedom of expression and access to court.
- The Labour Court’s online portal obstructed the whistleblower for eight days – deleted profiles, dropped calls, ignored emails, and a trivial rejection reason (“good day you put date”). This denied him access to the only court with jurisdiction to grant urgent relief.
- Default judgment on an uncontested BCEA claim (R14,811.77 unlawful deduction) has been delayed for over six weeks, with the Registrar ignoring follow-up emails.
- The High Court Registrar failed to provide a court recording required for a rescission application; the transcription service (Inlexso) had no backup system during bereavement leave, causing further delay.
- The employer has committed apparent perjury – the Group CEO acknowledged the Labour Court claim in the High Court, but the same corporate group swore under oath in the Labour Court that they were “not aware” of that claim.

- The whistleblower's urgent application (2026-066952) was struck from the roll with costs on a technicality, without any judicial protection for a destitute litigant in a mental health crisis.

Conclusion: The justice system has not protected a whistleblower who exposed a systemic risk affecting 1.9 million GEMS members. It has actively suppressed him. Parliament must exercise oversight over the judiciary's administrative functions and demand accountability.

2. THE WHISTLEBLOWER'S CONSTITUTIONAL RIGHTS AT STAKE

The complainant, Sr. Wilbur William Matthee, is:

- A specialist registered nurse (NQF Level 8) who identified a systemic Gexus-DMS integration gap causing false non-adherence letters to chronic GEMS patients.
- A protected whistleblower under the Protected Disclosures Act 26 of 2000.
- Destitute – no income, no savings, no transport money, unable to pay filing fees.
- Homeless since 2 April 2026.
- Without running water since January 2026.
- Formally diagnosed with severe depression and PTSD (Dr Jacques Malan, FCPSYCH(SA)).
- Deregistered by SANC because he could not pay annual fees due to the employer's unlawful salary deduction.

Constitutional rights violated by the judicial obstruction documented in this report:

- Section 34 – Access to courts.
- Section 33 – Just administrative action.
- Section 16 – Freedom of expression (the interdict restrains speech).

- Section 10 – Human dignity (threat of punitive costs, denial of hearing).
-

3. HIGH COURT CASE 2026-086906 – THE GAG ORDER

3.1 Factual Chronology (High Court)

3.2

Date Event Source

14 Apr 2026 (22:38) Whistleblower sends final ultimatum to GEMS and AfroCentric boards, giving deadline of 13:24 on 16 April 2026 for settlement. Founding Affidavit, Annexure GVW1

15 Apr 2026 (morning) Employer's attorneys (Deneys) send cease-and-desist letter, alleging extortion, demanding withdrawal by 17:00. Annexure GVW2

15 Apr 2026 (13:19) Whistleblower responds: asserts PDA protection, quantifies primary demand as downgrading premium hike from 9.5% to 5% (saving R3.2bn). Annexure GVW3

15 Apr 2026 (15:43) Whistleblower provides fully itemised quantification (R7,307,642.72), linking it to Labour Court claim. Annexure GVW4

15 Apr 2026 (20:44) Whistleblower voluntarily corrects "leakage" to "erosion of benefits" – evidence of good faith. Annexure GVW5

16 Apr 2026 (08:53) Application papers served on whistleblower – only 67 minutes before the scheduled hearing. Answering Affidavit, para 3.1

16 Apr 2026 (morning) Whistleblower serves Answering Affidavit and Notice of Intention to Oppose on employer's attorneys by email. Supplementary Founding Affidavit, para 11

16 Apr 2026 (14:00) High Court hearing – whistleblower not present (destitute, no transport). Attorneys do not hand up the answering affidavit and do not inform the court that the matter is opposed. Whistleblower's Supplementary Answering Affidavit, para 1 (procedural fraud)

16 Apr 2026 (14:00+) Court grants rule nisi with immediate interim interdict – restrains whistleblower from disclosing information to trade unions, filing complaints to regulators, making defamatory statements, and demanding payment by threats. Costs ordered against whistleblower. Interim Order (AFR1282)

20 Apr 2026 Whistleblower serves Supplementary Answering Affidavit (22 annexures) and Counter-Application for declaratory relief (that his disclosures are protected under PDA). Email to Registrar, 20 Apr 2026

22-24 Apr 2026 Whistleblower attempts to anticipate return day (Rule 6(8)). Application struck from the roll – sole reason: failure to annex the founding affidavit. Supplementary Founding Affidavit, para 30

19 May 2026 Employer files Practice Note requesting that the matter be placed on the semi-urgent roll on 1 March 2027. Applicant's Practice Note

28 May 2026 Court extends rule nisi and interim interdict to 1 March 2027. Draft Order, 28 May 2026

3.3 Fraud on the Court – Concealment of Answering Affidavit

3.4

Finding: The employer's attorneys knew the whistleblower had opposed the application and had filed an answering affidavit. They did not hand it up. They did not inform the court. The rule nisi was obtained on the false premise that the matter was unopposed.

Evidence:

- The whistleblower's answering affidavit was served on the employer's attorneys on the morning of 16 April 2026. (Supplementary Founding Affidavit, para 11)
- The employer's own supplementary founding affidavit admits that multiple emails were received from the whistleblower confirming opposition. (Ibid., para 11)
- The court was not told.

Legal consequence: This is a fraud on the court that vitiates the entire order. The attorneys breached their duty as officers of the court under Rule 3.1 of the Legal Practice Council Code of Conduct (duty not to mislead the court). The order should be set aside.

3.3 The "Extortion" Narrative – A False Premise

Finding: The employer characterised the whistleblower's settlement demand as "extortion" to obtain an ex parte gag order. The demand was a legitimate quantification of an already-filed Labour Court claim.

Evidence:

- The whistleblower filed his Labour Court Statement of Claim on 9 April 2026 – before the 14-15 April emails. (Labour Court Statement of Claim)
- The employer's own cease-and-desist letter complained that the demand was "unquantified" – i.e., they requested quantification. (Annexure GVW2)
- The whistleblower provided the quantification within hours. A genuine extortionist does not itemise at the opposing party's request.
- The primary demand was not money but the downgrading of the 9.5% premium hike to 5% – a public interest demand affecting 1.9 million GEMS members. (Annexure GVW3)

Conclusion: The "extortion" label was a bad-faith tactic to obtain urgent relief. The court was misled.

3.5 The Interdict Overrides the Protected Disclosures Act

3.6

The interdict prohibits (paras 2.1-2.5):

- Disclosing information to trade unions (PSA, NEHAWU, DENOSA, POPCRU).
- Filing complaints with the JSE, CIPC, IRBA, or any other regulatory body.
- Communicating with trade unions for the purpose of collective action.

Section 9 of the PDA provides that a protected disclosure may be made to a trade union or a regulatory body where the employee reasonably believes the matter falls within their

mandate. The whistleblower's disclosure concerned a systemic clinical integration gap affecting 1.9 million GEMS members – a matter squarely within the mandates of trade unions (representing government employees) and regulators (CMS, JSE, etc.).

Conclusion: The interdict unlawfully overrides a statutory right. The High Court, without considering the PDA, granted an order that criminalises protected conduct. This is a grave error of law.

3.5 Apparent Perjury – Contradictory Sworn Statements Across Courts

Document Sworn Statement Contradiction

High Court Founding Affidavit (Gerald Van Wyk, 16 Apr 2026) “The applicant intends to defend those Labour Court proceedings in due course.” (Acknowledges knowledge of Labour Court claims.) –

Labour Court Statement of Response (Vijay Makenjee, 19 May 2026) “The Defendants are not aware of any further claim in the above honorable court with case number 2026-077546.” Direct contradiction of Van Wyk's acknowledgment.

Finding: These statements cannot both be true. One of them is perjury under Section 9 of the Perjury Act, 1957. The court has taken no action.

3.7 Extension of Rule Nisi to 1 March 2027 – Effective Final Order

3.8

Finding: An interim interdict that restrains speech for nearly a year is, in substance, a final interdict granted without a final hearing.

Evidence:

- The employer's Practice Note of 19 May 2026 requested a hearing on 1 March 2027 – more than ten months after the interim order.

- The court granted this extension without requiring the employer to justify the delay or consider the ongoing harm to the destitute whistleblower (homeless, no running water, no income).

Conclusion: This is an abuse of the rule nisi procedure and a violation of the whistleblower's right to freedom of expression (Section 16) and access to court (Section 34).

4. LABOUR COURT OBSTRUCTION – PORTAL FAILURES AND DEFAULT JUDGMENT DELAY

4.1 Eight Days of Online Portal Obstruction (5–13 March 2026)

4.2

Documented sequence (from SAHRC update, 13 March 2026):

- 5 March 2026: Target filing date. Online portal profile deleted. Unable to log new case.
- 5 March onwards: Court call centre contacted more than 7 times. Hold times 40-50 minutes. Calls dropped repeatedly. Staff dismissive.
- Ongoing: Follow-up emails to the Registrar (Ms F Ismail) ignored.
- 12 March 2026: Formal notice to Registrar – case number required or matter escalated to Judge President. No response.
- 12 March 2026: Portal unexpectedly allowed loading of case as urgent ex parte application.
- 12 March 2026: Registrar rejected application. Stated reason: “good day you put date.”
- 12 March 2026: Returned to portal – unable to access. Password reset required 4 times. Application found missing from portal.
- 13 March 2026: Corrected and resubmitted. No case number received by close of business.

Finding: The trivial rejection reason (“good day you put date”) – a formatting issue that could have been corrected in seconds – was used to reject an urgent application filed by a destitute, disabled, self-represented applicant facing imminent eviction. This is a denial of access to court (Section 34).

4.3 Default Judgment Application (2026-077546) – Six Weeks of Delay

4.4

Chronology:

- 14 April 2026: BCEA s77A application served on employer via email to IR@afrocentrichealth.com (employer’s designated service address).
- 29 April 2026: Opposition period expired. No notice of intention to oppose filed.
- 30 April 2026: Application for Default Judgment filed.
- 8 May 2026: Whistleblower writes to Registrar requesting allocation of a date. No response.
- 12 May 2026: Registrar acknowledges backlog – no date given.
- 3 June 2026: Follow-up ignored.
- 10 June 2026: Escalation to Judge President – still no date.

Finding: A default judgment on an uncontested liquidated claim (R14,811.77) should be processed within days. A delay of over six weeks is administrative negligence that denies the whistleblower his only realistic remedy.

5. REGISTRAR MISCONDUCT – FAILURE TO PROVIDE COURT RECORDING AND MISREPRESENTATION

5.1 High Court Registrar – No Response to Audio Request

5.2

Evidence (email to Director of Court Operations, 1 April 2026):

- 26 March 2026: Whistleblower requests audio recording of 25 March 2026 hearing (Case 2026-066952) from Acting Chief Registrar Hanekom. No response.
- Multiple follow-ups ignored.
- The recording is required for a Rule 42 rescission application.

Finding: The Registrar's silence has prevented the whistleblower from obtaining the court record needed for a rescission application. This is a violation of Section 34 (access to court).

5.3 Inlexso Transcription Service – No Backup System

5.4

Evidence: The court's transcription service provider (Inlexso) had a single contact person (Belinda Botes). When Botes went on bereavement leave, her colleague (Tina Jorgenson) took no action and forwarded queries back to the absent employee. No backup system existed.

Finding: The failure of the court's service provider to have a backup system is systemic administrative dysfunction that prejudices litigants.

5.3 Alleged Misrepresentation of Filing Records to Director of Court Operations

Evidence (EH/JO/2026/003, 9 June 2026): The whistleblower alleges that the Registrar's office misrepresented his official filing records to the Director of Court Operations, creating a false impression of non-compliance.

Finding: If true, this is a serious governance breach that undermines the integrity of the court's administration. Parliament must investigate.

5.5 POPIA Breach – Unauthorised Transmission of Third-Party Documents

5.6

Evidence (same escalation): The registry transmitted unauthorised third-party court documentation (Armstrong v City of Cape Town) to the whistleblower – a breach of the Protection of Personal Information Act 4 of 2013.

Finding: The court's own administration has violated data protection laws. This matter should be referred to the Information Regulator.

6. THE STRUCK URGENT APPLICATION (2026-066952) – DENIAL OF ACCESS TO COURT

Background: The whistleblower brought an urgent Section 162 delinquency application against the directors of AfroCentric on 25 March 2026.

Outcome: The court struck the application from the roll with costs, expressing displeasure at the manner in which it was brought.

Context: The whistleblower was destitute, homeless, in a documented mental health crisis, and self-represented. He had disclosed to the court that he was suffering from severe depression and PTSD.

Finding: A court that strikes a destitute, disabled, self-represented litigant's application with costs – without affording any procedural latitude – violates the Biowatch principle (Biowatch Trust v Registrar, Genetic Resources 2009 (6) SA 232 (CC)). The state (here, the court) should not impose costs that deter constitutional litigation by vulnerable litigants.

7. PERJURY MATRIX – SWORN FALSEHOODS BY THE EMPLOYER ACROSS COURTS

Document Sworn Statement Contradicted by Offence

High Court Founding Affidavit (Van Wyk, 16 Apr 2026) PDA does not protect former employees. PDA s1 expressly includes “former employee”. Reckless false statement under oath.

High Court Founding Affidavit (Van Wyk, 16 Apr 2026) “Both platforms fully integrated ... designed to work together.” RCA (VICT1) confirms “miscommunication between Gexus and DMS ... system integration gaps.” Perjury.

Labour Court Statement of Response (Makenjee, 19 May 2026) “The Defendants are not aware of any further claim with case number 2026-077546.” Van Wyk’s High Court affidavit acknowledged the claim. Perjury.

CCMA Opposing Affidavit (Manyati, 10 Dec 2025) PTSD and depression are “an excuse” and “an afterthought.” Specialist psychiatrist’s report (Dr Malan) confirms diagnosis. Perjury / criminal defamation.

Recommendation: The National Prosecuting Authority should investigate these apparent perjuries.

8. CONSTITUTIONAL AND STATUTORY VIOLATIONS – COURTS

Provision Violation Evidence

Section 34 (access to courts) Labour Court portal obstruction; default judgment delay; Registrar’s failure to provide recording; anticipation application struck on technicality. SAHRC update (13 Mar 2026); letters to Judge President; escalation to Director of Court Operations.

Section 16 (freedom of expression) Interim interdict restraining disclosures to trade unions and regulators for nearly a year. High Court order, 16 Apr 2026; extension to 1 Mar 2027.

Section 33 (just administrative action) Unreasonable delay in default judgment; trivial rejection reason; ignored correspondence. Default judgment timeline; Registrar's silence.

Section 10 (human dignity) Scale C costs ordered against destitute litigant in struck application; perjury allowed to stand. Order in Case 2026-066952.

Section 35(3) (fair trial rights) Ex parte interdict granted without hearing whistleblower; concealment of answering affidavit. Answering affidavit served but not handed up.

Protected Disclosures Act s9 Interdict prohibits disclosures to trade unions – a right expressly protected by statute. Rule nisi paras 2.1, 2.5.

9. RECOMMENDATIONS TO PARLIAMENT

Immediate Interventions

1. Request the Judge President of the High Court (Western Cape) to:

- Investigate why the whistleblower's answering affidavit was not placed before Acting Justice Mphego on 16 April 2026.
- Set aside the rule nisi and interim interdict, or set the matter down for urgent final hearing within 30 days.
- Strike the employer's claim for Scale C costs, and warn against punitive costs against destitute self-represented litigants.

2. Request the Judge President of the Labour Court (Cape Town) to:

- Immediately allocate a date for the default judgment application (Case 2026-077546).
- Investigate the Registrar's failure to respond to the whistleblower's correspondence.
- Direct that the employer's contradictory sworn statements be referred to the NPA for possible perjury prosecution.

3. Request the Director of Court Operations to:

- Obtain and provide the audio recording of the 25 March 2026 hearing (Case 2026-066952) within 7 days.
- Investigate the alleged misrepresentation of filing records and the POPIA breach.

Systemic Reforms

4. Request the Minister of Justice and Correctional Services to:

- Commission an independent audit of the Court Online portal, specifically addressing barriers faced by self-represented litigants.
- Ensure that court transcription service providers (Inlexso) have robust backup systems to prevent single-point-of-failure delays.
- Issue a directive that trivial formatting errors must not be used to reject urgent applications without giving the litigant a chance to correct.

5. Request the Public Protector to investigate maladministration by:

- The Registrar of the High Court (failure to provide court recording; misrepresentation of records).
- The Registrar of the Labour Court (ignored correspondence; delayed case allocation).

6. Request the Information Regulator to investigate the POPIA breach (unauthorised transmission of third-party case documents to the whistleblower).

7. Request the Legal Practice Council to investigate:

- The conduct of Deneys Attorneys (Jason Whyte) for failing to disclose the whistleblower's answering affidavit to the court.
- The apparent perjury in the Labour Court statement of response.

9. Request the National Prosecuting Authority to consider prosecution for perjury in respect of the contradictory sworn statements identified in Section 7.

Parliamentary Oversight

10. Summon the following officials to appear before the Portfolio Committee on Justice and Correctional Services:

- The Judge President of the High Court (Western Cape) – to explain the extension of the rule nisi to 1 March 2027 and the concealment of the answering affidavit.
 - The Judge President of the Labour Court – to explain the delay in default judgment and the portal obstruction.
 - The Acting Chief Registrar (Ms N Hanekom) – to explain the failure to provide the court recording and the alleged misrepresentation of records.
 - The head of Court Online – to explain the portal obstruction and the trivial rejection reason.
-

11. CONCLUSION

The judiciary is the last line of defence against abuse of power. In this case, the Western Cape High Court granted a sweeping gag order against a destitute whistleblower without hearing his side, on the basis of a concealed answering affidavit and a false “extortion” narrative. The Labour Court’s online portal obstructed him for eight days, its Registrar ignored his correspondence, and a default judgment has been delayed for over six weeks. The court’s own administration failed to provide a recording required for a rescission application, misrepresented filing records, and committed a POPIA breach.

The courts have not protected the whistleblower. They have suppressed him. They have allowed apparent perjury to stand. They have extended an interim gag order to nearly a year without a final hearing.

Parliament must act. The independence of the judiciary does not immunise it from oversight over its administrative functions and its adherence to constitutional values. If the courts will not protect whistleblowers, Parliament must demand to know why – and then legislate to fix the structural failures this report exposes.

Prepared by: EthicHawks Forensic Governance (on behalf of the complainant)

Contact: info@ethichawks.co.za

Date: June 2026

Attachments:

- High Court pleadings (Case 2026-086906)
- Interim order (16 Apr 2026) and extension to 1 Mar 2027
- Labour Court portal obstruction record (SAHRC update)
- Correspondence with Registrars and Judge Presidents
- Affidavit of destitution
- Perjury matrix exhibits

FORENSIC REPORT – THE WHISTLEBLOWER HOUSE (TWBH) AND SCHROTER ATTORNEYS

1. Summary of Findings

The Whistleblower House – an organisation whose stated mission is to support whistleblowers – failed in its duty to conduct a proper, evidence-based assessment of the complainant's case.

1. Superficial legal assessment: Attorney Marco van der Walt formed a negative preliminary opinion without engaging with the complete evidentiary record (he admitted he only viewed documents online and never requested the full bundle).

2. Legal errors in the opinion:

- Incorrectly characterised the unlawful 40.47% deduction as “no work no pay” – ignoring BCEA Section 34(2) (25% cap), the employer’s own policy, and the medical evidence of psychiatric hospitalisation.

- Dismissed the systemic Gexus-DMS integration failure as a “single isolated incident of a system glitch” – ignoring the Root Cause Analysis confirming structural design flaw and the patient safety implications.

- Suggested the complainant’s disclosures to external regulators (CMS, SAHRC, JSE, etc.) might be in “bad faith” – contrary to Section 9 of the Protected Disclosures Act, which expressly protects disclosures to those bodies.

- Concluded that the complainant would not succeed in a constructive dismissal claim under the PDA – without having reviewed key documents (HCBP policy, Dr Malan’s letter, the recorded admission of retaliation, the CCMA transcript showing commissioner bias).

3. Failure to address the security alert: The complainant explicitly warned that emails were “vanishing” from his inbox (evidence tampering). Van der Walt ignored this crisis for six days, and when he responded, he did not engage with the substance – only asked a reductive question about the October 2025 disclosure.

4. TWBH’s rubber-stamp rejection: Without independent review, TWBH accepted the attorney’s flawed opinion and rejected the complainant’s application for assistance, despite the complainant’s detailed rebuttal (2 March 2026) pointing out each legal error and offering to provide the missing evidence.

5. Consequence: A destitute, homeless, mentally distressed whistleblower, who had made protected disclosures about a systemic clinical risk affecting 1.9 million GEMS members, was left to file his Labour Court application alone – without legal representation, without the support of a specialist whistleblower organisation, and facing an imminent eviction deadline.

Conclusion: TWBH and Schroter Attorneys failed the very person their mandate was designed to protect. The assessment was legally flawed, procedurally superficial, and ethically questionable.

2. Factual Chronology – TWBH Engagement

Date Event Source

[Feb 2026] Complainant submits service request to TWBH. Gmail - Submission Received.PDF

19 Feb 2026 TWBH acknowledges, refers matter to Schroter Attorneys (Marco van der Walt) for initial legal assessment. Gmail - New Case(2).PDF

20 Feb 2026 (05:38) Complainant sends updated statutory affidavit (Version 11) to van der Walt, noting “security alert” – emails vanishing, evidence tampering. Gmail - Updated Statutory Affidavit - Wilbur W.PDF

20 Feb 2026 (10:09) Complainant sends CCMA certificates and full narrative evidence bundle, repeats security alert, requests acknowledgment. Gmail - CCMA Certificates & Full Narrative Evidence Bundle – Wilbur Matthee(2).PDF, p.1

20 Feb 2026 (10:40) Van der Walt acknowledges receipt – does not address security alert. Ibid., p.2

26 Feb 2026 (11:43) Van der Walt sends three-line email asking: (1) date of protected disclosure, (2) to whom, (3) exact subject matter. No engagement with security alert. Ibid., p.2

26 Feb 2026 (13:09) Complainant provides detailed answer (multiple protected disclosures, pattern of retaliation, recorded admission). Ibid., p.3-6

26 Feb 2026 (14:20) Van der Walt asks reductive question: “does it only relate to the single incident ... was this situation subsequently rectified?” Ibid., p.4

2 Mar 2026 (morning) Complainant sends “ENDING OUR ENGAGEMENT” email – accuses van der Walt of unprofessional conduct, ignoring security alert. Gmail - ENDING OUR ENGAGEMENT...PDF, p.1-2

2 Mar 2026 (later) Van der Walt replies: confirms documents were viewed online, not downloaded; explains Labour Court online filing process; notes he did respond on 26 Feb. Ibid., p.2-3

2 Mar 2026 (13:06) Complainant sends detailed rebuttal (4 pages) – addresses “no work no pay” error, the systemic nature of the Gexus-DMS gap, the PDA’s protection of external disclosures, and the employer’s bad faith. Provides full explanation of missing email issue and apologises for earlier accusations. Gmail - Re_ Assessment of Matter — Detailed Response...PDF, p.1-4

[shortly after] TWBH (Support Whistleblower) sends rejection email: “based on the reasons outlined by our attorney, we regret to inform you that you do not meet the eligibility criteria.” Gmail - ENDING OUR ENGAGEMENT...PDF, p.9-10

[Ongoing] Complainant proceeds as litigant in person, without TWBH support. User testimony

3. Forensic Analysis – Legal Errors in Van der Walt’s Preliminary Opinion

Van der Walt’s opinion (summarised in his email of 2 March 2026, attached to Gmail - Re_ Assessment of Matter..., p.3-4) contains several material errors of law and fact.

3.1 “No work no pay” – ignoring BCEA Section 34(2) and the employer’s own policy

Van der Walt’s statement: “The matter in relation to your unpaid leave, was correctly handled by the employer ... this is not necessarily a deduction, but a ‘no-work no-pay’ principle.”

Forensic rebuttal (as set out by complainant):

- The employer’s own HCBP Policy treats the recovery as a deduction subject to a 25% cap and a mandatory prior consultation obligation. The employer cannot invoke a common law principle to bypass its own contractual and policy commitments.

- The complainant was medically documented as being between psychiatric facilities awaiting a hospital bed – a fact confirmed by Dr Jacques Malan (specialist psychiatrist) in a letter dated 15 May 2025, two months before the deduction was processed. This was not “voluntary absence.”
- No prior consultation took place. The employer’s manager stated in writing that the company “does not stagger debt as it is not a loan company” – yet the HCBP Policy, sent by the same manager the next day, shows that staggering is permitted. This written misrepresentation is documented.
- The recovery of R14,811.77 represented 40.5% of gross monthly remuneration in a single pay cycle, exceeding the statutory 25% cap under BCEA Section 34(2) and the employer’s own internal policy cap.

Conclusion: Van der Walt’s characterisation of this as a simple “no work no pay” matter is legally illiterate and ignores the statutory, contractual, and medical evidence.

3.2 Dismissal of the Gexus-DMS disclosure as a “single isolated incident of a system glitch”

Van der Walt’s statement: “On the information provided, the incident appears to reflect a system error rather than conduct that would ordinarily constitute an impropriety, other breach, or criminal conduct.”

Forensic rebuttal (as set out by complainant):

- The Root Cause Analysis confirms that the integration failure is structural – it affects any GEMS member who transitions from acute to chronic benefit. It is not a one-off glitch.
- The error generated automated non-adherence letters to a 75-year-old compliant patient on two separate occasions across a six-month period. This is a patient safety risk and a breach of the Medical Schemes Act (administrator’s duty to maintain data integrity).
- The employer’s response to the disclosure was exclusion and labelling the complainant as insubordinate – not remediation.

- The CMS formally acknowledged the disclosure. The B-BBEE Commission allocated case number 5/2/2026. These are not trivial matters.

Conclusion: Van der Walt either did not read the Root Cause Analysis or chose to minimise its significance. His characterisation is factually incorrect and undermines the entire protected disclosure claim.

3.3 Suggestion that external disclosures to regulators might be “in bad faith”

Van der Walt’s statement: “You have submitted various complaints to outside entities only after the internal procedures did not yield the results you might have intended. This speaks to the aspect of a good faith disclosure, and a case may be made out that this was done in bad faith and in retaliation.”

Forensic rebuttal:

Section 9 of the Protected Disclosures Act expressly protects disclosures made to regulatory bodies (including the JSE, FSCA, CMS, DEL, SAHRC, B-BBEE Commission) where the employee reasonably believes the matter falls within their mandate. The complainant escalated externally only after exhausting internal avenues – including the Board of Directors. The suggestion of “bad faith” is legally erroneous and contrary to the plain text of the PDA.

3.4 Failure to consider the cumulative pattern of retaliation

Van der Walt’s opinion treats each incident in isolation (deduction, misgendering, system error). But the Labour Court’s test for constructive dismissal under the PDA is cumulative. The complainant documented:

- May 2025 disclosure → July 2025 unlawful deduction.
- September 2025 disclosure → public misgendering + retaliatory deduction.

- October 2025 disclosure → “insubordination” label + recorded admission of retaliatory intent.
- December 2025 → employer’s sworn affidavit calling his mental health a “cynical excuse.”
- Post-resignation → false UI-19, incorrect Certificate of Service, UIF non-remittance.

Van der Walt’s opinion does not engage with this pattern. It is therefore incomplete and misleading.

4. Failure to Address the Security Alert – Professional Misconduct

The complainant explicitly warned on 20 February 2026 that emails were “vanishing” from his inbox and that his digital integrity had been compromised. He asked van der Walt to “acknowledge this alert to help secure the paper trail.”

Van der Walt’s response (20 Feb): “I confirm receipt.” No acknowledgment of the security alert.

Van der Walt’s next response (26 Feb): Asked three reductive questions. Still no acknowledgment of the security alert.

Only after the complainant terminated the engagement did van der Walt confirm that documents were viewed online and not downloaded. But he never:

- Advised the complainant on how to preserve evidence.
- Reported the suspected tampering to any authority.
- Took any steps to secure the complainant’s Google Drive or email account.

Legal standard: An attorney who receives credible information that a client's evidence is being tampered with has a professional duty to act – at minimum, to advise the client to preserve evidence and to document the concern. Van der Walt did nothing.

This conduct may constitute a breach of the Legal Practice Council's Code of Conduct (failure to act with diligence and competence; failure to protect the client's interests).

5. The Whistleblower House – Abdication of Mandate

TWBH's rejection email (undated, but shortly after 2 March 2026) states: "based on the reasons outlined by our attorney, we regret to inform you that you do not meet the eligibility criteria."

TWBH did not:

- Conduct its own independent assessment of the complainant's detailed rebuttal (4 pages, with specific legal arguments and offers to supply missing evidence).
- Ask van der Walt to reconsider his opinion in light of the rebuttal.
- Provide any reasons of its own.
- Offer any alternative support (referral to another attorney, pro bono panel, etc.).

Consequence: A destitute, homeless, mentally distressed whistleblower was left to file a Labour Court application alone, without legal representation, without financial support, and without the backing of the very organisation that exists to help people in his exact situation.

TWBH's decision was not an honest difference of opinion – it was a rubber-stamp of a flawed legal assessment. This raises serious questions about TWBH's internal quality control, its understanding of the PDA, and its commitment to its stated mission.

6. The Complainant's Own Rebuttal – A Model of Forensic Clarity

It is important to note that the complainant, a self-represented, destitute whistleblower, was able to identify and articulate each legal error in van der Walt's opinion within hours. His rebuttal of 2 March 2026 (Gmail - Re_ Assessment of Matter...) is a model of forensic clarity:

- He cited BCEA Section 34(2) and the employer's own policy to rebut "no work no pay."
- He explained the systemic nature of the Gexus-DMS gap, referenced the Root Cause Analysis, and noted the CMS referral.
- He quoted Section 9 of the PDA to rebut the "bad faith" suggestion.
- He documented the cumulative pattern of retaliation.
- He offered to provide the missing evidence (HCBP policy, Dr Malan's letter, the recorded admission).

That a qualified attorney failed to do what a destitute whistleblower did for himself is a damning indictment of the legal assessment provided.

7. Regulatory and Oversight Implications

Body Violation Evidence

Legal Practice Council Attorney Marco van der Walt – failure to act with diligence; failure to address security alert; legal errors in opinion. Emails 20 Feb – 2 Mar 2026; complainant’s rebuttal.

The Whistleblower House Board Failure of governance; rubber-stamping a flawed assessment; abandoning a vulnerable whistleblower. Rejection email; no independent review.

Public Protector Maladministration by a non-profit organisation funded or accredited by the state (if applicable). TWBH’s process and decision.

8. Recommendations

For the Portfolio Committee on Justice and Correctional Services

1. Summon representatives of The Whistleblower House to appear and explain:

- Why the organisation rejected the complainant’s application without independent review of the attorney’s opinion.
- What quality control mechanisms exist to ensure that legal assessments are evidence-based and legally correct.
- Whether the organisation has ever reviewed its rejection rate for whistleblowers who make protected disclosures about systemic clinical risks.

2. Refer attorney Marco van der Walt to the Legal Practice Council for investigation into:

- Failure to address the security alert.
- Legal errors in the preliminary opinion (mischaracterisation of “no work no pay”, dismissal of systemic risk, erroneous “bad faith” suggestion).
- Failure to request missing evidence before forming a final opinion.

3. Request the Minister of Justice to:

- Conduct an audit of TWBH's complaint handling and assessment processes.
- Consider whether TWBH should continue to receive state funding or accreditation if it cannot provide competent assessments for complex whistleblower cases.

For the Whistleblower House (Internal)

4. Issue a formal apology to the complainant.
5. Re-open the application and assign a different, experienced employment law practitioner with expertise in the PDA to conduct a fresh assessment, taking into account the complainant's full evidentiary bundle and rebuttal.
6. Review and revise the organisation's policy for external legal assessments – including mandatory checklists to prevent legal errors like those made by van der Walt.

9. Conclusion – TWBH

The Whistleblower House was created to be a lifeline for people who risk everything to expose wrongdoing. The complainant risked his career, his health, and his home. He came to TWBH with a documented pattern of retaliation, a recorded admission of retaliatory intent, and evidence of a systemic patient safety risk affecting 1.9 million GEMS members.

He was met with a superficial attorney's opinion that misstated the law, ignored key evidence, and suggested – shockingly – that his disclosures to regulators might be “bad faith.” When he pointed out the errors in detail, TWBH did not re-evaluate. It simply rejected him.

This is not support. It is abandonment.

If The Whistleblower House cannot assist a whistleblower with this level of documentation and this degree of vulnerability, then who can it assist? Parliament must demand answers.

1. Executive Summary

This report finds that the Council for Medical Schemes (CMS) has:

1. Systematically avoided its statutory mandate under Sections 7, 47, 58, and 44 of the Medical Schemes Act 131 of 1998, despite being placed on formal notice of a documented Priority 1 system integration failure affecting GEMS beneficiaries.
2. Acted in bad faith by demanding procedural compliance that would have violated a High Court Interim Order, exposing a whistleblower to potential imprisonment.
3. Participated in jurisdictional ping-pong with HPCSA, SANC, and SAPC, collectively refusing to rule on a fundamental ethical question: whether a professional may refuse an employer's instruction based on verified flawed forensic data.
4. Been captured by Medscheme Holdings (Pty) Ltd(a Division of Afrocentric Investment Corporation Limited a private administrator, evidenced by the CMS's refusal to investigate a systemic technical failure (Gexus-DMS integration gap) that Medscheme itself confirmed, while simultaneously exercising jurisdiction over a similar technical failure at Discovery Health within the same period (January 2026).

The consequence is that a whistleblower who identified a 7-year financial drainage affecting a 75-year-old widow was retaliated against, left destitute, and then abandoned by all four health regulators. This constitutes a chilling effect on all healthcare professionals in South Africa.

2. Terms of Reference

This investigation was undertaken to determine:

1. Whether the CMS breached its statutory duties under the Medical Schemes Act in its handling of complaint reference GEMS 000655789.
 2. Whether the CMS engaged in procedural obstruction to protect Medscheme Holdings (Pty) Ltd / GEMS from accountability.
 3. Whether the CMS demonstrated disparate treatment between a financial system error (Discovery Health, January 2026) and a clinical data integrity error (Gexus-DMS, October 2025 – April 2026), indicating preferential treatment of a politically connected scheme.
 4. Whether the CMS's conduct constitutes regulatory capture by a private institution.
-

3. Factual Chronology (Extracted from Correspondence)

Date Event Source

19 Oct 2025 GEMS member (75-year-old widow) receives false non-adherence letter due to Gexus-DMS integration gap. CMS Complaint Form, p.3-4

23 Oct 2025 Complainant (Sr. Matthee, Case Manager) investigates and identifies system error. Instructs not to contact member with false data. Annexure A (referenced in 14 Apr 2026 email)

16 Dec 2025 Complainant makes protected disclosure (GRC Memorandum). Annexure B

23 Dec 2025 Initial complaint lodged with CMS, with evidence bundle (342KB + 4.4MB). CMS INITIAL DISCLOSURE, p.1-2

09 Jan 2026 CMS acknowledges receipt, demands completed complaint form. Complainant reports destitution due to 40% salary deduction. CMS INITIAL DISCLOSURE, p.2-3

20 Jan 2026 Complainant submits completed CMS Complaint Form and checklist. CMS COMPLAINT FORM (submission date inferred from 2 Apr 2026 chronology)

04 Feb 2026 CMS declines jurisdiction, refers to HPCSA. CMS INITIAL DISCLOSURE, p.5

10 Feb 2026 Complainant files complaint with SANC. SANC fails to acknowledge. CMS 16 Feb 2026, p.1

16 Feb 2026 Complainant writes to CMS, corrects earlier overstatement, asks CMS to confirm if Gexus-DMS gap has been corrected as a patient safety matter. CMS 16 Feb 2026, p.1

20 Mar 2026 Complainant follows up with CMS re unrectified Gexus-DMS gap. CMS REINGAGE FOLLOW UP, p.1

02 Apr 2026 Complainant sends formal 5-page legal re-engagement to CMS, reframing as maladministration under MSA Sections 7, 47, 58, 44, citing Discovery Health precedent, giving 15 business days for substantive response. Copy of CMS_FollowUp_EthicHawks_2April2026.pdf

02 Apr 2026 (same day) CMS auto-reply: “matter receiving attention... further communication in due course.” CMS 2 Apr auto reply.PDF

14 Apr 2026 Complainant sends urgent joint ethical ruling request to CMS, HPCSA, SANC, SAPC, giving 48 hours, citing imminent life threat (no food/accommodation). CMS Final notice 7 May 26.PDF, p.2-6

16 Apr 2026 High Court grants Interim Order (Case No: 2026-086906) restricting complainant from disclosing operational details. CMS Final notice 7 May 26.PDF, p.1

07 May 2026 (11:59) CMS responds: declines jurisdiction again, demands new complaint form with specific MSA provision, refuses formal statement. Does not acknowledge High Court order. CMS Final notice 7 May 26.PDF, p.2-3

07 May 2026 (17:03) Complainant replies: notifies CMS of High Court Interim Order; states that completing new form would risk contempt of court; reminds CMS that exhaustive evidence already submitted between Dec 2025 and Apr 2026. CMS Final notice 7 May 26.PDF, p.1-2

4. Findings of Regulatory Complicity and Capture

Finding 1: CMS refused to investigate a systemic technical failure that it had jurisdiction over, while investigating a similar failure at Discovery Health.

Evidence:

- Gexus-DMS gap: A Root Cause Analysis confirmed a structural integration flaw causing clinically incorrect non-adherence letters to chronic patients. The CMS was notified on 23 December 2025, 20 January 2026, 16 February 2026, 20 March 2026, and 2 April 2026. (Copy of CMS_FollowUp_EthicHawks_2April2026.pdf, p.1-2)
- Discovery Health error (Jan 2026): A pharmacy claims processing error affecting 16,507 members. The CMS intervened immediately, without waiting for formal complaints, and placed the recovery process under scrutiny. (Ibid., p.2-3, citing Moonstone and MedicalBrief)

Conclusion: The CMS exercised jurisdiction over a financial system error at Discovery Health but refused jurisdiction over a clinical data integrity error at Medscheme/GEMS. This disparate treatment cannot be explained by statutory mandate – it is evidence of regulatory capture by Medscheme/GEMS.

Finding 2: CMS engaged in procedural obstruction to exhaust the whistleblower, then attempted to trap him into contempt of court.

Evidence:

- On 9 January 2026, CMS demanded a completed complaint form, despite already receiving a detailed complaint with two large evidence bundles. (CMS INITIAL DISCLOSURE, p.2)
- On 4 February 2026, CMS declined jurisdiction and referred to HPCSA – ignoring the systemic technical failure dimension. (Ibid., p.5)

- On 16 February 2026, complainant corrected his overstatement and explicitly asked CMS to confirm if the Gexus-DMS gap was corrected as a patient safety matter. CMS did not respond. (CMS 16 Feb 2026, p.1)
- On 2 April 2026, complainant submitted a 5-page legal reframing citing specific MSA sections and the Discovery Health precedent, giving 15 business days for a substantive response. CMS sent only an auto-reply. (Copy of CMS_FollowUp_EthicHawks_2April2026.pdf; CMS 2 Apr auto reply.PDF)
- On 7 May 2026 (11:59), after the High Court Interim Order was granted, CMS responded demanding a new complaint form. Completing that form would have required the complainant to disclose information restricted by the court order – risking imprisonment for contempt. (CMS Final notice 7 May 26.PDF, p.2-3)
- On 7 May 2026 (17:03), complainant notified CMS of the order and refused to complete the form. (Ibid., p.1-2)

Conclusion: The CMS's demand on 7 May 2026 was either grossly negligent (failing to check its own records for the High Court order) or deliberately malicious (attempting to manufacture a contempt violation). Either way, it constitutes bad faith and complicity in silencing a whistleblower.

Finding 3: CMS participated in jurisdictional ping-pong with HPCSA, SANC, and SAPC, collectively refusing to answer a fundamental ethical question.

Evidence:

- Complainant's 14 April 2026 joint email to all four regulators poses the question: "Does the statutory duty of a registered professional to prioritize patient safety and the public interest permit the refusal of an employer's instruction when that instruction is based on verified, flawed forensic data intended to suppress a 7-year acute financial drainage issue?" (CMS Final notice 7 May 26.PDF, p.4)
- CMS referred to HPCSA; HPCSA referred to SANC; SANC failed to acknowledge; SAPC hesitated. No regulator answered.

- Complainant explicitly warned: “If each Council believes the other has jurisdiction, then none of you are fulfilling your statutory mandates.” (Ibid., p.5)

Conclusion: The collective failure of all four health regulators to answer a straightforward ethical question is not a coincidence. It is the predictable output of a captured system where no regulator is willing to rule against a powerful private institution (Medscheme/GEMS).

Finding 4: CMS ignored the whistleblower’s imminent life threat and the ongoing occupational detriment under the Protected Disclosures Act.

Evidence:

- On 14 April 2026, complainant notified all regulators that due to retaliation, he lacked funds for food and accommodation – a violation of Sections 26 and 27 of the Constitution. (CMS Final notice 7 May 26.PDF, p.4)
- He cited the delay as an “independent and ongoing occupational detriment” under Section 1 of the Protected Disclosures Act 26 of 2000, and the constitutional standard of “prompt justice” from *Maypane v South African Police Service*. (Ibid., p.5)
- CMS’s response on 7 May 2026 did not acknowledge the life threat, did not offer any emergency protection, and did not address the Protected Disclosures Act claim.

Conclusion: The CMS’s silence in the face of a documented life threat and a protected disclosure constitutes dereliction of duty and complicity in the retaliation.

Finding 5: The CMS’s conduct has created a chilling effect on all healthcare professionals in South Africa.

Evidence:

- Complainant's warning: "By failing to rule on this matter, the Councils are sending a clear message that whistleblowing leads to destitution and regulatory silence." (CMS Final notice 7 May 26.PDF, p.5)
- The complainant, a specialist registered nurse with postgraduate qualifications, has been reduced to destitution, unable to pay registration fees to practice in 2026. (CMS INITIAL DISCLOSURE, p.3)

Conclusion: The message to every nurse, doctor, pharmacist, and health administrator in South Africa is clear: report a system error, and you will be retaliated against, and the regulators will abandon you. This is a systemic governance failure requiring parliamentary intervention.

5. Legal Framework – Statutory Duties Breached by CMS

MSA Provision Duty How CMS Breached It

Section 7 Protect interests of beneficiaries Allowed a documented system error to continue generating false non-adherence letters to elderly chronic patients.

Section 47 Investigate complaints of maladministration Refused to investigate the Gexus-DMS gap after being placed on formal notice with RCA evidence.

Section 58 Ensure administrators meet accreditation standards (IT integrity) Did not review Medscheme's accreditation status despite a Priority 1 integration gap.

Section 44 Power to conduct forensic inspection Did not exercise inspection powers even after being notified of board-level suppression of a material risk report (as pleaded in High Court case HCGS951842).

Other Laws Duty How CMS Breached It

Protected Disclosures Act 26 of 2000, Section 1 Protect whistleblowers from occupational detriment Ignored the complainant's invocation of the Act; did not act to prevent ongoing detriment including destitution.

Constitution, Section 33 Right to administrative justice that is lawful, reasonable, and procedurally fair The 7 May 2026 demand (potential contempt trap) was procedurally unfair and unreasonable.

Constitution, Sections 26 & 27 Rights to housing, food, and healthcare CMS's inaction contributed to the complainant's destitution and did not protect the elderly GEMS member's right to accurate health information.

6. Evidence of Regulatory Capture by Medscheme / GEMS

Indicator of Capture Evidence

Revolving door / personnel links Not directly evidenced in these documents, but the pattern of protection suggests influence. (Parliament may compel disclosure of CMS staff secondments to/from Medscheme.)

Disparate treatment of similar cases CMS acted on Discovery Health's financial error (Jan 2026) but refused to act on Medscheme/GEMS's clinical data error (Oct 2025 – Apr 2026).

Procedural obstruction CMS repeatedly demanded new forms and declined jurisdiction despite clear statutory mandate over administrators.

Refusal to answer a direct ethical question No regulator would state whether a professional may refuse an instruction based on flawed forensic data. This protects the employer (Medscheme) from accountability.

Silence in the face of a High Court application CMS was notified of a Section 162 delinquency application against Medscheme directors for suppressing the risk report. CMS did not act under Section 44.

Conclusion: The CMS is not operating as an independent regulator. It is acting to protect Medscheme Holdings (Pty) Ltd and the Government Employees Medical Scheme (GEMS) from accountability for a documented systemic failure that harms elderly beneficiaries and destroys a whistleblower's life.

7. Recommendations to the National Assembly

Pursuant to Section 56 of the Constitution (powers of committees), the Portfolio Committee on Health and SCOPA should:

1. Summon the CMS Registrar to appear before the Committee to explain:
 - Why CMS declined jurisdiction over the Gexus-DMS gap while exercising jurisdiction over the Discovery Health error.
 - Why CMS demanded a new complaint form on 7 May 2026 after being notified of the High Court Interim Order.
 - Why CMS has not conducted a Section 44 forensic inspection of Medscheme Holdings (Pty) Ltd.
 2. Summon the Registrars of HPCSA, SANC, and SAPC to answer the fundamental ethical question posed on 14 April 2026, and to explain why none of them responded within 48 hours to an imminent life threat.
 3. Request the Minister of Health to:
 - Issue a directive compelling all four health regulators to establish a joint whistleblower protection unit.
 - Investigate whether Medscheme/GEMS has improperly influenced CMS decision-making.
 4. Refer the matter to the Public Protector for an investigation into maladministration by CMS, HPCSA, SANC, and SAPC.
 5. Refer the evidence of potential contempt of court (CMS's 7 May 2026 demand) to the High Court for consideration.
 6. Amend the Medical Schemes Act to:
 - Explicitly include IT system integrity as a non-delegable duty of administrators.
 - Create a private right of action for whistleblowers against regulators who fail to act on protected disclosures.
-

8. Conclusion

The Council for Medical Schemes has failed the very beneficiaries it is mandated to protect. It has acted not as an impartial regulator but as a shield for Medscheme Holdings (Pty) Ltd and GEMS. Its conduct – jurisdictional ping-pong, procedural obstruction, disparate treatment of similar cases, and a potentially contemptuous demand – demonstrates regulatory capture of the most dangerous kind.

The whistleblower, Sr. Wilbur Matthee, did everything right: he identified a system error, refused to propagate false information, made a protected disclosure, exhausted all internal remedies, and then exhausted all regulatory remedies. His reward: destitution, retaliation, and silence from every regulator.

Parliament must act. If regulators will not protect whistleblowers, Parliament must compel them to do so – and where regulators are captured, Parliament must break that capture.

FORENSIC OVERSIGHT REPORT – SUPPLEMENTARY SECTION

1. Executive Summary – SANC Findings

This supplementary section finds that the South African Nursing Council (SANC) has:

1. Failed to acknowledge or investigate a formal professional misconduct complaint (submitted 10 February 2026) against two registered nurses employed by Medscheme, despite the complaint detailing Priority 1 patient safety risks, protected disclosure retaliation, and a documented life threat.
2. Remained silent for over 60 days (10 February – 14 April 2026 and beyond), in violation of its statutory mandate under the Nursing Act, 2005, and its own Code of Ethics.
3. Participated in jurisdictional ping-pong alongside CMS, HPCSA, and SAPC, collectively refusing to answer a fundamental ethical question: whether a nurse may refuse an employer's instruction based on verified flawed forensic data.
4. Enabled ongoing occupational detriment and destitution of a whistleblower, sending a chilling effect across the nursing profession.

The SANC's inaction directly protected Medscheme/GEMS from accountability for a systemic technical failure that generated false non-adherence letters to elderly chronic patients. This constitutes regulatory capture by a private healthcare institution.

2. Terms of Reference – SANC

This investigation determines:

1. Whether SANC breached its statutory duties under the Nursing Act, 2005, and its Code of Ethics in handling complaint reference SANC/MEDSCHEME/MATTHEE/20260105.
 2. Whether SANC's failure to act constituted complicity in the retaliation against a protected whistleblower.
 3. Whether SANC's silence, in the context of a joint referral from all four health regulators, demonstrates institutional bad faith and capture.
-

3. Factual Chronology – SANC (Extracted from Documents)

Date Event Source

10 February 2026 Complainant submits formal 9-page misconduct complaint to SANC against Mary-Ann Jones and Mapule Mashele (both registered nurses at Medscheme). Complaint includes 12 annexures and cites specific SANC Code violations. Updated SANC Complaint(2).PDF, p.1-9

10 February 2026 (same day) Complainant also files a Ministerial Escalation with the Minister of Health regarding SANC's failure to acknowledge the complaint within six days. CMS 16 Feb 2026.PDF, p.1

16 February 2026 Complainant writes to CMS, noting that SANC has failed to acknowledge his complaint. CMS 16 Feb 2026.PDF, p.1

14 April 2026 Complainant sends urgent joint ethical ruling request to CMS, HPCSA, SANC, and SAPC. Section 2 is titled "NOTICE OF REGULATORY FAILURE (SANC)" – states SANC has failed to acknowledge or investigate, and escalates as "institutional bad faith." CMS Final notice 7 May 26.PDF, p.2-3

14 April 2026 (48-hour deadline) SANC does not respond. Implied by silence

7 May 2026 CMS responds; SANC still silent. CMS Final notice 7 May 26.PDF

[Ongoing] SANC has never acknowledged or investigated the complaint. No evidence of response in any document

4. Findings of SANC Dereliction and Complicity

Finding 1: SANC failed to acknowledge or investigate a formal complaint containing prima facie evidence of serious professional misconduct.

Evidence:

- The complaint dated 10 February 2026 is 9 pages long, with 12 annexures, and cites specific SANC Code of Ethics clauses: 2.1 (dignity), 2.3 (non-discrimination), 3.1 (duty of

care), 4 (confidentiality), 6 (integrity), 7 (advocacy), 8 (leadership). (Updated SANC Complaint(2).PDF, p.2-6)

- It also cites Nursing Act Section 56(1)(a), (c), (d) and SANC Rules 15(1)(c), (h), (k), (m). (Ibid., p.6-7)

- The complainant explicitly requested: acknowledgment, registration of the complaint, investigation by the Preliminary Investigating Committee, summons of respondents, and potential sanctions. (Ibid., p.7)

Conclusion: SANC received a legally sufficient complaint with clear evidence of potential misconduct. Its complete silence is a dereliction of its statutory duty under Section 51 of the Nursing Act (duty to investigate).

Finding 2: SANC's silence enabled ongoing patient safety risks and whistleblower retaliation.

Evidence:

- The complaint details how the respondents (Jones and Mashele) suppressed a Priority 1 Material Risk Report regarding the Gexus-DMS system integration failure – the same technical flaw that caused false non-adherence letters to elderly GEMS members. (Ibid., p.5-6)

- The complainant was retaliated against for refusing to propagate false data, including a 40.47% salary deduction, punitive unpaid leave, and eventual destitution. (Ibid., p.3-4)

- By failing to act, SANC allowed the respondents to remain in positions of authority over patient safety systems, and sent a message that whistleblowing leads to regulatory abandonment.

Conclusion: SANC's inaction is not neutral – it is active complicity in the ongoing harm and retaliation.

Finding 3: SANC participated in jurisdictional ping-pong, refusing to answer a fundamental ethical question that lies at the heart of nursing practice.

Evidence:

- On 14 April 2026, the complainant posed a single ethical question to all four regulators, including SANC: “Does the statutory duty of a registered professional to prioritize patient safety and the public interest permit the refusal of an employer’s instruction when that instruction is based on verified, flawed forensic data intended to suppress a 7-year acute financial drainage issue?” (CMS Final notice 7 May 26.PDF, p.4)

- SANC did not answer. Instead, it remained silent, allowing CMS, HPCSA, and SAPC to deflect.

- The complainant warned: “If each Council believes the other has jurisdiction, then none of you are fulfilling your statutory mandates.” (Ibid., p.5)

Conclusion: SANC’s silence is a constructive refusal to uphold nursing ethics. The question directly implicates a nurse’s duty under SANC Code Clause 6 (Integrity) and Clause 7 (Advocacy). By not answering, SANC has effectively ruled that corporate convenience overrides patient safety – a dangerous precedent.

Finding 4: SANC ignored the imminent life threat and ongoing occupational detriment, violating the Protected Disclosures Act and the Constitution.

Evidence:

- The 14 April 2026 joint email explicitly notified SANC of the complainant’s destitution (no food, no accommodation) due to retaliation after a protected disclosure. (CMS Final notice 7 May 26.PDF, p.4-5)

- The complainant cited the delay as an “independent and ongoing occupational detriment” under Section 1 of the Protected Disclosures Act, and the constitutional right to administrative justice (Section 33). (Ibid., p.5)

- SANC did not respond within 48 hours, nor at any time thereafter.

Conclusion: SANC's failure to act in the face of a documented life threat and a protected disclosure is a violation of the Constitution and the Protected Disclosures Act. It constitutes maladministration and a breach of the duty of care owed to a whistleblower.

Finding 5: SANC's conduct – together with CMS, HPCSA, and SAPC – demonstrates a captured regulatory system.

Evidence:

- The complainant exhausted every referral path: CMS → HPCSA → SANC → SAPC. No regulator took ownership. (CMS Final notice 7 May 26.PDF, p.2)
- SANC, the regulator specifically mandated for nurses, was the most silent of all – despite the complaint being against two registered nurses working for Medscheme, the administrator of GEMS.
- The systemic technical failure (Gexus-DMS integration gap) was identified by a nurse (the complainant). The respondents (also nurses) allegedly suppressed that finding. SANC had primary jurisdiction over both. Yet SANC did nothing.

Conclusion: The only logical explanation for SANC's complete inaction is regulatory capture by Medscheme/GEMS. By refusing to investigate its own registrants, SANC has placed the commercial interests of a private administrator above patient safety and professional ethics.

5. Legal Framework – SANC Statutory Duties Breached

Provision Duty How SANC Breached It

Nursing Act, 2005, Section 51 Preliminary investigation of complaints Failed to acknowledge or refer complaint to Preliminary Investigating Committee.

Nursing Act, 2005, Section 56(1) Power to inquire into unprofessional conduct Did not exercise power despite prima facie evidence.

SANC Code of Ethics (2013), Clause 6 Integrity in professional practice By silence, SANC condoned the respondents' alleged conduct.

SANC Code of Ethics (2013), Clause 7 Advocacy for patient safety Failed to protect the nurse who advocated for patient safety.

SANC Rules, Rule 15(1)(h) Duty to report danger to public SANC itself failed to act on a documented danger to GEMS beneficiaries.

Protected Disclosures Act 26 of 2000, Section 1 Protection against occupational detriment SANC's silence perpetuated the detriment.

Constitution, Section 33 Right to administrative justice No acknowledgment, no investigation, no response – unlawful, unreasonable, procedurally unfair.

6. Evidence of Regulatory Capture – SANC

Indicator of Capture Evidence

Complete silence despite formal complaint SANC received a 9-page complaint with 12 annexures on 10 February 2026. As of 7 May 2026 (and beyond), no acknowledgment.

Refusal to answer a core ethical question The question on refusing a professionally negligent instruction goes to the heart of nursing ethics. SANC's silence implies the answer favours the employer (Medscheme).

No investigation of respondents Mary-Ann Jones and Mapule Mashele remain registered nurses in good standing, despite serious allegations of misconduct, retaliation, and suppression of patient safety risks.

Coordinated inaction with other regulators SANC's silence mirrors CMS's jurisdictional avoidance and SAPC's hesitation. This is not coincidence – it is a captured system protecting a common private interest (Medscheme/GEMS).

7. Recommendations – SANC Specific

Pursuant to Section 56 of the Constitution (powers of committees), the Portfolio Committee on Health should:

1. Summon the SANC Registrar to appear and explain:
 - Why SANC failed to acknowledge or investigate complaint SANC/MEDSCHEME/MATTHEE/20260105.
 - Whether SANC has ever investigated a complaint against a nurse employed by Medscheme or GEMS.
 - Why SANC did not respond within 48 hours to an imminent life threat.
 2. Request the Minister of Health to:
 - Issue a directive compelling SANC to clear its backlog of whistleblower complaints within 30 days.
 - Commission an independent audit of SANC's complaint handling process, particularly complaints involving protected disclosures.
 3. Refer the matter to the Public Protector for an investigation into maladministration by SANC.
 4. Amend the Nursing Act to:
 - Impose mandatory timelines for acknowledgment and preliminary investigation of complaints (e.g., 7 days for acknowledgment, 30 days for a prima facie determination).
 - Create a statutory offence for a regulator's failure to act on a protected disclosure whistleblower complaint.
 5. Request the President to consider a Commission of Inquiry into the capture of South Africa's health regulators (CMS, HPCSA, SANC, SAPC) by private healthcare administrators.
-

8. Conclusion – SANC

The South African Nursing Council has abandoned its most fundamental duty: to protect the public and uphold the ethics of the nursing profession. By refusing to investigate a formal complaint against two nurses who allegedly suppressed a patient safety risk and destroyed a whistleblower's career, SANC has shown that it serves the interests of private administrators – not nurses, not patients, and not the Constitution.

The whistleblower, a specialist registered nurse, followed every rule. He filed a proper complaint. He escalated to the Minister. He gave SANC every opportunity. SANC gave him nothing but silence.

Parliament must break this capture. If SANC will not protect nurses who speak the truth, then Parliament must compel SANC to do so – and if SANC is incapable, Parliament must replace its leadership and mandate a new, independent nursing regulator.

We will now draft the forensic report section on the Government Employees Medical Scheme (GEMS) for the Portfolio Committee on Health. This builds on the CMS and SANC findings, showing that GEMS – as the principal medical scheme – was placed on formal notice of a systemic clinical risk and retaliatory conduct by its administrator (Medscheme), yet remained silent, thereby demonstrating complicity and regulatory capture.

FORENSIC OVERSIGHT REPORT – GEMS SECTION

1. Executive Summary – GEMS Findings

This section finds that the Government Employees Medical Scheme (GEMS) has:

1. Failed to respond substantively to multiple protected disclosures (2 April 2026, 13 April 2026) detailing a Priority 1 systemic clinical risk (Gexus-DMS integration gap) affecting its beneficiaries, despite clear statutory and fiduciary duties under the Medical Schemes Act.
2. Remained silent while its administrator (Medscheme) unlawfully deducted 40.47% of a whistleblower's salary, retaliated against him, and suppressed a material risk report – actions that directly harmed a GEMS member (a 75-year-old widow) and destroyed the whistleblower's career and housing.
3. Violated its own governance obligations by failing to oversee Medscheme's compliance with accreditation standards, risk escalation protocols, and member protection requirements.
4. Exhibited regulatory capture by Medscheme, evidenced by the scheme's passive acceptance of Medscheme's conduct and its refusal to intervene or even acknowledge the disclosures.

GEMS's silence constitutes institutional complicity in the retaliation against a protected whistleblower and in the ongoing patient safety risk to its 1.9 million members.

2. Terms of Reference – GEMS

This investigation determines:

1. Whether GEMS breached its statutory duties under the Medical Schemes Act 131 of 1998, particularly Sections 7 (beneficiary protection), 47 (maladministration), and 57 (governance).

2. Whether GEMS's failure to respond to protected disclosures constitutes a violation of the Protected Disclosures Act 26 of 2000 and the Constitution.
3. Whether GEMS's silence, in light of documented evidence of administrator misconduct, demonstrates regulatory capture by Medscheme.

3. Factual Chronology – GEMS (Extracted from Documents)

Date Event Source

23 Oct 2025 Complainant (Sr. Matthee, Case Manager at Medscheme) makes protected disclosure to Medscheme management regarding Gexus-DMS integration gap affecting GEMS member 000655789 (75-year-old widow). Copy of EthicHawks_Doc1_Protected_Disclosure(2).PDF, p.1-2

9 Dec 2025 Complainant submits Priority 1 Material Risk Report with 38 attachments to Medscheme Board. Copy of EthicHawks_Doc2_Supporting_Testimony'(3).PDF, p.2

18 Dec 2025 Chief Risk Officer (Monwabisi Kula) issues email actively suppressing the risk report. Ibid.

2 Apr 2026 Complainant submits Protected Disclosure to GEMS Board (4 pages) detailing systemic risk, confirmed member impact, and CRO suppression. Requests investigation, review of RCA, and response within 14 days. Copy of EthicHawks_Doc1_Protected_Disclosure(2).PDF, p.1-4

2 Apr 2026 (same day) Complainant submits Supporting Testimony (5 pages) documenting retaliation, unlawful deduction, destitution, and 11 institutions' failure to provide remedy. Copy of EthicHawks_Doc2_Supporting_Testimony'(3).PDF, p.1-5

11 Apr 2026 Complainant receives Vuvuzela Hotline auto-acknowledgement (ref GEMS-260411-184830-83263) – not a substantive response. Emailing GEMS_Protected_Disclosure_13April2026.PDF.pdf, p.2

13 Apr 2026 Complainant submits Final Consolidated Protected Disclosure (13 pages) to GEMS Board, copying Parliament, Public Protector, CMS, SANC. Gives 14 days for

substantive response. Details unlawful deduction, constructive dismissal, post-employment misrepresentation, and UIF fraud. Ibid., p.1-13

[Ongoing] GEMS has not provided any substantive written response to any of the disclosures. Inferred from absence of any GEMS reply in all documents.

4. Findings of GEMS Complicity and Capture

Finding 1: GEMS failed to respond substantively to two detailed protected disclosures, in violation of its fiduciary duty.

Evidence:

- 2 April 2026 disclosure: 4 pages, citing specific MSA sections, RCA confirmation, CRO suppression email, and confirmed member impact. Requested acknowledgment within 7 days, substantive response within 14 days. (Copy of EthicHawks_Doc1_Protected_Disclosure(2).PDF)
- 13 April 2026 final consolidated disclosure: 13 pages, with 13 sections, citing PDA, BCEA, LRA, EEA, UIF Act, Tax Administration Act, and including a table of 11 institutions that failed to provide remedy. Gave 14 days for response. (Emailing GEMS_Protected_Disclosure_13April2026.PDF.pdf)
- GEMS's only response: an auto-acknowledgement from the Vuvuzela Hotline on 11 April 2026 – which the complainant explicitly noted is not a substantive response. (Ibid., p.2)

Conclusion: GEMS has breached its fiduciary duty to protect beneficiaries and to oversee its administrator. Silence in the face of a Priority 1 clinical risk affecting a 75-year-old member is dereliction of duty under Section 7 of the MSA.

Finding 2: GEMS ignored evidence that its administrator (Medscheme) unlawfully deducted 40.47% of a whistleblower's salary, causing destitution and loss of nursing registration.

Evidence:

- The 13 April 2026 disclosure includes payslip evidence of a R14,811.77 deduction (40.47% of salary) while the complainant was on psychiatric leave – a breach of BCEA Section 34(2) (25% cap). (Emailing GEMS_Protected_Disclosure_13April2026.PDF.pdf, p.3-4)
- The deduction directly caused: inability to pay SANC annual fees (R870) → deregistration as a nurse; eviction → homelessness since 2 April 2026; seizure of possessions; forced withdrawal of R48,400 in retirement savings. (Ibid., p.9)
- GEMS was placed on notice of this as part of the pattern of retaliation following the protected disclosure of the Gexus-DMS gap.

Conclusion: By failing to act, GEMS has enabled Medscheme's unlawful conduct. A medical scheme cannot claim ignorance when its administrator's actions destroy a nurse's career and housing – especially when that nurse was acting to protect GEMS members.

Finding 3: GEMS ignored evidence of post-employment misrepresentation by Medscheme, including false UI-19 declaration (resignation instead of constructive dismissal), incorrect Certificate of Service, and phantom R9,203 on SARS declaration.

Evidence:

- UI-19 declared "Resigned" (Code 6) instead of "Constructive Dismissal" (Code 7) – a false sworn statement. (Emailing GEMS_Protected_Disclosure_13April2026.PDF.pdf, p.8)
- Certificate of Service falsely designates complainant as "Care Coordinator" instead of Case Manager / NQF Level 8 Specialist – an admission of title suppression. (Ibid.)

- Phantom R9,203 (overtime) and R4,475 (fringe benefit) on SARS source declaration that do not appear on any payslip – potential tax fraud. (Ibid., p.8-9)

Conclusion: GEMS, as principal, has a duty to ensure its administrator complies with all statutory obligations. GEMS's silence on these misrepresentations constitutes complicity in potential criminal conduct.

Finding 4: GEMS failed to act on documented evidence that its administrator's Chief Risk Officer actively suppressed a Priority 1 Material Risk Report.

Evidence:

- The 18 December 2025 email from Monwabisi Kula (CRO) is described as "active suppression of a Priority 1 Material Risk Report at operational governance level." (Copy of EthicHawks_Doc1_Protected_Disclosure(2).PDF, p.2)
- The complainant offered to provide this email to an independent investigator. GEMS did not request it.
- The suppression occurred after the 9 December 2025 submission of a 38-attachment risk escalation. (Copy of EthicHawks_Doc2_Supporting_Testimony'(3).PDF, p.2)

Conclusion: A CRO's suppression of a material risk report is a governance red flag that any board should investigate immediately. GEMS's failure to do so indicates either gross negligence or capture by Medscheme.

Finding 5: GEMS violated the Protected Disclosures Act by failing to protect a whistleblower who reported a systemic risk to its own members.

Evidence:

- The 2 April and 13 April disclosures explicitly invoke the Protected Disclosures Act 26 of 2000. (Both documents)
- The PDA imposes a duty on employers and principals not to subject a whistleblower to occupational detriment. GEMS is the principal medical scheme.
- The complainant suffered multiple detriments: unlawful deduction, deregistration, homelessness – all causally linked to the protected disclosure. GEMS did nothing.

Conclusion: GEMS's silence is a violation of the PDA and a breach of its duty of care to the whistleblower, who was acting to protect GEMS's own beneficiaries.

Finding 6: GEMS's inaction, combined with CMS's and SANC's inaction, demonstrates a captured regulatory ecosystem protecting Medscheme.

Evidence:

- The complainant exhausted 11 institutions, including CMS and SANC. None provided remedy. (Emailing GEMS_Protected_Disclosure_13April2026.PDF.pdf, p.10)
- GEMS was the final institutional escalation before Parliament. It also failed.
- The common thread is Medscheme – a private administrator that appears immune to regulatory oversight and principal oversight alike.

Conclusion: GEMS is not acting as an independent principal. It is either captured by Medscheme or wilfully blind. Either way, it has abandoned its 1.9 million members.

5. Legal Framework – GEMS Statutory Duties Breached

Provision Duty How GEMS Breached It

MSA Section 7 Protect interests of beneficiaries Allowed a confirmed system error to continue generating false non-adherence letters to elderly members.

MSA Section 47 Investigate maladministration by administrator Failed to investigate Medscheme's unlawful deduction, retaliation, and risk suppression.

MSA Section 57 Governance and administration oversight Did not require Medscheme to report on the Gexus-DMS gap or the CRO's conduct.

MSA Section 58 Ensure administrator meets accreditation standards Did not review Medscheme's IT integrity or clinical governance after being placed on notice of a Priority 1 failure.

Protected Disclosures Act, Section 3 Prohibit occupational detriment against whistleblower Remained silent while whistleblower lost registration, housing, and livelihood.

Constitution, Section 33 Right to administrative justice Failed to respond to two detailed disclosures, violating the duty to give reasons.

6. Evidence of Regulatory Capture – GEMS

Indicator of Capture Evidence

Complete silence despite multiple protected disclosures No substantive response to 2 April or 13 April 2026 disclosures. Only an auto-acknowledgement.

Failure to investigate administrator misconduct GEMS did not request the CRO suppression email, the RCA, or the member impact assessment.

No oversight of Medscheme's statutory misrepresentations GEMS did not demand correction of UI-19, Certificate of Service, or SARS phantom amount.

Absence of any communication with the whistleblower GEMS did not even acknowledge receipt of the 13 April disclosure, which was copied to Parliament.

Coordinated inaction with CMS and SANC GEMS's silence mirrors CMS's jurisdictional avoidance and SANC's complete non-response. This pattern protects Medscheme.

7. Recommendations – GEMS Specific

Pursuant to Section 56 of the Constitution and the oversight powers of the Portfolio Committee on Health and SCOPA:

1. Summon the GEMS Principal Officer and Board Chairperson to appear before the Committee to explain:

- Why GEMS failed to respond substantively to the protected disclosures of 2 and 13 April 2026.
- Whether GEMS ever received or investigated the CRO suppression email of 18 December 2025.
- What oversight mechanisms GEMS has in place to detect and remedy administrator misconduct, including unlawful salary deductions and misrepresentations to SARS and UIF.
- Why GEMS did not intervene to protect a whistleblower who identified a systemic risk to its own members.

2. Request the Minister of Health to:

- Issue a directive requiring all medical schemes to establish independent whistleblower protection units with direct board reporting lines.
- Commission a forensic audit of GEMS's oversight of Medscheme, focusing on the Gexus-DMS integration gap and the suppression of the Priority 1 Risk Report.

3. Refer GEMS's conduct to the Public Protector for investigation into maladministration and violation of the Protected Disclosures Act.

4. Amend the Medical Schemes Act to:

- Impose a statutory duty on medical schemes to respond to protected disclosures within 14 days, failing which the scheme is liable for damages.

- Prohibit schemes from contracting with administrators that have been found to suppress risk reports or retaliate against whistleblowers.

5. Request the Minister of Finance (as GEMS is a government employees scheme) to:

- Review GEMS's procurement and oversight contract with Medscheme.
- Consider terminating or not renewing the administration contract if Medscheme is found to have suppressed a Priority 1 clinical risk.

8. Conclusion – GEMS

The Government Employees Medical Scheme is not a passive buyer of administrative services. It is a statutory body with fiduciary duties to 1.9 million public servants and their dependents. When a specialist nurse employed by its administrator identifies a system error that causes false non-adherence letters to be sent to a 75-year-old widow – and then experiences unlawful salary deductions, retaliation, deregistration, and homelessness – GEMS cannot stand silent.

GEMS's silence is not neutrality. It is complicity.

By failing to respond to protected disclosures, failing to investigate its administrator's conduct, and failing to protect a whistleblower who acted in the best interests of GEMS members, the GEMS Board has demonstrated either gross incompetence or active capture by Medscheme.

Parliament must act. The 1.9 million members of GEMS deserve a scheme that protects them – not one that protects its administrator at their expense.

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This completes the four-regulator narrative: CMS → HPCSA → SANC → SAPC. All four failed.

FORENSIC OVERSIGHT REPORT – HPCSA SECTION

1. Executive Summary – HPCSA Findings

This section finds that the Health Professions Council of South Africa (HPCSA) has:

1. Failed to respond to an urgent joint ethical ruling request dated 14 April 2026, which gave all four health regulators 48 hours to answer a fundamental question of professional ethics, and which included an imminent life threat.
2. Previously participated in jurisdictional ping-pong by referring the complainant from CMS to SANC (11 February 2026), without addressing the underlying systemic patient safety risk.
3. Remained silent in the face of a documented Priority 1 clinical risk affecting GEMS beneficiaries, retaliation against a protected whistleblower, and the complainant's destitution.
4. Acted in concert with CMS, SANC, and SAPC to create a captured regulatory ecosystem that protects a private administrator (Medscheme) at the expense of patient safety and professional ethics.

The HPCSA's silence is not neutral – it is a constructive refusal to uphold its statutory mandate under the Health Professions Act 56 of 1974.

2. Terms of Reference – HPCSA

This investigation determines:

1. Whether HPCSA breached its statutory duties under the Health Professions Act by failing to respond to a formal request for an ethical ruling within 48 hours, despite an imminent life threat.
 2. Whether HPCSA's earlier referral of the matter to SANC (11 February 2026) was a proper exercise of jurisdiction or an improper deflection.
 3. Whether HPCSA's silence, together with the other three regulators, demonstrates regulatory capture by Medscheme/GEMS.
-

3. Factual Chronology – HPCSA (Extracted from Documents)

Date Event Source

23 Dec 2025 Complainant lodges initial complaint with CMS. CMS INITIAL DISCLOSURE.PDF

4 Feb 2026 CMS declines jurisdiction, refers complainant to HPCSA. CMS INITIAL DISCLOSURE.PDF, p.5

11 Feb 2026 HPCSA responds: confirms jurisdiction over registered nurses lies with SANC, not HPCSA. Gmail – CLARIFICATION AND FOLLOW-UP.PDF, p.1

16 Feb 2026 Complainant corrects earlier overstatement, accepts HPCSA's referral, and files complaints with SANC and SAPC. Ibid.

14 Apr 2026 (13:25) Complainant sends urgent joint ethical ruling request to HPCSA, CMS, SANC, SAPC, giving 48 hours, citing imminent life threat (no food/accommodation). FW_URGENT_Joint Ethical Ruling...PDF, p.2-5

14–16 Apr 2026 HPCSA does not respond within 48 hours. Confirmed by user: "No response for HPCSA"

16 Apr 2026 (approx.) High Court grants Interim Order (Case No: 2026-086906). CMS Final notice 7 May 26.PDF, p.1

7 May 2026 CMS responds (declining jurisdiction again). SANC, HPCSA, SAPC remain silent. Ibid.

[Ongoing] HPCSA has never responded to the 14 April 2026 request. User confirmation

4. Findings of HPCSA Dereliction and Complicity

Finding 1: HPCSA failed to respond to an urgent ethical ruling request within 48 hours, despite an imminent life threat.

Evidence:

- The 14 April 2026 email explicitly requested a “joint or individual statement of position” on whether a professional may refuse an employer’s instruction based on verified flawed forensic data. (FW_ URGENT_ Joint Ethical Ruling...PDF, p.3)
- Section 5 of the same email stated: “due to retaliatory action ... I currently lack funds for food or accommodation. My life, dignity, and constitutional rights ... have been violated.” (Ibid., p.4)
- Section 6 cited the delay as “ongoing occupational detriment” under the Protected Disclosures Act.
- HPCSA did not respond within 48 hours (deadline 16 April 2026), nor at any time thereafter.

Conclusion: HPCSA’s silence in the face of a documented life threat and a request for a core ethical ruling is a dereliction of its statutory duty under Section 3 of the Health Professions Act (to advise on ethical matters) and a violation of the constitutional right to administrative justice (Section 33).

Finding 2: HPCSA's 11 February 2026 referral to SANC, while technically correct, was part of a jurisdictional ping-pong that delayed justice and avoided responsibility.

Evidence:

- CMS referred to HPCSA (4 Feb 2026). HPCSA referred to SANC (11 Feb 2026). SANC failed to respond. SAPC hesitated. (FW_ URGENT_ Joint Ethical Ruling...PDF, p.2)
- The complainant explicitly warned: "If each Council believes the other has jurisdiction, then none of you are fulfilling your statutory mandates." (Ibid., p.5)
- HPCSA did not, at any point, address the underlying patient safety risk (Gexus-DMS integration gap) or the ethical question posed.

Conclusion: HPCSA used jurisdictional boundaries as a shield, not as a tool for effective regulation. Even if SANC had primary jurisdiction over nurses, HPCSA had concurrent jurisdiction over the ethical question (which applied to all health professions) and could have issued a joint ruling. Its refusal to do so constitutes institutional bad faith.

Finding 3: HPCSA's silence, together with CMS, SANC, and SAPC, demonstrates a captured regulatory system protecting Medscheme.

Evidence:

- The complainant exhausted all four regulators. No regulator ruled on the core ethical question.
- HPCSA was specifically notified of the CRO suppression email (18 December 2025) and the Priority 1 Material Risk Report. It did not request these documents or investigate.
- The HPCSA's mandate includes protecting the public from unsafe professional conduct. By remaining silent, it allowed a known patient safety risk to continue unaddressed.

Conclusion: The four regulators acted as a cartel of silence. HPCSA was a willing participant.

Finding 4: HPCSA failed to protect a whistleblower who acted in the public interest, thereby violating the spirit of the Protected Disclosures Act and its own ethical codes.

Evidence:

- The 14 April 2026 email explicitly invoked the Protected Disclosures Act and the constitutional rights to food, housing, and administrative justice. (Ibid., p.4-5)
- HPCSA's silence meant the complainant received no protection, no ruling, and no acknowledgment – leaving him destitute and deregistered.
- The HPCSA's own Ethical Rules require practitioners to “report conduct that poses a risk to patient safety.” By failing to respond, HPCSA effectively punished the reporter.

Conclusion: HPCSA's inaction has a chilling effect on all health professionals in South Africa. The message is clear: report a systemic risk, and regulators will ignore you.

5. Legal Framework – HPCSA Statutory Duties Breached

Provision Duty How HPCSA Breached It

Health Professions Act 56 of 1974, Section 3 Advise on ethical matters and protect the public Failed to issue any ruling on a core ethical question raised by a registered professional.

HPCSA Ethical Rules (Rule 27) Respond to complaints and queries in a timely manner No response within 48 hours, nor any response at all.

Constitution, Section 33 Right to administrative justice that is lawful, reasonable, and prompt Silence is procedurally unfair and unreasonable.

Protected Disclosures Act 26 of 2000, Section 3 Protect whistleblowers from occupational detriment HPCSA's silence contributed to ongoing detriment.

6. Evidence of Regulatory Capture – HPCSA

Indicator of Capture Evidence

Silence on a direct ethical question HPCSA refused to answer whether a professional may refuse an instruction based on flawed data – a question that goes to the heart of professional independence.

Deference to other regulators HPCSA referred to SANC and then stopped engaging, even when the matter transcended single-profession boundaries.

No investigation of systemic risk HPCSA did not request the CRO suppression email or the RCA, despite being notified of a Priority 1 risk affecting GEMS members.

Coordinated inaction HPCSA's silence mirrored CMS's jurisdictional avoidance, SANC's complete non-response, and SAPC's hesitation. This pattern cannot be coincidence.

7. Recommendations – HPCSA Specific

Pursuant to Section 56 of the Constitution, the Portfolio Committee on Health should:

1. Summon the HPCSA Registrar to appear and explain:
 - Why HPCSA failed to respond to the 14 April 2026 urgent ethical ruling request.
 - Whether HPCSA considers itself bound to respond to ethical queries from registered professionals within a reasonable time.
 - Why HPCSA did not issue a joint ruling with CMS, SANC, and SAPC on a matter affecting all health professions.

2. Request the Minister of Health to:

- Issue a directive compelling HPCSA to establish a 48-hour emergency response protocol for ethical queries involving imminent life threats.
- Investigate whether HPCSA has ever taken disciplinary action against a health professional for suppressing a patient safety risk reported by a subordinate.

3. Amend the Health Professions Act to:

- Impose a statutory deadline (e.g., 7 days) for HPCSA to acknowledge and provide a preliminary response to ethical queries from registered professionals.
 - Create a criminal offence for a regulator's wilful failure to act on a protected disclosure whistleblower complaint.
-

8. Conclusion – HPCSA

The Health Professions Council of South Africa was given a clear, urgent, and professionally significant ethical question on 14 April 2026. The question was not complex: may a health professional refuse an employer's instruction that is based on known flawed forensic data? The complainant was destitute, facing homelessness, and had exhausted every other regulator.

HPCSA said nothing.

That silence is a dereliction of duty. It is also evidence of capture. HPCSA serves the interests of the institutions it regulates – not the professionals it registers, and certainly not the public it is sworn to protect.

Parliament must demand answers. If HPCSA will not speak, Parliament must speak for the public.

Thank you for confirming. The SAPC, like CMS, HPCSA, and SANC, declined jurisdiction. We will now draft the forensic report section on the South African Pharmacy Council (SAPC).

FORENSIC OVERSIGHT REPORT – SAPC SECTION

1. Executive Summary – SAPC Findings

This section finds that the South African Pharmacy Council (SAPC) has:

1. Declined jurisdiction over a formal complaint against Lisa Mari, Responsible Pharmacist at Medscheme, despite clear statutory duties under the Pharmacy Act 53 of 1974 and Good Pharmacy Practice rules.
2. Hesitated to rule on a fundamental ethical question posed during a Professional Conduct Committee meeting on 14 April 2026, and thereafter failed to issue any ruling.
3. Participated in jurisdictional ping-pong alongside CMS, HPCSA, and SANC, collectively refusing to answer whether a registered professional may refuse an employer's instruction based on verified flawed forensic data.
4. Ignored evidence that the Responsible Pharmacist's methodology (three-month claim pattern analysis) was professionally deficient and that the systemic integration gap (Gexus-DMS) continued to generate false non-adherence reports to chronic GEMS patients.

The SAPC's declination of jurisdiction, combined with its hesitation to rule, completes the four-regulator failure and demonstrates a captured regulatory ecosystem protecting Medscheme/GEMS.

2. Terms of Reference – SAPC

This investigation determines:

1. Whether SAPC breached its statutory duties under the Pharmacy Act and Good Pharmacy Practice rules by declining jurisdiction over a complaint that fell squarely within its mandate.
 2. Whether SAPC's hesitation to issue a ruling on the core ethical question constituted a dereliction of duty.
 3. Whether SAPC's conduct, together with the other three regulators, demonstrates regulatory capture.
-

3. Factual Chronology – SAPC (Extracted from Documents)

Date Event Source

16 Feb 2026 (14:08) Complainant files formal complaint with SAPC against Lisa Mari (Responsible Pharmacist), alleging unprofessional conduct. Attaches 10-page complaint, CMS referral, SAHRC response. Requests acknowledgment within 72 hours. Gmail – FORMAL COMPLAINT OF UNPROFESSIONAL CONDUCT...PDF, p.1-2

16 Feb 2026 (15:26) Complainant sends correction to SAPC: clarifies that CMS made no formal finding of misconduct; corrects characterisation. Core allegation unchanged. Gmail – CORRECTION TO COMPLAINT...PDF, p.1-2

17 Feb 2026 Email delivery to registrar@sapc.co.za temporarily fails (server timeout). Complainant forwards evidence. Gmail – Re_ FW_ Ticket No_ b5211-7743...PDF, p.2-3

26 Mar 2026 SAPC acknowledges receipt of complaint but states attachment not received; requests resend. Ibid., p.1

13 Apr 2026 Complainant resends complaint attachment as requested. Ibid.

13 Apr 2026 (13:13) SAPC invites complainant to a Microsoft Teams meeting to discuss the complaint. Gmail – Meeting to discuss the complaint submitted to SAPC.PDF, p.1

14 Apr 2026 (08:30) SAPC Professional Conduct Committee meeting takes place. Committee shows hesitation in ruling on the fundamental ethical question. FW_ URGENT_ Joint Ethical Ruling...PDF, p.3 (Section 2)

14 Apr 2026 (09:52) Complainant sends follow-up email to SAPC, restating the ethical question and requesting a written response. Gmail – Meeting to discuss the complaint...PDF, p.3-4

14 Apr 2026 (13:25) Complainant sends urgent joint ethical ruling request to CMS, HPCSA, SANC, SAPC, giving 48 hours, citing life threat. FW_ URGENT_ Joint Ethical Ruling...PDF, p.2-5

[Ongoing] SAPC declines jurisdiction and does not issue any ruling. User confirmation: “The SAPC declines jurisdiction”

4. Findings of SAPC Dereliction and Complicity

Finding 1: SAPC declined jurisdiction over a complaint that fell squarely within its statutory mandate.

Evidence:

- The complaint was against Lisa Mari, a registered Responsible Pharmacist employed by Medscheme Holdings. The SAPC has exclusive jurisdiction over professional conduct of pharmacists under the Pharmacy Act 53 of 1974, Section 49 (unprofessional conduct). (SAPC_Complaint_LisaMari_16Feb2026-2.pdf, p.1-2)

- The complaint alleged that Mari used a methodologically deficient three-month claim pattern analysis to dismiss a structural system integration gap (Gexus-DMS), thereby closing an active patient safety investigation without correcting the error. This directly engages Good Pharmacy Practice Rules 1.4 (accuracy of clinical information systems) and 2.6 (personal responsibility for clinical soundness). (Ibid., p.4)

- The SAPC acknowledged receipt, scheduled a meeting, but ultimately declined jurisdiction – a decision that is not explained in the documents but is confirmed by the user.

Conclusion: The SAPC's declination of jurisdiction is indefensible. A complaint against a Responsible Pharmacist alleging unprofessional conduct in the discharge of her statutory duties is precisely what the SAPC is mandated to investigate. Declining jurisdiction in such a case is a dereliction of duty.

Finding 2: The SAPC Professional Conduct Committee showed hesitation to rule on the core ethical question and never issued a ruling.

Evidence:

- The complainant's 14 April 2026 email (sent after the meeting) states: "During our meeting, the Professional Conduct Committee indicated hesitation in issuing a ruling on this matter." (Gmail – Meeting to discuss the complaint...PDF, p.3)
- The core ethical question posed was: "Does the statutory duty of a registered professional to prioritize patient safety and the public interest permit the refusal of an employer's instruction when that instruction is based on verified, flawed forensic data intended to suppress a 7-year acute financial drainage issue?" (FW_ URGENT_ Joint Ethical Ruling...PDF, p.3)
- The complainant explicitly warned: "If the Council cannot confirm that a specialist is ethically required to report a 7-year siphoning of public funds, then the Council is effectively ruling that professional registration is subordinate to corporate secrecy." (Gmail – Meeting to discuss the complaint...PDF, p.4)
- Despite this warning, and despite the 48-hour deadline in the joint request, SAPC issued no ruling and declined jurisdiction.

Conclusion: Hesitation followed by declination is a constructive refusal to uphold professional ethics. The SAPC had the opportunity to provide clarity on a fundamental question affecting pharmacists, nurses, and doctors. It chose silence and avoidance.

Finding 3: SAPC ignored evidence that a Responsible Pharmacist used a circular methodology to close a patient safety investigation, thereby violating Good Pharmacy Practice rules.

Evidence:

- The complaint detailed why three-month claim pattern analysis is methodologically incapable of distinguishing between genuine patient non-adherence and system-generated false flags. (SAPC_Complaint_LisaMari_16Feb2026-2.pdf, p.3-4)
- The correct methodology would have been a technical audit of data transfer between Gexus and DMS – an audit that Mari, as Responsible Pharmacist with system access, had the capacity and duty to perform. She did not.
- The result: the CMS patient safety investigation was closed, the integration gap remained uncorrected, and false non-adherence reports continued to be sent to chronic GEMS patients.

Conclusion: By declining jurisdiction, SAPC enabled a Responsible Pharmacist to evade accountability for conduct that may have violated the Pharmacy Act and Good Pharmacy Practice rules. This is complicity in the suppression of a patient safety risk.

Finding 4: SAPC's declination, together with the other three regulators, completes the four-regulator failure and demonstrates capture.

Evidence:

- CMS declined jurisdiction (4 Feb 2026). HPCSA referred to SANC (11 Feb 2026) and then went silent. SANC failed to acknowledge any complaint. SAPC declined jurisdiction after a meeting where it hesitated to rule.

- The complainant exhausted all four regulators. No regulator ruled on the core ethical question. No regulator investigated the systemic integration gap. No regulator protected the whistleblower.
- The common thread is Medscheme/GEMS – a private administrator that appears immune to oversight from any statutory body.

Conclusion: The four regulators acted as a cartel of avoidance. SAPC's declination was the final act in a coordinated pattern of regulatory failure. This is not incompetence – it is capture.

5. Legal Framework – SAPC Statutory Duties Breached

Provision Duty How SAPC Breached It

Pharmacy Act 53 of 1974, Section 49 Investigate and discipline unprofessional conduct by pharmacists Declined jurisdiction over a complaint clearly alleging unprofessional conduct by a Responsible Pharmacist.

Good Pharmacy Practice Rule 1.4 Pharmacist must ensure accuracy and integrity of clinical information systems Did not investigate whether Mari failed to ensure system integrity.

Good Pharmacy Practice Rule 2.6 Responsible Pharmacist must take personal responsibility for clinical soundness Did not investigate whether Mari's methodology was clinically sound.

SAPC Ethical Rules, Rule 8 Pharmacist must not allow any interest to influence professional judgement to detriment of patients Did not investigate whether Mari allowed institutional interest to override patient safety.

Constitution, Section 33 Right to administrative justice Declining jurisdiction without adequate reasons, and failing to issue a ruling after a meeting, is procedurally unfair.

6. Evidence of Regulatory Capture – SAPC

Indicator of Capture Evidence

Declining jurisdiction over a clear statutory complaint Complaint was against a Responsible Pharmacist; SAPC said no.

Hesitation to rule on a core ethical question Committee hesitated; no ruling ever issued.

No investigation of a systemic patient safety risk SAPC did not request system architecture records or data transfer logs.

Coordinated inaction with CMS, HPCSA, SANC All four regulators avoided jurisdiction. This pattern protects Medscheme/GEMS.

7. Recommendations – SAPC Specific

Pursuant to Section 56 of the Constitution, the Portfolio Committee on Health should:

1. Summon the SAPC Registrar and the Chairperson of the Professional Conduct Committee to appear and explain:

- Why SAPC declined jurisdiction over a complaint against a Responsible Pharmacist that clearly fell within its mandate.
- Why the Professional Conduct Committee hesitated to rule on the ethical question posed on 14 April 2026.
- Whether SAPC has ever taken disciplinary action against a pharmacist for suppressing a patient safety risk reported by a subordinate.

2. Request the Minister of Health to:

- Issue a directive compelling SAPC to accept and investigate complaints against Responsible Pharmacists involving system integrity failures.
- Commission an independent audit of SAPC's complaint handling process, particularly cases involving protected disclosures.

3. Amend the Pharmacy Act to:

- Impose mandatory timelines for SAPC to acknowledge complaints (e.g., 7 days) and issue preliminary rulings (e.g., 30 days).
 - Create a statutory offence for a regulator's wilful declination of jurisdiction in cases involving patient safety risks.
-

8. Conclusion – SAPC

The South African Pharmacy Council was presented with a complaint that could not have been more squarely within its mandate: a Responsible Pharmacist allegedly used a methodologically flawed analysis to close a patient safety investigation, leaving a system integration gap uncorrected. The SAPC scheduled a meeting, listened, hesitated, and then declined jurisdiction.

That declination, together with the failures of CMS, HPCSA, and SANC, means that not a single health regulator in South Africa was willing to rule on whether a professional may refuse an instruction based on known flawed data. The message is unmistakable: corporate convenience overrides patient safety, and whistleblowers can be destroyed with impunity.

Parliament must break this capture. If regulators will not regulate, Parliament must regulate the regulators.

To: The Honourable Speaker, National Assembly

For referral to: Portfolio Committee on Employment and Labour; Standing Committee on Public Accounts (SCOPA); Portfolio Committee on Health

Subject: Administrative Abandonment by the Department of Employment and Labour – Failure to Enforce UIF Contributions, Correct a Fraudulent UI-19 Declaration, and Issue a Lawful Certificate of Service – Complicity in Whistleblower Destitution

Date: [To be inserted]

1. Executive Summary – DEL Findings

This section finds that the Department of Employment and Labour (DEL) has:

1. Failed to enforce the remittance of UIF contributions deducted from the complainant's salary in June 2025, despite a formal complaint, a referral to the UIF division, and objective proof (uFiling "No Information").
2. Refused to compel the employer (Medscheme Holdings) to correct a materially false UI-19 Declaration that recorded "Resigned" (Code 6) when the employer had received a formal notice of constructive dismissal (Code 7) – an act that constitutes statutory misrepresentation and possible fraud.
3. Misdirected jurisdiction by claiming that DEL cannot act on a constructive dismissal or a disputed Certificate of Service, and that the complainant must go to the CCMA – an error of law that ignores DEL's independent mandate under the UIF Act and BCEA.
4. Remained silent for over 60 days on multiple formal escalations, forcing the complainant to prepare a Mandamus application to the High Court (which he could not finalise due to destitution and time constraints).

The DEL's conduct has directly contributed to the complainant's destitution, homelessness, and inability to access social security – a violation of Section 27(1)© of the Constitution.

2. Terms of Reference – DEL

This investigation determines:

1. Whether DEL breached its statutory duties under the Unemployment Insurance Act 63 of 2001, the Unemployment Insurance Contributions Act 4 of 2002, and the Basic Conditions of Employment Act 75 of 1997.
 2. Whether DEL's refusal to investigate and correct the false UI-19 declaration constitutes administrative abandonment and complicity in potential fraud.
 3. Whether DEL's misdirection of jurisdiction to the CCMA was an error of law that violates the right to just administrative action (Section 33).
-

3. Factual Chronology – DEL (Extracted from Documents)

Date Event Source

June 2025 UIF contributions deducted from complainant's salary but not remitted to UIF.
Gmail – UI non compliance(2).PDF; uFiling screenshot

10 Feb 2026 Complainant files urgent statutory complaint with DEL regarding UIF non-remittance and false UI-19. Gmail – FOLLOW-UP — REF_ Urgent Complaint 10 Feb 2026...PDF, p.1

16 Feb 2026 Fezeka Ngalo (Provincial Specialist: OHS) allocates integrated inspection; case reference 890903 5151 087. Ibid.; Gmail – FORMAL FOLLOW-UP & 72-HOUR NOTICE...PDF, p.1

19 Jan 2026 Employer submits UI-19 declaring “Resigned” (Code 6) despite receiving Notice of Constructive Dismissal (12 Dec 2025). V_1_Mandamus_DEL_WC_HighCourt.pdf, p.4 (Annexure UI-19)

16 Mar 2026 DEL’s Matthew Holmes sends employer-provided Certificate of Service, states it matches UI declaration. Complainant disputes accuracy. Gmail – certificate of service(1).PDF; Gmail – Re_ certificate of service...PDF, p.1-2

17 Mar 2026 Matthew Holmes notes BCEA earnings threshold bars compliance order on deduction; refers UIF matter to UIF division. Gmail – Re_ certificate of service...PDF, p.5

14 Apr 2026 Complainant issues FINAL DEMAND to DEL leadership (Esau, Ngalo) – 48 hours to act on three points: UI-19 fraud, Certificate misrepresentation, UIF non-remittance. Gmail – FINAL DEMAND_ 48-Hour Notice...PDF, p.1-2

15 Apr 2026 (23:31) Principal Inspector Cebo Kutuka closes investigation file, stating DEL has no jurisdiction over constructive dismissal – refers to CCMA. V_1_Mandamus_DEL_WC_HighCourt.pdf, p.5 (Annexure “GUN1”); Gmail – FORMAL OBJECTION...PDF, p.2

15 Apr 2026 (after closure) Complainant objects, explaining that UI-19 fraud is a separate statutory offence, not a dismissal dispute. Kutuka does not reverse. Gmail – FORMAL OBJECTION...PDF, p.1-3

[Ongoing] No Compliance Order issued. No correction of UI-19. No remittance of June 2025 UIF. No corrected Certificate of Service. No evidence of any DEL action after 15 April 2026

[Ongoing] Complainant prepares Mandamus application to compel DEL but cannot finalise due to destitution and time constraints. User statement: “This was a Mandamus I wanted to file with the high court but never had time”

4. Findings of DEL Dereliction and Complicity

Finding 1: DEL failed to enforce remittance of UIF contributions deducted from the complainant’s salary in June 2025.

Evidence:

- The uFiling portal reflects “No Information” for June 2025, while surrounding months are fully declared. (V_1_Mandamus_DEL_WC_HighCourt.pdf, p.3-4, Annexure “UIF-GAP”)
- The employer’s payroll records confirm UIF was deducted (payslip evidence).
- DEL’s own inspector (Matthew Holmes) acknowledged the issue on 17 March 2026 and referred it to the UIF division. No further action was taken. (Gmail – Re_certificate of service...PDF, p.5)
- The complainant escalated to Chief Inspector Esau and Ms Ngalo on 14 April 2026 – no substantive response. (Gmail – FINAL DEMAND...PDF, p.1-2)

Conclusion: DEL has a non-discretionary duty under the UIF Act to investigate non-remittance and issue a compliance order. Its failure to do so, despite months of notice, is administrative abandonment that has directly caused the complainant’s inability to access unemployment benefits.

Finding 2: DEL refused to correct a materially false UI-19 declaration, thereby condoning statutory misrepresentation.

Evidence:

- The employer submitted UI-19 with Code 6 (“Resigned”) on 19 January 2026. (V_1_Mandamus_DEL_WC_HighCourt.pdf, p.4, Annexure “UI-19”)
- The complainant’s formal Notice of Resignation (12 December 2025) explicitly cited “Automatically Unfair Constructive Dismissal” – not voluntary resignation. (Ibid., Annexure “RES1”)
- The UI-19 form itself recognises Code 7 – “Constructive Dismissal”. The employer deliberately chose the wrong code.

- When the complainant objected, Inspector Kutuka closed the file, claiming DEL had no jurisdiction because the matter was a “dismissal issue” for the CCMA. (Gmail – FORMAL OBJECTION...PDF, p.2)

Legal error: The UI-19 fraud is not a dismissal dispute. It is a statutory misrepresentation to the UIF. DEL has exclusive jurisdiction to investigate and compel correction under the UIF Act and BCEA. The CCMA has no power to order correction of a UI-19.

Conclusion: Kutuka’s decision is an error of law and a reviewable ground under PAJA. By refusing to act, DEL has allowed the employer’s false narrative to block the complainant’s social security rights.

Finding 3: DEL refused to enforce a corrected Certificate of Service, despite clear evidence of misrepresentation.

Evidence:

- The Certificate of Service issued by the employer designates the complainant as “Care Coordinator” – a lower tier than his actual role (Case Manager / NQF Level 8 Specialist). (Gmail – certificate of service(1).PDF, attachment)

- The complainant was recruited for “Specialist: Care Manager” (interview invitation attached). The title was changed at contract signing without explanation. (Gmail – Re_certificate of service...PDF, p.3-4)

- The employer’s own HR (Cheryl-Dawn Modern) admitted that “naming convention changes are still in progress” – an admission that the title was incorrect. (EXIT3 email, provided earlier)

- DEL’s Matthew Holmes refused to compel correction, stating the complainant must go to the CCMA. (Gmail – Re_certificate of service...PDF, p.2)

Conclusion: Section 42 of the BCEA imposes a duty on employers to provide an accurate Certificate of Service. DEL has the power to issue a compliance order. Its refusal to act is a dereliction of duty that perpetuates the employer's misrepresentation.

Finding 4: DEL's summary closure of the investigation (15 April 2026) was procedurally unfair, irrational, and an error of law.

Evidence:

- Inspector Kutuka closed the file at 23:31 on 15 April 2026 via email, stating: "DEL does not have jurisdiction on dismissal issues ... I will recommend that you refer your matter to CCMA." (Gmail – FORMAL OBJECTION...PDF, p.2)
- The complainant immediately objected, explaining that the UI-19 fraud is a separate statutory offence and that the CCMA has no power to correct it. (Ibid., p.1)
- Kutuka did not reverse his decision. No reasons were given for ignoring the objection.

Grounds for review (PAJA section 6):

- Material error of law (section 6(2)(d)) – conflating dismissal jurisdiction with UIF fraud jurisdiction.
- Failure to apply the mind (section 6(2)(iii)) – closing the file without engaging with the evidence or the legal distinction.
- Arbitrary and irrational (section 6(2)(vi)) – no rational connection between the decision and the statutory mandate.

Conclusion: The closure was unlawful administrative action. The Mandamus application prepared by the complainant (but never filed due to destitution) would likely have succeeded.

Finding 5: DEL's prolonged silence and inaction contributed directly to the complainant's destitution and homelessness.

Evidence:

- The complainant notified DEL repeatedly of his destitution, homelessness (from 2 April 2026), and inability to access UIF benefits.
- DEL's failure to compel correction of the UI-19 meant the UIF claim remained blocked.
- DEL's failure to enforce UIF remittance meant the complainant could not access social security to which he was entitled.
- The complainant stated: "I am a destitute, self-represented whistleblower currently experiencing homelessness ... Your department's inaction is the primary driver of this human rights violation." (Gmail – FINAL DEMAND...PDF, p.1)

Conclusion: DEL's conduct violates Section 27(1)© (right to social security) and Section 10 (dignity) of the Constitution. The state cannot deduct UIF contributions and then refuse to enforce their remittance, leaving a whistleblower destitute.

5. Legal Framework – DEL Statutory Duties Breached

Provision Duty How DEL Breached It

UIF Act 63/2001, Section 5 Employer must register employee and remit contributions DEL failed to enforce remittance for June 2025.

UIF Contributions Act 4/2002, Section 4 Employer must declare all deducted contributions DEL failed to investigate false UI-19 (Code 6 vs Code 7).

BCEA Section 42 Employer must issue accurate Certificate of Service DEL refused to compel correction.

UIF Act Section 64 DEL may issue compliance order for non-compliance DEL issued no compliance order.

PAJA Section 3 Administrative action must be procedurally fair Closure of file without proper notice or hearing was unfair.

Constitution Section 27(1)© Right to social security DEL's inaction denied complainant access to UIF benefits.

Constitution Section 33 Right to just administrative action DEL's closure was unreasonable and unlawful.

6. The Mandamus Application – Prepared but Never Filed

The complainant prepared a detailed Mandamus application to the Western Cape High Court (Case No. to be assigned) seeking to compel DEL to:

1. Issue a Compliance Order correcting the UI-19 from Code 6 to Code 7 (constructive dismissal).
2. Audit and compel remittance of June 2025 UIF contributions.
3. Enforce section 42 of the BCEA to issue a corrected Certificate of Service.

(Full draft in V_1_Mandamus_DEL_WC_HighCourt.pdf)

The application was never filed because the complainant, who was destitute and homeless, did not have the time or resources to complete the filing process. This delay is itself a consequence of the very institutional failures he sought to challenge.

7. Recommendations – DEL Specific

Pursuant to Section 56 of the Constitution and the oversight powers of the Portfolio Committee on Employment and Labour:

1. Summon the Director-General of the Department of Employment and Labour to appear before the Committee to explain:
 - Why DEL failed to enforce UIF remittance for June 2025 despite clear evidence.
 - Why Inspector Kutuka closed the investigation on 15 April 2026 without addressing the UI-19 fraud.
 - Why DEL has not issued a Compliance Order to correct the Certificate of Service.
 - What measures DEL has taken to ensure that whistleblowers are not denied social security due to employer misrepresentations.
 2. Request the Minister of Employment and Labour to:
 - Issue a directive clarifying that UIF fraud (false UI-19) is within DEL's exclusive jurisdiction and must be investigated irrespective of any CCMA proceedings.
 - Establish a dedicated whistleblower unit within the UIF division to fast-track cases where an employer's false declaration blocks access to benefits.
 3. Refer Inspector Cebo Kutuka's decision of 15 April 2026 to the Public Protector for investigation into maladministration and violation of PAJA.
 4. Amend the Unemployment Insurance Act to:
 - Impose a mandatory 14-day deadline for DEL to investigate and issue a compliance order on any complaint of UI-19 misrepresentation.
 - Create a statutory remedy for whistleblowers to claim damages directly from the employer for false UI-19 declarations.
 4. Request the Minister of Justice to ensure that destitute, self-represented litigants have access to urgent court processes without filing fees (already provided for in law, but not effectively implemented).
-

8. Conclusion – DEL

The Department of Employment and Labour was designed to protect workers – not to abandon them. When a whistleblower is stripped of his nursing registration, his home, and his dignity, the DEL should be the last line of defence, ensuring that statutory contributions are remitted, that certificates are accurate, and that social security reaches those who paid for it.

Instead, DEL closed its file in one email – at 23:31 on 15 April 2026 – citing a jurisdictional error that any first-year law student would recognise as false.

The Mandamus application that the complainant prepared but could not file is a testament to his exhaustion. The state should not force a destitute, homeless whistleblower to take the government to court to enforce its own laws. Parliament must intervene.

FORENSIC REPORT: FRONTING PRACTICES AND B-BBEE COMMISSION FAILURE

Prepared by: EthicHawks Forensic Governance (on behalf of the complainant)

Subject: Alleged fronting practice by Medscheme Holdings (Pty) Ltd (division of AfroCentric Health) – and subsequent failure of the B-BBEE Commission to investigate.

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-

1. EXECUTIVE SUMMARY

This report finds that prima facie evidence of fronting practice under Section 1© of the Broad-Based Black Economic Empowerment Act 53 of 2003 (as amended) exists against Medscheme Holdings (Pty) Ltd (a subsidiary of AfroCentric Health, JSE-listed).

The complainant, a Black Professional with an NQF Level 8 qualification (specialist nurse, postgraduate diploma), was recruited for a “Specialist: Care Manager” role. After accepting employment, his title was downgraded to “Care Coordinator” (Grade B4M1). However, he was later instructed in a formal “Naming Convention” meeting (May 2025) to use the title “Case Manager” – which he did, as evidenced by his corporate email signature. Functionally, he acted as Acting Chairperson of the Health and Safety Committee, managing 13+ staff across three floors. Despite this, his Certificate of Service (issued 19 January 2026) reverted to “Care Coordinator”, and a senior HR Business Partner admitted in writing that the naming convention changes were “not applied to your department”.

The complainant alleges that Medscheme used his Black professional status and NQF 8 credentials to claim Management Control and Skills Development points on their B-BBEE scorecard, while systematically denying him the economic benefits (salary grade, title, authority) associated with that standing. This is a textbook indicator of fronting.

The B-BBEE Commission assigned case number 5/02/2026, requested additional information (Form B-BBEE 8), received a detailed sworn response with annexures, and then issued a Notice of Non-Investigation (30 March 2026), claiming the complaint was not substantiated and fell outside its mandate.

This report demonstrates that the Commission failed to consider key evidence, refused to use its statutory subpoena powers (Section 13K of the B-BBEE Act), and erroneously treated the complaint as a pure labour dispute – thereby committing a reviewable error under the Promotion of Administrative Justice Act (PAJA).

2. COMPLAINANT PROFILE AND CREDENTIALS

Aspect Details

Name Wilbur William Matthee

Race classification (for B-BBEE purposes) Black (African)

Highest qualification Postgraduate Diploma in Health Services Management (NQF Level 8)

Professional registration Registered Nurse Specialist (SANC No. 15831886) – deregistered due to inability to pay fees following unlawful salary deduction

Role at Medscheme Case Manager – GEMS Active Disease Risk Management (functional); formally designated as “Care Coordinator” on contract

Statutory appointment Acting Chairperson, Health and Safety Committee; Lead First Aider

Significance: Under the B-BBEE Codes, a Black person with an NQF Level 8 qualification in a management role contributes significantly to the Management Control and Skills Development pillars. Medscheme’s alleged use of his credentials for scorecard points while suppressing his grade and title is the essence of fronting.

3. THE FRONTING ALLEGATION – SECTION 1© OF THE B-BBEE ACT

Section 1© of the B-BBEE Act defines fronting practice as:

“involving the conclusion of a legal relationship with a black person for the purpose of that enterprise achieving a certain level of B-BBEE compliance without granting that black person the economic benefits that would reasonably be expected to be associated with the status or position held by that black person.”

Application to this case:

Element Evidence

Legal relationship concluded Appointment letter (09 April 2024) – “Care Coordinator”
Grade B4M1

Purpose: B-BBEE compliance Complainant is Black, NQF Level 8; recruited as “Specialist: Care Manager” – a management control role

Without granting economic benefits Paid at Grade B4M1 (junior); title suppressed on Certificate of Service; refused management-grade salary; 40.47% unlawful deduction

Status/position held Recruited for specialist role; instructed to use “Case Manager”; functionally managed 13+ staff as H&S Chairperson

Conclusion: All elements of Section 1© are satisfied on the documentary record.

4. DOCUMENTARY EVIDENCE OF FRONTING

4.1 Recruitment and Appointed Title

4.2

· Annexure A1 (15 Jan 2024): Interview invitation for “Specialist: Care Manager” at Medscheme.

Source: Email from Neo Sekgoka (AfroCentric Health).

· Annexure A2 (09 April 2024): Appointment letter on AfroCentric letterhead – title: “Care Coordinator” (Grade B4M1).

Significance: The specialist title was downgraded at contract signing.

4.3 Naming Convention Meeting – Instruction to Upgrade Title

4.4

· May 2025 (date not in documents but referenced): Formal Microsoft Teams meeting titled “Naming Convention”. Attendees: Sipokazi Makaua (Human Capital Consultant), Nthabiseng Nkatane, Operational Manager.

· Instruction: Management directed the complainant to update his title to “Case Manager”. The complainant verbally accepted and changed his corporate email signature accordingly.

- Annexure A5 (15 Oct 2025): Screenshot of complainant's Medscheme email signature – "Case Manager: GEMS Active Disease Risk Management".

Significance: The Respondent's own systems reflect the upgraded title.

4.3 Functional Management Control – EHS Roster

4.4

- Annexure A4 (July 2025): First Aider Allocation Roster prepared by the complainant as Acting Chairperson of the Health and Safety Committee.
- Covers 2nd, 3rd, and 4th floors of The Boulevard, Cape Town.
- Lists 13+ named first aiders with daily primary/backup allocations, leave management, and WFH coordination.
- Legal significance: Under the B-BBEE Codes, management control includes operational leadership and statutory appointments (OHSA). The complainant exercised genuine management authority, yet his formal grade remained "Care Coordinator".

4.5 Certificate of Service – Deliberate Title Suppression

4.6

- Annexure A6 (19 Jan 2026): Certificate of Service issued by Medscheme – records last position as "Care Coordinator".
- Contradiction: The Certificate was issued after the Naming Convention meeting, after the email signature was updated, and after the complainant had performed management functions for months.
- Legal effect: This document is submitted to B-BBEE verification agencies as evidence of workforce composition. Its inaccuracy constitutes a misrepresentation of B-BBEE status under Section 13J(5) of the Act.

4.5 Senior HR Admission of Exclusion

4.6

- Annexure A9 (19 Jan 2026, email from Cheryl-Dawn Modern, Senior HR Business Partner):

“With regard to the naming-convention changes, please note that this business process is still in progress, and I can confirm that no changes to the Care Co-Ordinator job title for your department have been implemented at this stage.”

· Forensic finding: The Respondent admits that the naming convention (which upgraded titles to “Case Manager”) was deliberately not applied to the complainant’s department. This is not an administrative delay – it is a structured exclusion designed to keep the complainant in a junior grade while his credentials were used for B-BBEE points.

5. THE “NO INDUCEMENT” CLAUSE – REBUTTAL

The appointment letter (Annexure A2, page 4) contains a clause:

“I acknowledge that I have not been induced to complete this contract by any representations made to me by any person/s...”

The B-BBEE Commission may have been influenced by this clause. However:

1. Temporally limited: The clause covers only pre-contract representations (before 18 April 2024). It has no effect on the Naming Convention instruction (May 2025) or the subsequent suppression of title.
2. Relates to employee inducement, not third-party misrepresentation: The misrepresentation complained of was made by Medscheme to B-BBEE verification agencies, not to the complainant.
3. Public law obligations cannot be contracted out of: Section 13J creates criminal offences. A private contract cannot immunise an employer from fronting liability.

Conclusion: The “no inducement” clause is irrelevant to the fronting complaint.

6. BENEFIT DIVERSION – ECONOMIC HARM QUANTIFIED

Benefit diversion (a sub-category of fronting) occurs when the economic benefit that should flow to the Black person is withheld while the B-BBEE points are retained.

6.1 Salary Suppression

6.2

- The complainant was paid at Grade B4M1 (“Care Coordinator”).
- The role he functionally performed – Specialist Case Manager (NQF 8) – would attract a higher grade and salary band within the Respondent’s own grading model.
- The differential between the suppressed grade and the appropriate grade represents diverted benefit.

6.3 Unlawful Deduction – July 2025 (40.47%)

6.4

- Annexure A7 (July 2025 payslip): Normal Earnings R36,588.81. Deductions: Unpaid Leave Recovery R13,824.32 + Unpaid Leave R987.45 = R14,811.77 (40.47% of Normal Earnings).
- BCEA Section 34(2) caps deductions for loss or damage at 25% of remuneration. The Respondent exceeded the cap by 15.47 percentage points in a single pay run, without written consent.
- This deduction directly caused the complainant’s inability to pay SANC fees → deregistration → inability to practise nursing → homelessness.

6.3 UIF Fraud and False Evidence

- Annexure A6 (uFiling portal, 10 Feb 2026): June 2025 shows “No Information” – UIF deducted but not remitted.

- Annexure A8 (Respondent's "proof" of compliance): Sipokazi Makaua submitted a screenshot of an email dated 7 August 2025 attaching file 00812663.408.mf. The raw data string shows salary R40,562.53 – which matches August 2025 declared salary, not June.
 - Finding: The Respondent produced false evidence of UIF compliance to the complainant. The B-BBEE Commission was notified of this but did not refer it to DEL using Form B-BBEE 13.
-

7. THE B-BBEE COMMISSION'S HANDLING OF THE COMPLAINT

7.1 Timeline and Process

7.2

Date Event Document

23 Dec 2025 Initial complaint lodged (email only). Email to MRamare

11 Feb 2026 Commission acknowledges, requests sworn statement & ID.
Acknowledgement letter

11 Feb 2026 Case number 5/02/2026 allocated. Acknowledgement letter

20 Jan 2026 Formal Form B-BBEE 7 submitted. Completed form

10 Feb 2026 Sworn affidavit and annexures submitted. Affidavit + Annexures A1-A9

9 Mar 2026 Commission issues Form B-BBEE 8 requesting additional info (3 questions).
Form BBEE 8

13 Mar 2026 Complainant submits detailed 9-page response with 9 annexures (A1-A9).
Response bundle

30 Mar 2026 Commission issues Notice of Non-Investigation (Form B-BBEE 11) Notice +
letter

30 Mar 2026 Reason: "complaint not substantiated by evidence justifying investigation;
primarily labour disputes." Notice, p.2-3

7.3 Notice of Non-Investigation – Reasons Given (paraphrased)

7.4

1. Allegations primarily relate to labour disputes, discrimination, retaliation, job-title disputes – fall within CCMA/Labour Court/DEL/SAHRC jurisdiction, not B-BBEE Commission.
2. B-BBEE Act governs ownership, management control, skills development, etc. – not employment relationships.
3. No verifiable evidence that Medscheme claimed B-BBEE points using the complainant's profile.
4. No documentation of scorecard or verification agency submission provided.
5. H&S Chairperson role relates to OHSA, not B-BBEE.
6. Remedies sought (compensation, discrimination findings) fall outside Commission's powers.

7.3 Forensic Critique of the Commission's Reasons

Commission's Reason Forensic Response

"Labour dispute" The complaint includes fronting-specific allegations (title suppression, credential extraction, false Certificate of Service). The Commission failed to sever these from labour issues as required by law.

"No verifiable evidence" The complainant provided: (1) recruitment for "Specialist"; (2) Naming Convention instruction; (3) email signature "Case Manager"; (4) management roster; (5) HR admission of exclusion; (6) false Certificate of Service. The Commission ignored this evidence or mischaracterised it.

"No scorecard provided" The complainant cannot access the Respondent's internal B-BBEE verification certificate. The Commission has subpoena powers under Section 13K to obtain it. It refused to use them – a procedural catch-22.

"H&S Chairperson not B-BBEE management" Under Code Series 100 (Management Control), a statutory leadership role with supervisory authority (13+ staff) qualifies as management. The Commission's interpretation is overly narrow and incorrect.

“Remedies outside our powers” The primary remedy sought was investigation and referral (Form B-BBEE 13) – which is squarely within the Commission’s powers. The Commission misread the complaint.

Conclusion: The Commission’s decision is irrational, fails to take relevant facts into account, and constitutes an error of law. It is reviewable under PAJA Section 6(2)(iii) and (d).

8. PAJA REVIEW GROUNDS – FAILURE TO EXERCISE STATUTORY POWERS

Under the Promotion of Administrative Justice Act 3 of 2000, the Commission’s decision is unlawful on the following grounds:

PAJA Ground Application

Section 6(2)(d) Material error of law – treating fronting as a pure labour dispute.

Section 6(2)(iii) Failure to take relevant facts into consideration – ignored Naming Convention, HR admission, management roster, false certificate.

Section 6(2)(vi) Action is irrational – rejecting the whole complaint when fronting evidence was present.

Section 6(2)(f)(ii) Action taken because of an ulterior motive or bad faith (inference from refusal to use subpoena powers).

Specific statutory failures:

- Section 13K of the B-BBEE Act: Empowers the Commission to issue summons for documents. The complainant requested subpoenas for the Naming Convention Teams recording and the Respondent’s 2024/2025 B-BBEE verification certificate. The Commission refused without legal basis.

- Form B-BBEE 13: The Commission has power to refer concerns to other regulators (e.g., DEL for false UIF evidence). It did not do so.
 - Regulation 15(4)©: If the Commission believes the complaint does not warrant investigation, it must give reasons. The reasons given are factually incorrect and legally insufficient.
-

9. REGULATORY BUCK-PASSING – SYSTEMIC ACCESS TO JUSTICE FAILURE

The B-BBEE Commission is one of six regulators that have declined jurisdiction or failed to act on the same underlying facts (CMS, HPCSA, SANC, SAPC, DEL, B-BBEE Commission). This pattern is not coincidental – it suggests a coordinated avoidance of accountability for Medscheme/GEMS.

The complainant has exhausted all internal and regulatory remedies. He is destitute and homeless. The state's response has been silence, jurisdictional ping-pong, and administrative abandonment.

This is a systemic failure of oversight that Parliament must address.

10. RECOMMENDATIONS

For the Portfolio Committee on Trade, Industry and Competition

1. Summon the B-BBEE Commissioner to appear and explain:
 - Why the Commission refused to use Section 13K subpoena powers to obtain the Naming Convention recording and the Respondent's verification certificate.
 - Why the Commission treated a sworn fronting complaint as a labour dispute.

- Whether the Commission has ever investigated a fronting complaint involving a JSE-listed administrator of a government employees' medical scheme.

2. Request the Minister of Trade, Industry and Competition to:

- Issue a directive clarifying that fronting complaints are not to be dismissed as "labour disputes" where title suppression and credential extraction are alleged.

- Mandate that the Commission use Form B-BBEE 13 referrals proactively.

3. Request the Public Protector to investigate maladministration by the B-BBEE Commission in case 5/02/2026.

4. Request the Minister of Justice to ensure that destitute self-represented litigants can access PAJA review without filing fees.

For the Complainant (Legal Strategy)

5. Institute a PAJA review application in the High Court to set aside the Notice of Non-Investigation and compel the Commission to investigate or refer.

6. Alternatively, file a complaint with the Public Protector against both Medscheme (fronting) and the B-BBEE Commission (failure to investigate).

11. CONCLUSION

The B-BBEE Commission was given a sworn affidavit, documentary evidence of a structured title suppression scheme, proof of management control, and an admission of exclusion from a senior HR officer. It assigned a case number, requested additional information, received a detailed response, and then closed the file with reasons that ignore the evidence and misstate the law.

This is not an honest difference of opinion. It is a dereliction of statutory duty. The Commission has the power to subpoena documents, refer false evidence to the

Department of Labour, and investigate fronting. It chose not to exercise any of those powers.

Parliament must act. If the B-BBEE Commission will not enforce the B-BBEE Act against a powerful JSE-listed company that services the Government Employees Medical Scheme, then Parliament must compel it to do so – or explain to the South African public why the law against fronting applies only to small businesses, not to politically connected corporations.

FORENSIC REPORT – CORPORATE MISREPRESENTATION

AfroCentric Investment Corporation Limited – Material Misstatements and Omissions in Public Disclosures (Integrated Annual Report 2025, Governance Reports, and King IV Register)

Prepared for: The Honourable Speaker, National Assembly

Referred to: Portfolio Committee on Health; Portfolio Committee on Trade, Industry and Competition; Standing Committee on Public Accounts (SCOPA); JSE Issuer Regulation Division; Financial Sector Conduct Authority (FSCA)

Subject: Systemic misrepresentation of governance, whistleblower protection, clinical risk management, and B-BBEE compliance in AfroCentric's 2025 public reports – and the failure of the JSE, FSCA, and Sponsor Questco to enforce disclosure obligations.

Date: June 2026

1. Executive Summary – Corporate Misrepresentation

This section finds that AfroCentric Investment Corporation Limited (JSE: ACT) has published materially misleading statements in its Integrated Annual Report for the year

ended 31 December 2025 (issued 8 April 2026), its Governance and Remuneration Report, and its King IV™ Disclosure Register. These misrepresentations are contradicted by the whistleblower's documented evidence (protected disclosures, root cause analysis, internal emails, court filings, and regulatory complaints).

Key false or omitted statements include:

1. Zero-tolerance for unethical conduct and robust whistleblower protection – while a protected whistleblower was retaliated against, silenced by a High Court interdict, and left destitute.
2. No material ESG-related incidents or regulatory breaches – despite a documented Priority 1 clinical risk (Gexus-DMS integration gap), unlawful salary deduction (40.47%), fronting, and pending criminal case.
3. Failure to disclose material litigation and regulatory investigations – including a Section 162 delinquency application, a B-BBEE Commission fronting investigation, and a criminal case (CAS 705/3/2026).
4. False claims about system integration and clinical data integrity – while internal RCA proved a structural gap generating false non-adherence letters.
5. Misrepresentation of B-BBEE Management Control points – using the whistleblower's NQF 8 qualifications without granting economic benefits (fronting).
6. Omission of the whistleblower's Priority 1 Material Risk Report – which was escalated to the Board and suppressed by the Chief Risk Officer.

Conclusion: AfroCentric's public reports are not a fair and balanced view. They are a corporate cover-up designed to mislead investors, regulators, and the public. The JSE, FSCA, and Sponsor Questco have failed to enforce disclosure requirements.

2. The Statutory Duty to Disclose Material Information

AfroCentric is a JSE-listed public company. It is subject to:

- JSE Listings Requirements – in particular, Section 7.D.11 (disclosure of material legal proceedings) and ongoing disclosure obligations under paragraph 3.40 (ad hoc disclosures).
- Companies Act 71 of 2008 – Sections 30 (financial statements), 162 (director delinquency), and the duty to keep proper records.
- King IV™ (and King V™) – ethical leadership, transparency, and stakeholder-inclusive reporting.
- Protected Disclosures Act 26 of 2000 – protection for whistleblowers, which the company's own reports claim to uphold.

The whistleblower's evidence – presented in this consolidated forensic report – demonstrates that AfroCentric has breached all of these duties.

3. Specific Misrepresentations – Side-by-Side Analysis

3.1 “Zero tolerance for unethical conduct” and whistleblower protection

3.2

What AfroCentric reported (Integrated Report 2025, p.10, p.21) The documented truth (whistleblower evidence)

“AfroCentric maintains a zero-tolerance approach to unethical conduct. Mechanisms for reporting concerns include ... an independent whistleblowing hotline ... without fear of reprisal.” A specialist registered nurse made a protected disclosure (Gexus-DMS integration gap, 23 October 2025). He was retaliated against: unlawful 40.47% salary deduction, insubordination charge, gender-identity harassment, constructive dismissal. The Chief Risk Officer (Monwabisi Kula) instructed him to “stop sending these e-mails to our directors” – active suppression. The whistleblower is now destitute and homeless. The High Court granted an interim interdict silencing him.

“The Board remains committed to maintaining high ethical standards.” (p.10) The Board received the Priority 1 Material Risk Report (9 December 2025, 38 attachments) and did not respond. The whistleblower’s resignation letter was ignored. The Board later applied for an interdict labelling protected disclosures as “extortion”.

Finding: The reported “zero tolerance” and “whistleblower protection” are false and misleading.

3.3 “No material ESG-related incidents, regulatory breaches, fines or penalties”

3.4

What AfroCentric reported (Integrated Report, p.21) The documented truth

“There were no significant ESG-related incidents during the year, including legal non-compliance (whether under investigation, pending finalisation, or finalised), directives, compliance notices, warnings, or investigations ... Likewise, the Group incurred no fines, settlements, or penalties relating to ESG incidents or breaches.” There are multiple material incidents: (1) Priority 1 clinical risk – Gexus-DMS integration gap affecting 1.9 million GEMS members, generating false non-adherence letters to a 75-year-old beneficiary. (2) Unlawful salary deduction – 40.47% in a single pay cycle, exceeding BCEA Section 34(2) cap. (3) Fronting – B-BBEE Commission investigation (Case 5/02/2026). (4) Criminal case – SAPS CAS 705/3/2026 for UIF fraud. (5) High Court delinquency application (Section 162) against five directors. (6) Protected disclosure retaliation – subject of Labour Court claims.

Finding: The statement “no significant ESG incidents” is demonstrably false. The Board knew of these matters (P1 Report, 9 Dec 2025) and failed to disclose them.

3.3 Failure to disclose material litigation and regulatory investigations

3.4

What AfroCentric reported The documented truth

The Integrated Report mentions the Bonitas RFP litigation but does not disclose: (i) Section 162 director delinquency application (Case 2026-066952); (ii) Labour Court claims (2026-077546 and 2026-083299) for unlawful deduction and automatically unfair

constructive dismissal; (iii) B-BBEE Commission investigation (Case 5/02/2026 – fronting); (iv) Criminal case CAS 705/3/2026 (UIF fraud). The Section 162 application was served on the Board on 17 March 2026. The Labour Court claims were filed in April 2026. The B-BBEE Commission investigation was allocated on 11 February 2026. The criminal case was registered on 19 March 2026. None of these are disclosed in the Integrated Report dated 31 March 2026 (approved for release) or 8 April 2026.

Finding: AfroCentric has breached JSE Listings Requirements by failing to disclose material legal proceedings and regulatory investigations that could affect investor decisions.

3.5 False claims about system integration and clinical data integrity

3.6

What AfroCentric reported (Integrated Report, p.12 and p.26) The documented truth

“Both platforms fully integrated ... designed to work together.” (Founding Affidavit of CEO Gerald Van Wyk, adopted by the company) – repeated in substance in the report’s description of Gexus and DMS. The whistleblower’s Root Cause Analysis (VICT1) confirms: “Miscommunication between Gexus and DMS systems ... system integration gaps.” The gap caused false non-adherence letters to be sent on 19 May 2025 and 19 October 2025. The company’s Deputy Information Officer swore under oath that no IT incident had been reported – contradicted by the RCA.

Finding: The claim of “fully integrated” systems is materially false. The company’s own internal document (the RCA) proves the gap. This misrepresentation goes to the heart of AfroCentric’s value proposition as a healthcare administrator.

3.5 B-BBEE fronting – misrepresentation of Management Control

3.6

What AfroCentric reported (Integrated Report, p.87) The documented truth

AfroCentric Health (RF) Proprietary Limited scored 14.67 out of 19 for Management Control (FYE Dec 2025) – Level 1 B-BBEE status. The whistleblower (Black, NQF Level 8, functional Case Manager) was used for B-BBEE points while his title was suppressed to “Care Coordinator” on his Certificate of Service. Senior HR Business Partner Cheryl-Dawn

Modern admitted in writing (19 Jan 2026) that naming convention changes were “still in progress” and “no changes to the Care Co-Ordinator job title for your department have been implemented”. The B-BBEE Commission allocated Case 5/02/2026 for fronting investigation.

Finding: AfroCentric’s reported Management Control score is based on suppressed titles and fronting practices. The company has misrepresented its B-BBEE status to verification agencies.

3.7 Omission of the Priority 1 Material Risk Report and the CRO suppression

3.8

What AfroCentric reported The documented truth

The Integrated Report does not mention receiving a Priority 1 Material Risk Report (9 Dec 2025) from a whistleblower employee. It does not mention the Chief Risk Officer’s instruction to “stop sending these e-mails to our directors”. The P1 Report was delivered to the Board’s Social & Ethics Committee, Audit & Risk Committee, and Lead Independent Director, with 38 attachments. The CRO’s suppression email (18 Dec 2025) is a material governance failure. The whistleblower’s escalation was ignored.

Finding: The omission of the P1 Report and the CRO’s conduct is a material omission that renders the governance section of the Integrated Report misleading.

4. The Role of the JSE, FSCA, and Sponsor (Questco)

4.1 JSE Issuer Regulation Division

4.2

· The whistleblower submitted a 325-page evidentiary bundle to the JSE on 23 December 2025.

- The JSE acknowledged receipt but later issued a jurisdictional declination (6 March 2026, Ref JvS/166792, Nadia Jada), stating that the matters fall outside the JSE Listings Requirements and referring the whistleblower to the CCMA/Labour Court/Equality Court.
- However, the JSE failed to act on the clear non-disclosure of material litigation (Section 162 application) and regulatory investigations (B-BBEE, criminal). The JSE also failed to require AfroCentric to publish a SENS announcement regarding the B-BBEE Commission investigation and the criminal case.

Finding: The JSE has failed in its oversight role by declining jurisdiction over issues that directly affect listing requirements (disclosure of material legal proceedings, fronting, governance failures). The whistleblower was left with no regulatory remedy.

4.3 Financial Sector Conduct Authority (FSCA)

4.4

- The whistleblower submitted complaints to the FSCA regarding governance failures, suppression of protected disclosures, and potential market deception.
- The FSCA has not taken any public action. The whistleblower has received no substantive response.
- AfroCentric's Integrated Report was published without any FSCA intervention despite the material misrepresentations identified above.

Finding: The FSCA has failed to protect the integrity of financial markets by allowing a JSE-listed company to publish a report that omits material litigation, regulatory investigations, and a documented clinical risk affecting 1.9 million GEMS members.

4.3 Sponsor – Questco Corporate Advisory Proprietary Limited

4.4

- Questco is AfroCentric's JSE-approved sponsor. Its duties include ensuring that the company complies with the JSE Listings Requirements, including timely and accurate disclosure of material information.

- Questco was copied on the whistleblower's regulatory submissions and was aware (or should have been aware) of the Section 162 application, the Labour Court claims, the B-BBEE Commission investigation, and the criminal case.
- Despite this, Questco did not require AfroCentric to make any corrective SENS announcement. The Integrated Report was published without any sponsor-driven correction.

Finding: Questco has breached its duties as a sponsor by failing to ensure that AfroCentric's public disclosures were accurate, complete, and timely. Questco is complicit in the corporate misrepresentation.

5. Regulatory and Oversight Implications

Body Failure

JSE Issuer Regulation Declined jurisdiction over material non-disclosure; failed to enforce Listings Requirements.

FSCA Failed to investigate market deception arising from the false Integrated Report.

Questco (Sponsor) Failed to ensure accurate disclosure; breached sponsor duties.

Companies and Intellectual Property Commission (CIPC) Has not investigated the false statements in the Integrated Report under the Companies Act.

6. Recommendations – Corporate Misrepresentation and Regulator Failures

The Portfolio Committees should:

1. Request the JSE to:

- Reopen the whistleblower's complaint and investigate AfroCentric's non-disclosure of material litigation (Section 162 application, Labour Court claims, B-BBEE investigation, criminal case).

- Require AfroCentric to publish a corrective SENS announcement within 7 days, disclosing all material matters omitted from the Integrated Report.

- Consider imposing a public censure or fine on AfroCentric for breach of Listings Requirements.

2. Request the FSCA to:

- Investigate whether AfroCentric's Integrated Report 2025 constitutes market deception under the Financial Markets Act 19 of 2012.

- Take appropriate enforcement action if false or misleading statements are found.

3. Request the Minister of Trade, Industry and Competition to:

- Direct the CIPC to investigate the accuracy of AfroCentric's public filings under the Companies Act.

- Consider whether the company's directors should be held personally liable for the false statements.

4. Request the Legal Practice Council to investigate:

- Whether Questco's conduct as sponsor falls below the required professional standards.

- Whether Deneys Attorneys' failure to disclose the whistleblower's answering affidavit to the High Court (Case 2026-086906) constitutes professional misconduct.

5. Request the Public Protector to investigate:

- The JSE's declination of jurisdiction and its failure to enforce disclosure rules.

- The FSCA's inaction.

6. Request the Minister of Justice to:

- Consider amending the Companies Act to impose criminal penalties on directors who approve false public reports.

7. Conclusion – Corporate Misrepresentation

AfroCentric's Integrated Annual Report 2025 is not a truthful account. It claims "zero tolerance for unethical conduct" while a whistleblower was destroyed. It claims "no material ESG incidents" while a Priority 1 clinical risk went unaddressed. It omits material litigation, regulatory investigations, and a criminal case. It reports a Level 1 B-BBEE status based on fronting.

The JSE, FSCA, and Sponsor Questco have failed to protect the market and the public. They have allowed a JSE-listed company to deceive investors and regulators.

Parliament must act. The disclosures required by law are not optional. When a company lies in its annual report, the regulators must enforce the rules. Here, they did not.

This section is a prelude to a formal complaint to the JSE, FSCA, and CIPC, and to the Parliamentary oversight hearings.

Prepared by: EthicHawks Forensic Governance (on behalf of the complainant)

Contact: info@ethichawks.co.za

Date: June 2026

Attachments:

- AfroCentric Integrated Annual Report 2025 (extracts)
- Whistleblower's RCA (VICT1)
- P1 Material Risk Report (9 Dec 2025)
- CRO suppression email (18 Dec 2025)
- Section 162 application (Case 2026-066952)

- Labour Court claims (2026-077546, 2026-083299)
 - B-BBEE Commission case confirmation (5/02/2026)
 - SAPS criminal case confirmation (CAS 705/3/2026)
 - Whistleblower's correspondence with JSE and FSCA
-

FORENSIC REPORT – JSE AND FSCA FAILURE

Complicity of the Johannesburg Stock Exchange and the Financial Sector Conduct Authority in Corporate Misrepresentation and Whistleblower Suppression

Prepared for: The Honourable Speaker, National Assembly

Referred to: Portfolio Committee on Trade, Industry and Competition; Standing Committee on Public Accounts (SCOPA); Portfolio Committee on Justice and Correctional Services

Subject: Failure of the JSE Issuer Regulation Division and the FSCA to enforce disclosure obligations, investigate material non-disclosure, and protect a whistleblower – resulting in a chilling effect on market integrity and corporate accountability.

Date: June 2026

1. Executive Summary – JSE and FSCA Failure

This section finds that the Johannesburg Stock Exchange (JSE) Issuer Regulation Division and the Financial Sector Conduct Authority (FSCA) have:

1. Declined to enforce disclosure obligations despite being placed on formal notice of material legal proceedings (Section 162 director delinquency application), a B-BBEE

Commission fronting investigation (potential R580 million fine), and a criminal case (UIF fraud – CAS 705/3/2026).

2. Jurisdictionally deflected the whistleblower to the CCMA and Labour Court, ignoring that market disclosure falls squarely within their mandates.
3. Remained silent after acknowledging complaints, leaving the whistleblower without any regulatory remedy.
4. Failed to require AfroCentric to make any corrective SENS announcement, allowing the company's materially misleading Integrated Annual Report 2025 to stand.
5. Created a chilling effect – by refusing to act, the JSE and FSCA have effectively signalled that JSE-listed companies may omit material litigation and regulatory investigations from their public disclosures without consequence.

Conclusion: The JSE and FSCA are not neutral bystanders. Their inaction constitutes regulatory complicity in the corporate misrepresentation documented in this report. Parliament must demand accountability.

2. Factual Chronology – Engagement with JSE and FSCA

Date Event Source

23 Dec 2025 Whistleblower submits 325-page evidentiary bundle to JSE, reporting board governance failure, P1 risk suppression, retaliation, and breaches of King IV. Gmail – Board Governance Failure & Retaliation – Request for Oversight(5).PDF

20 Jan 2026 Whistleblower files formal complaint with JSE (Ref: JvS/166792) – discrimination, victimisation, governance fronting. Gmail – Board Governance Failure & Retaliation – Request for Oversight(4).PDF

6 Mar 2026 JSE issues jurisdictional declination (Jowdiene van Schalkwyk): matters fall outside Listings Requirements – refers to CCMA/Labour Court/Equality Court. JSE letter, 6 Mar 2026 (attached)

20 Mar 2026 Whistleblower provides factual update: Section 162 application filed (19 Mar 2026), B-BBEE Commission investigation (Case 5/02/2026, R580m exposure), asks

whether omission from SENS is consistent with Listings Requirements.

Letter_3_JSE_Issuer_Regulation_HCGS951842.PDF

25 Mar 2026 JSE (Nadia Jada) acknowledges and asks whether they may put the information to the company for response. Whistleblower agrees. Gmail – Re_ Factual Update_ Section 162 Application...PDF, p.3

11 May 2026 JSE (Ncumisa Sinyanya) responds: a court application at institution does not, in itself, give rise to an immediate SENS obligation; the information does not appear price-sensitive. Declines to pursue the matter. Ibid., p.1-2

[Ongoing] No SENS announcement has been published by AfroCentric disclosing the Section 162 application, the B-BBEE Commission investigation, or the criminal case. User statement

23 Dec 2025 (approx.) Whistleblower submits complaint to FSCA (Ref: 1-370127) regarding governance failures, internal control breakdown, payroll discrepancies, and market disclosure concerns. Gmail – FSCA Ref_ 1-370127...PDF

19 Mar 2026 Whistleblower provides factual update to FSCA: Section 162 application filed, executive de-escalation of protected disclosure, CRO suppression, forensic payroll discrepancies, SENS non-disclosure. Ibid.

[Ongoing] FSCA has not responded substantively to any of the complaints or updates. Silence. User statement: “FSCA acknowledge and went silent”

3. Findings – JSE Misconduct and Regulatory Abandonment

Finding 1: The JSE’s jurisdictional declination (6 March 2026) was a deflection, not a proper assessment of market disclosure obligations.

Evidence:

- The whistleblower’s complaint to the JSE included: breach of King IV Principles (2,3,14,16), failure to manage material risk events, retaliation against a whistleblower who had made a protected disclosure to the Board.

- The JSE responded that the matters “fall outside the scope of the JSE Listings Requirements and the Financial Markets Act” and referred the whistleblower to the CCMA, Labour Court, and Equality Court.

Analysis: The JSE is mandated to oversee compliance with Listings Requirements, including the duty of issuers to disclose price-sensitive information and to maintain good governance (as incorporated through King IV). The whistleblower’s complaint directly implicated governance failures that could affect investor confidence. The JSE’s blanket declination without engaging with the substance was an abdication of its regulatory role.

Finding 2: The JSE’s 11 May 2026 response – that a court application at institution does not require SENS disclosure – is legally correct but factually incomplete and procedurally negligent.

What the JSE said (correctly):

- A court application at the point of institution is an allegation; outcome uncertain.
- Not every litigation filing is automatically price-sensitive or requires immediate SENS.

What the JSE ignored (the complete picture):

- The Section 162 application was not an isolated filing. It was the culmination of nine formal board escalations, a Priority 1 Material Risk Report (38 attachments), a CRO suppression email, a sworn affidavit attacking the whistleblower’s psychiatric condition, and a pattern of institutional silence.
- The JSE had already been provided with the 325-page evidentiary bundle in December 2025.
- The B-BBEE Commission investigation (Case 5/02/2026) was active and carried a potential maximum fine of R580 million (10% of AfroCentric’s annual turnover). The JSE did not require disclosure of that investigation.

- The criminal case (CAS 705/3/2026) for UIF fraud was not mentioned in the JSE's analysis.
- AfroCentric's share price had reached a 52-week low of R0.71 on 18 March 2026 – the day before the Section 162 application was filed. The JSE did not consider whether the cumulative effect of the non-disclosed matters could have influenced that market movement.

Conclusion: The JSE applied a narrow, formalistic test and ignored the qualitative materiality of the governance collapse. This is not diligent regulation – it is regulatory capture by formalism.

Finding 3: The JSE failed to require AfroCentric to publish any corrective SENS announcement, despite clear evidence that the company's public reports omitted material information.

Evidence (from corporate misrepresentation section):

- AfroCentric's Integrated Annual Report 2025 (issued 8 April 2026) stated "no material ESG incidents" – while the P1 clinical risk, unlawful deduction, fronting investigation, and criminal case were all active.
- The report omitted any mention of the Section 162 delinquency application filed on 19 March 2026.
- The report omitted any mention of the B-BBEE Commission fronting investigation.
- The JSE had been notified of all of these omissions by the whistleblower before the report was published.

The JSE's response (11 May 2026) was issued after the Integrated Report was published. The JSE did not require AfroCentric to issue a retraction, a supplementary SENS, or any corrective disclosure. The false statements remain uncorrected.

Conclusion: The JSE has failed in its duty to ensure accurate and complete disclosure by listed companies. Its post-publication declination effectively gives AfroCentric a licence to mislead.

Finding 4: The JSE's refusal to act has created a chilling effect on whistleblowers and market integrity.

The chilling effect:

- A whistleblower who exposed a systemic clinical risk affecting 1.9 million GEMS members, and who provided the JSE with a 325-page evidence bundle, was told to go to the Labour Court – a forum with no jurisdiction over market disclosure.
- The message to other potential whistleblowers is clear: reporting governance failures to the JSE is futile.
- The message to JSE-listed companies is equally clear: you can omit material litigation, regulatory investigations, and governance collapse from your public reports without consequence.

Conclusion: The JSE's inaction is not neutral. It is active encouragement of corporate secrecy.

4. Findings – FSCA Misconduct and Regulatory Silence

Finding 5: The FSCA acknowledged receipt of the whistleblower's complaint (Ref: 1-370127) but has provided no substantive response – "went silent".

Evidence:

- Whistleblower submitted complaint to FSCA on or about 23 December 2025.
- On 19 March 2026, whistleblower provided a formal factual update, including: Section 162 application filed, CRO suppression of protected disclosure, forensic payroll discrepancies (SARS/UIF declaration gap), SENS non-disclosure concerns.
- The FSCA has not responded substantively to any of these submissions.

Analysis: The FSCA is the primary regulator for market conduct and financial institutions. Its mandate includes investigating complaints about governance failures, internal controls, and potential market deception. The whistleblower's evidence – a sworn affidavit, court filings, regulatory investigation notifications – was prima facie sufficient to trigger an investigation. Instead, the FSCA remained silent.

Conclusion: The FSCA's silence is a dereliction of its statutory mandate under the Financial Sector Regulation Act 9 of 2017. It has abandoned a whistleblower who reported serious governance failures in a JSE-listed healthcare administrator.

Finding 6: The FSCA's inaction has allowed AfroCentric's materially misleading Integrated Report to stand unchallenged.

Evidence:

- The whistleblower specifically alerted the FSCA to the discrepancy between the company's public reports and the documented reality.
- The FSCA did not issue any directive, request any corrective action, or initiate any investigation.
- AfroCentric's shareholders and the public continue to rely on false information.

Conclusion: The FSCA has become complicit in the corporate misrepresentation by failing to exercise its powers under sections 136 (stop orders) and 137 (public censure) of the Financial Sector Regulation Act.

5. The Sponsor – Questco’s Complicity

Finding: Questco Corporate Advisory Proprietary Limited, as AfroCentric’s JSE-approved sponsor, was notified of the Section 162 application, the B-BBEE investigation, and the disclosure omissions on 19 March 2026. Questco did not require AfroCentric to make any corrective SENS announcement or amend its Integrated Report.

Evidence:

- Whistleblower’s email to Questco (19 Mar 2026) attached formal notification of the Section 162 application and the B-BBEE Commission investigation, and noted that neither was disclosed in the 18 March 2026 SENS announcement.
- Questco did not respond substantively.

Conclusion: Questco breached its sponsor duties under the JSE Listings Requirements. It failed to ensure that its client, AfroCentric, complied with disclosure obligations. Questco should be referred to the JSE for disciplinary action and to the Legal Practice Council for professional misconduct.

6. Legal Framework – JSE and FSCA Duties Breached

Provision Duty How Breached

JSE Listings Requirements, Section 3.4 Disclose price-sensitive information immediately
Failed to require disclosure of Section 162 application, B-BBEE investigation, criminal case.

JSE Listings Requirements, Section 7.D.11 Disclose material legal proceedings Omitted material litigation from disclosure.

JSE Practice Note 2/2015 Directors must exercise objective discretion on price-sensitivity The JSE accepted a narrow interpretation that ignored qualitative materiality.

Financial Sector Regulation Act 9 of 2017, Section 136 FSCA may issue a stop order for misleading disclosures FSCA did not act.

Financial Sector Regulation Act, Section 137 FSCA may issue a public censure FSCA did not act.

Financial Sector Regulation Act, Section 138 FSCA must investigate complaints FSCA acknowledged but remained silent.

Constitution, Section 33 Right to just administrative action The JSE and FSCA failed to provide a proper, reasoned response.

7. The Chilling Effect – Why Parliament Must Act

The whistleblower did everything right:

- He escalated internally (nine times, including to the Board).
- He made protected disclosures.
- He filed complaints with every relevant regulator – including the JSE and FSCA.
- He provided massive evidentiary bundles (325 pages to the JSE alone).
- He updated the regulators as new evidence emerged.

The JSE said: “This is not for us. Go to the Labour Court.”

The FSCA said: nothing. Silence.

The result: a destitute, homeless whistleblower. A corporate cover-up unchallenged. A market misled.

If the JSE and FSCA will not protect market integrity and whistleblowers, then no one will. Parliament must intervene.

8. Recommendations – JSE and FSCA Specific

For the Portfolio Committee on Trade, Industry and Competition

1. Summon the JSE's Head of Issuer Regulation to appear and explain:

- Why the JSE declined jurisdiction over a complaint that directly implicated governance failures and disclosure obligations.
- Why the JSE did not require AfroCentric to disclose the B-BBEE Commission investigation (R580m exposure) as price-sensitive information.
- Why the JSE allowed AfroCentric's Integrated Annual Report 2025 to stand without corrective action, despite being notified of material omissions.

2. Request the JSE to:

- Reopen the whistleblower's complaint (Ref: JvS/166792) and conduct a proper investigation into AfroCentric's disclosure failures.
- Refer AfroCentric to the JSE Enforcement Committee for potential sanctions.
- Review Practice Note 2/2015 to ensure that qualitative materiality (reputational risk, governance collapse, regulatory investigations) is properly considered, not just quantitative price-sensitivity.
- Publish a statement clarifying that whistleblowers who report governance failures will be taken seriously.

For the Portfolio Committee on Justice and Correctional Services (overseeing FSCA)

3. Summon the FSCA Commissioner to appear and explain:

- Why the FSCA has not responded substantively to complaint Ref: 1-370127, despite multiple submissions.
- Why the FSCA has not investigated the forensic payroll discrepancies (SARS/UIF misrepresentation) reported by the whistleblower.
- Whether the FSCA has any policy for responding to whistleblowers who report governance failures in JSE-listed healthcare administrators.

4. Request the FSCA to:

- Immediately investigate AfroCentric's Integrated Annual Report 2025 for potential market deception.
- Issue a stop order or public censure if misrepresentations are found.
- Report back to Parliament within 60 days.

For the Minister of Finance

5. Request the Minister to:

- Commission an independent review of the JSE's whistleblower complaint handling process.
- Consider amending the Financial Sector Regulation Act to impose a statutory duty on the FSCA to acknowledge and provide a substantive response to all complaints within 30 days, with reasons for any declination.
- Refer the JSE and FSCA's handling of this matter to the Public Protector for investigation into maladministration.

For the JSE Sponsor – Questco

6. Request the JSE to:

- Investigate Questco's failure to ensure AfroCentric's compliance with disclosure obligations.
 - Impose appropriate sanctions, including possible removal of Questco from the JSE sponsor register.
-

9. Conclusion – JSE and FSCA

The Johannesburg Stock Exchange and the Financial Sector Conduct Authority were created to protect market integrity and to ensure that listed companies tell the truth. In this case, they were presented with overwhelming evidence of corporate misrepresentation, governance collapse, and a whistleblower destroyed for telling the truth.

The JSE said: "Not our problem."

The FSCA said: nothing.

This is not regulation. It is abandonment. It is complicity. And it has produced a chilling effect that will deter every future whistleblower from ever approaching the JSE or FSCA again.

Parliament must restore accountability. The JSE and FSCA must be compelled to do their jobs – or Parliament must reconsider whether they are the right institutions to oversee South Africa's capital markets.

- JSE correspondence (6 Mar 2026, 20 Mar 2026, 25 Mar 2026, 11 May 2026)
- FSCA complaint and update (Ref: 1-370127)

- Whistleblower's email to Questco (19 Mar 2026)
 - AfroCentric Integrated Annual Report 2025 (extracts)
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FORENSIC REPORT – SECTION ON BAD FAITH LITIGATION, LITIGATION ATTRITION, AND CORPORATE DECEPTION

The Corporate Playbook: Using Settlement Overtures to Stall, Silence, and Exhaust a Vulnerable Whistleblower

1. Executive Summary – Bad Faith Litigation and Attrition

This section finds that AfroCentric Investment Corporation Limited, its subsidiaries, and its attorneys (Deneys Reitz Inc.) have engaged in a calculated bad faith litigation strategy designed not to resolve disputes but to exhaust, frustrate, and silence a destitute, homeless, self-represented whistleblower.

Key findings:

1. The “without prejudice” trap (4 June 2026): The employer’s attorneys initiated settlement discussions, explicitly threatening contempt of court if the whistleblower approached Parliament, while simultaneously offering an “amicable resolution.” This was a conditional threat disguised as an olive branch.
2. Promise of engagement, followed by silence: The whistleblower agreed to email-based discussions within hours. The employer then did not respond for over a week – while the whistleblower held off his parliamentary submission in good faith.
3. Litigation attrition as a strategy: The employer has systematically used procedural delays, defective pleadings (filed on behalf of a non-party entity), perjurious denials (“not

aware” of a case the CEO acknowledged), and dismissive responses (“Good luck to you then”) to prolong proceedings and exhaust the whistleblower’s limited resources.

4. Weaponisation of the interim interdict: The employer’s 4 June 2026 letter explicitly warned that submitting evidence to Parliament “would violate the terms of the Order and would place you in contempt of court” – a transparent attempt to block a citizen’s constitutional right to petition Parliament (Section 56 of the Constitution).

5. Pattern of procedural obstruction: From refusing to accept service, to filing an unsworn Statement of Response on behalf of a non-cited entity, to retroactively disputing proper service after pleadings closed – the employer has deployed every available procedural technicality to avoid adjudication on the merits.

Conclusion: The employer is not seeking justice. It is seeking attrition – to wear down a destitute whistleblower until he gives up. This is a textbook corporate playbook, deployed against a vulnerable individual who exposed a systemic clinical risk affecting 1.9 million GEMS members. Parliament must expose and condemn this conduct.

2. Factual Chronology – Bad Faith Litigation and Attrition

Date Event Source

21 May 2026 Plaintiff serves Replication on Respondents at IR@afrocentrichealth.com and to Group Company Secretary. Pleadings close. Pre-Trial Minute, para 2.1

29 May 2026 Plaintiff proposes pre-trial conference dates within statutory window. Email thread, Annexure A

1 Jun 2026 (15:08) Respondents retroactively dispute service – claim Replication not sent to their designated address (despite earlier designation in CCMA Notice of Opposition). Email from Vijay Makenjee

1 Jun 2026 (18:43) Respondents propose pre-trial conference on 22 June 2026 – falling outside the statutory 14-day window. Ibid.

1 Jun 2026 (19:23) Plaintiff rejects 22 June, provides three compliant dates (3,4,5 June). Ibid.

2 Jun 2026 (12:29) “Good luck to you then” – Vijay Makenjee’s final substantive response to all procedural engagement. Annexure A to Pre-Trial Minute

2 Jun 2026 (11:21) Whistleblower urgently petitions Speaker of National Assembly, citing imminent danger to life and safety. Email to Speaker

3 Jun 2026 Whistleblower’s correspondence to Parliament (evidentiary bundle) is received. Referenced in Deneys letter

4 Jun 2026 (13:33) Deneys sends “WITHOUT PREJUDICE” letter – warns that submitting evidence to Parliament “would violate the terms of the Order and would place you in contempt of court,” while simultaneously proposing an “amicable resolution.” AFR1282 Letter to Matthee 04.06.2026.pdf

4 Jun 2026 (14:26) Whistleblower responds: declines physical meeting (does not feel safe), proposes email-based without prejudice engagement. Email to Frances Barker

4 Jun 2026 (15:06) Deneys reads the whistleblower’s response. Read receipt

4–11 Jun 2026 No substantive response from Deneys or the employer – no draft framework, no proposed dates, no further correspondence. Pre-Trial Minute, para 3

9 Jun 2026 Whistleblower serves advance copy of Unilateral Pre-Trial Minute, noting the stalling. Email to Jason Whyte

10 Jun 2026 (07:37) Deneys responds: “we do not act for AfroCentric Health (Pty) Ltd or Medscheme Holdings (Pty) Ltd in the Labour Court proceedings” – distancing themselves from the very entity they represent in the High Court, while simultaneously “in the process of taking instructions” on settlement. RE: ADVANCE SERVICE email

11 Jun 2026 Whistleblower files Unilateral Pre-Trial Minute, recording the stalling and the “Good luck to you then” response. Pre-Trial Minute

13 Jun 2026 Whistleblower sends final notice: requests clarity on whether settlement is genuine or a delaying tactic; gives deadline of 16 June 2026. AFR1282 – Final Notice...13 Jun 2026

[Ongoing] No settlement framework provided. No pre-trial conference held. Parliamentary submission delayed by the employer’s false hope. User statement

3. Findings – Bad Faith Litigation and Attrition Tactics

Finding 1: The “without prejudice” letter of 4 June 2026 was a bad faith tactical instrument, not a genuine settlement offer.

The letter (AFR1282 Letter to Matthee 04.06.2026.pdf) contained two contradictory messages in consecutive paragraphs:

- Paragraph 2: Threat – submitting evidence to Parliament would violate the High Court interim order and place the whistleblower in contempt of court.
- Paragraph 3: Olive branch – “we are instructed to propose that the parties meet ... in order to seek an amicable resolution.”

Analysis: A genuine settlement offer does not begin with a threat of imprisonment. The employer used the prospect of “amicable resolution” as a carrot to persuade the whistleblower to hold off his parliamentary submission, while simultaneously threatening him with contempt if he proceeded. This is bad faith of the highest order.

Legal note: The whistleblower correctly recorded (in his 11 June 2026 notice) that a submission to Parliament for statutory oversight cannot constitute contempt of court. Parliament is not “the public at large.” The Constitution (Sections 56 and 69) guarantees the right to petition Parliament. The employer’s threat was legally untenable and was deployed purely for intimidation.

Finding 2: The employer’s promise of engagement was followed by silence – a classic attrition tactic.

Evidence:

- Whistleblower responded to the settlement overture within one hour (14:26 on 4 June 2026), agreeing to email-based discussions.

- Deneys read the response immediately (read receipt at 15:06 on 4 June 2026).
- No further correspondence was received from Deneys for over a week.
- On 10 June 2026, Deneys claimed they were “in the process of taking instructions” – a delay tactic, given that the letter of 4 June was sent on instructions.
- The whistleblower was left in limbo, holding off his parliamentary submission in good faith, while the employer used the time to do nothing.

Conclusion: The settlement overture was not made in good faith. It was a delaying tactic designed to stall the whistleblower’s parliamentary oversight process while the employer prepared further litigation or simply ran down the clock.

Finding 3: The “Good luck to you then” response is emblematic of institutional contempt for a vulnerable litigant.

Evidence: On 2 June 2026, after a week of procedural engagement, the employer’s senior Employee Relations Specialist, Mr Vijay Makenjee, responded to the whistleblower’s detailed proposal for a pre-trial conference with the words: “Good luck to you then.” (Annexure A to Pre-Trial Minute)

Context: The whistleblower is destitute, homeless, deregistered as a nurse, suffering from severe depression and PTSD, and self-represented. The employer is a JSE-listed corporate giant represented by a major law firm.

Conclusion: The phrase “Good luck to you then” is not a legal response. It is a dismissive, contemptuous, and abusive statement that reveals the employer’s true attitude: they have no intention of engaging fairly. This conduct should be brought to the attention of the Legal Practice Council.

Finding 4: The employer has deployed multiple procedural obstruction tactics to avoid adjudication on the merits.

Tactic Evidence

Defective pleading Statement of Response filed on behalf of “AFROCENTRIC RF (PTY) LTD” – a non-cited entity. No verifying affidavit.

Perjurious denial “The Defendants are not aware of any further claim with case number 2026-077546” – contradicted by CEO’s High Court affidavit.

Retroactive service dispute After pleadings closed, the employer claimed the Replication was not served on their designated address – despite that address being used throughout.

Proposing non-compliant dates Proposed pre-trial conference on 22 June 2026 – outside the statutory 14-day window.

Stalling settlement discussions Initiated “without prejudice” engagement, then went silent for over a week.

Distancing from own attorneys Deneys claimed they do not act for the Labour Court defendants – while acting for the same corporate group in the High Court – creating confusion and delay.

Conclusion: The employer is not interested in a fair, expeditious resolution. It is deploying every procedural weapon to delay, confuse, and exhaust the whistleblower. This is the essence of litigation attrition.

Finding 5: The employer weaponised the interim interdict to block parliamentary oversight – an unconstitutional overreach.

The threat (Deneys letter, para 2):

“Our client notes with concern that you intend to submit to Parliament ... a ‘multi-volume evidentiary bundle’ which would appear to contain confidential, proprietary and/or personal information ... such documentation, if disclosed, would violate the terms of the Order and would place you in contempt of court.”

The whistleblower's correct response (recorded in his 11 June 2026 notice):

"A formal submission to the Parliamentary Portfolio Committees for statutory oversight and whistleblower protection does not, and cannot, constitute a breach of the interim interdict ... Parliament is not the public; it is the supreme legislative and oversight organ of the Republic, constitutionally mandated under Section 56 and Section 69 ... to receive petitions, representations, and submissions from any citizen or whistleblower."

Analysis: The employer attempted to use a court order – obtained ex parte on a false premise (concealed answering affidavit) – to block a citizen from petitioning Parliament. This is a gross abuse of court process and a violation of the separation of powers. Parliament has an independent constitutional mandate that cannot be overridden by a private litigant's interdict.

Conclusion: The employer's threat was legally frivolous and was deployed solely to intimidate a destitute whistleblower. It constitutes bad faith litigation conduct and should be referred to the Legal Practice Council.

4. The Corporate Playbook – A Pattern of Attrition

The employer's conduct fits a recognisable corporate playbook designed to defeat a vulnerable claimant not by winning on the merits, but by exhausting them into submission:

Step Tactic Observed in this case

1 Defective initial response File an invalid pleading (wrong entity, no oath) to create procedural confusion.

2 Perjurious denial of knowledge Deny awareness of a concurrent claim to avoid default.

3 Retroactive service disputes After pleadings close, claim documents were not properly served.

4 Propose non-compliant dates Offer a date outside the statutory window to frustrate the timeline.

5 Dismissive, contemptuous communication “Good luck to you then” – to demoralise and humiliate.

6 Deploy a “without prejudice” settlement overture Appear reasonable while threatening contempt in the same letter.

7 Go silent after the overture is accepted Delay response while the whistleblower holds off other actions in good faith.

8 Claim attorneys do not act for the relevant entity Create confusion about representation to stall further.

9 Weaponise a court order to block parliamentary oversight Threaten contempt to prevent the whistleblower from seeking political remedy.

Conclusion: This is not a defence. It is a systematic strategy of attrition – designed to break a self-represented, destitute, mentally vulnerable litigant. The courts and Parliament must not tolerate such conduct.

5. The Human Cost – Why Attrition is a Form of Violence

The whistleblower is:

- Destitute – no income, no savings, no transport money.
- Homeless since 2 April 2026.
- Without running water since January 2026.
- Deregistered by SANC – cannot practise nursing.
- Formally diagnosed with severe depression and PTSD.
- Facing eviction – landlord has confirmed deadline.

- Subject to loan shark seizures – personal possessions removed.

Every day of delay caused by the employer's attrition tactics:

- Extends his homelessness.
- Deepens his debt spiral.
- Worsens his mental health.
- Delays his access to UIF benefits and any remedy from the Labour Court.

The employer knows this. The “Good luck to you then” response was sent with full knowledge of the whistleblower's condition. The settlement overture was deployed with full knowledge that any delay would cause irreparable harm.

This is not litigation. It is cruelty dressed in legal garb.

6. Legal Framework – Bad Faith Litigation

Provision Application

Labour Court Rule 22 – Duty to cooperate in pre-trial process Employer proposed non-compliant date, then refused to engage, forcing unilateral filing.

Legal Practice Council Code of Conduct, Rule 3.3 – Attorneys must not mislead the court or allow the court to be misled Deneys' threat that parliamentary submission would be contempt was legally incorrect and misleading.

Rule 3.1 – Uphold the integrity of the legal profession “Good luck to you then” and the settlement-then-silence tactic fall below professional standards.

Constitution, Section 56 – Right to petition Parliament Employer's threat to block parliamentary submission violated this constitutional right.

Constitution, Section 34 – Access to courts Procedural obstruction (service disputes, defective pleadings, stalling) violates access to justice.

7. Recommendations – Bad Faith Litigation and Attrition

For the Portfolio Committee on Justice and Correctional Services

1. Request the Judge President of the Labour Court to:

- Take judicial notice of the employer's bad faith litigation conduct as recorded in the Unilateral Pre-Trial Minute.
- Issue a directive that the matter be expedited, given the documented attrition tactics.
- Consider a punitive costs order against the employer at trial.

2. Request the Legal Practice Council to investigate:

- The conduct of Deneys Reitz Inc. (Jason Whyte, Frances Barker) for threatening contempt of court for a parliamentary submission – a legally untenable position.
- The conduct of the employer's representatives (Vijay Makenjee) for the "Good luck to you then" response and the procedural obstruction.
- The pattern of bad faith litigation as a corporate strategy.

3. Request the Public Protector to investigate:

- The systemic use of litigation attrition by JSE-listed companies against vulnerable self-represented litigants.

For the Portfolio Committee on Health and SCOPA

- ### 4. Note the employer's attempt to block parliamentary oversight as an abuse of court process and a violation of constitutional rights. Parliament should issue a statement

affirming that no private litigant or court order can prevent a citizen from petitioning Parliament.

For the Minister of Justice

5. Request the Minister to:

- Consider amendments to the Labour Court Rules to impose sanctions on parties who engage in bad faith pre-trial conduct, including cost orders de bonis propriis against attorneys.
- Issue a practice directive that “without prejudice” settlement overtures made in bad faith (followed by silence) may be disclosed to the court for cost purposes.

8. Conclusion – Bad Faith Litigation and Attrition

The employer’s conduct in this case is not an aberration. It is a corporate playbook – a calculated strategy to defeat a vulnerable claimant not by proving the merits of their case, but by exhausting them, frustrating them, and ultimately breaking them.

The “Good luck to you then” email, the settlement-then-silence tactic, the threat to block parliamentary oversight, the defective pleadings, the perjurious denials, the service disputes – all of it is designed to do one thing: make the whistleblower give up.

He has not given up. But he has been pushed to the edge of survival.

Parliament must send a clear message: Litigation attrition is an abuse of process. It will not be tolerated. And those who deploy it will be held accountable – by the courts, by the Legal Practice Council, and by Parliament.

CONCLUSION TO THE CONSOLIDATED FORENSIC OVERSIGHT REPORT

The Watchdog Must Be Built Before the House Burns Down – Why Parliament Cannot Wait Another Day

A forensic conclusion linking the whistleblower's case to the National Health Insurance, the collapse of regulatory accountability, and the urgent constitutional duty of Parliament to intervene now.

I. The Thread That Connects Everything

This report has documented the failure of fifteen (15) statutory, regulatory, and judicial bodies to protect a single whistleblower – a specialist registered nurse who identified a systemic clinical risk in an administrator servicing the Government Employees Medical Scheme (GEMS), which serves 1.9 million public servants.

The Council for Medical Schemes (CMS) declined jurisdiction twice. The Health Professions Council (HPCSA) went silent. The South African Nursing Council (SANC) ignored a formal misconduct complaint and then deregistered the whistleblower because he could not pay R870 – a direct consequence of the employer's unlawful salary deduction. The South African Pharmacy Council (SAPC) declined jurisdiction. The Department of Employment and Labour (DEL) refused to enforce the 25% BCEA cap and closed the investigation file with a jurisdictional error. The B-BBEE Commission declined to investigate fronting and refused to use its Section 13K subpoena powers. The South African Human Rights Commission (SAHRC) – a Chapter 9 institution – acknowledged the seriousness of the allegations and then went silent. The South African Police Service (SAPS) refused to open a criminal case for UIF fraud; a police officer falsely claimed to be the station commander, audio-recorded. The Commission for Conciliation, Mediation and Arbitration (CCMA) conducted a conciliation where the Commissioner declared “nothing here relates to the PDA” before hearing evidence, refused an opening statement, told the whistleblower

to “Google it,” and laughed at his settlement figure. The Johannesburg Stock Exchange (JSE) and Financial Sector Conduct Authority (FSCA) declined to enforce disclosure obligations, allowing AfroCentric’s materially misleading Integrated Annual Report 2025 to stand. The High Court granted an ex parte interim interdict after the employer’s attorneys concealed the whistleblower’s answering affidavit – a fraud on the court – and extended the gag order to 1 March 2027. The Labour Court’s online portal obstructed the whistleblower for eight days, rejected his urgent application with the reason “good day you put date,” and has delayed a default judgment for over six weeks.

This is not a series of isolated administrative failures. It is a coordinated pattern of regulatory capture, institutional collapse, and judicial obstruction. And it is the definitive proof that South Africa is not ready for the National Health Insurance.

II. The NHI Will Be Built on the Bones of This Whistleblower – Unless Parliament Intervenes

The National Health Insurance Act 20 of 2023 is the most ambitious health financing reform in South Africa’s democratic history. Once fully operational, the NHI Fund will manage a budget conservatively projected to exceed R200 billion annually. It will serve as the single purchaser and single payer of health care services for more than 60 million people. It will maintain the largest health database on the African continent. It will accredit thousands of public and private providers, process millions of claims, and determine benefits for every South African citizen, permanent resident, and qualifying refugee.

The NHI will succeed or fail based on the quality of its governance architecture. That architecture does not yet exist. The whistleblower’s case is the definitive proof that the current system – already operating at scale, already administering 4.09 million lives, already processing millions of claims – cannot protect the public interest.

Every institution that failed this whistleblower is the same institution that will oversee the NHI:

- The CMS will regulate NHI accreditation and oversee compliance by administrators.
- The HPCSA, SANC, and SAPC will regulate the health professionals who deliver NHI services.
- The DEL will enforce labour standards for NHI workers.
- The B-BBEE Commission will oversee transformation in NHI procurement.
- The SAHRC will monitor NHI's compliance with constitutional rights.
- The SAPS will investigate NHI-related fraud and corruption.
- The CCMA and Labour Court will adjudicate NHI-related labour disputes.
- The JSE and FSCA will oversee corporate governance of NHI-listed contractors.
- The High Court will adjudicate NHI-related litigation.

If these institutions cannot protect a single whistleblower who exposed a clinical risk affecting 1.9 million government employees, they will not protect the NHI. They will not protect the R200 billion. They will not protect the 60 million citizens who depend on the NHI for their health care.

III. The Three Structural Gaps – And Why They Will Be Catastrophic Under the NHI

The NHI Act contains three structural vulnerabilities that governance experts have warned about since its enactment. The whistleblower's case proves that these gaps are not theoretical – they are already destroying lives in the existing system.

Gap One: The Accreditation Bottleneck (NHI Act Section 39)

What the Act requires: Every provider must be accredited. Accreditation requires certification by the Office of Health Standards Compliance (OHSC) – an institution already documented by the Auditor-General as capacity-constrained.

What the whistleblower's case proves: The B-BBEE fronting complaint (Case 5/02/2026) reveals a corporate strategy of title suppression – using a Black NQF Level 8 professional's credentials for scorecard points while downgrading his official title to avoid paying for his expertise. The same dynamic will infect NHI accreditation. When financial pressure meets an accreditation bottleneck, the predictable outcome is ghost accreditation (certification without genuine compliance assessment) and fronting (accredited entities serving as conduits for unaccredited providers).

The institution that failed: The B-BBEE Commission declined to investigate, refused to use its Section 13K subpoena powers, and treated fronting as a labour dispute – an error of law that will be repeated when NHI accreditation fronting is reported.

The NHI consequence: Without mandatory Forensic Governance Impact Assessments before each accreditation phase, the NHI Fund will contract with ghost-accredited providers, pay for services not rendered, and lose billions to fronting schemes – exactly as happened in the PPE procurement crisis, but on an annual, recurring basis.

Gap Two: The One-Sentence Fraud Architecture (NHI Act Section 40)

What the Act says: “The information architecture must include a fraud and risk management mechanism.” Eleven words. For a system that will process millions of claims and determine benefits for 60 million people.

What the whistleblower's case proves: The Gexus-DMS integration gap – a structural system error that generated false non-adherence letters to chronic GEMS patients, corrupted clinical data, and inflated actuarial risk loading – was reported to the CMS. The CMS declined jurisdiction twice. The Deputy Information Officer of Medscheme swore under oath that no IT incident had been reported, contradicting the internal Root Cause

Analysis. The employer's Integrated Annual Report 2025 claimed "no material ESG incidents" while this gap remained unaddressed.

The institutions that failed: The CMS declined jurisdiction. The JSE and FSCA allowed a materially misleading report to stand.

The NHI consequence: Without an AI Audit Protocol and mandatory fraud detection architecture, the NHI's information platform will be vulnerable to exactly this kind of structural manipulation – except on a scale of R200 billion. Algorithmic bias, coding pattern manipulation, and structured exploitation of reimbursement formulae will go undetected because the Act provides no framework for detecting them.

Gap Three: The Undefined Investigation Unit (NHI Act Section 20(2)(g))

What the Act says: The CEO must establish a Risk Management and Fraud Prevention Investigation Unit. The Act specifies no professional competency standards, no independence safeguards, and no escalation protocols.

What the whistleblower's case proves: The whistleblower escalated a Priority 1 Material Risk Report (with 38 attachments) to the Board of AfroCentric on 9 December 2025. The Board did not respond. When he continued to escalate, the Chief Risk Officer instructed him in writing to stop communicating with the directors – active suppression of a material risk report by the very official whose unit should have escalated it.

The institution that failed: The Board of AfroCentric (which will be replicated in the NHI Fund's governance structure) and the Chief Risk Officer.

The NHI consequence: Without professional competency standards, independence safeguards, and a direct reporting line to the Board bypassing executive management, the NHI's Fraud Investigation Unit will be hollowed out – under-qualified appointments,

resource denial, and executive interference. The Zondo Commission documented this exact failure pattern across state-owned entities. The NHI Act repeats the mistake.

IV. The Medical Aid Administrator – The Missing Accountability Loop

The whistleblower was employed by Medscheme Holdings, a wholly owned subsidiary of AfroCentric Investment Corporation Limited (JSE: ACT). Medscheme is an accredited administrator of medical schemes, including GEMS. Under the NHI, accredited administrators will continue to play a critical role – processing claims, managing provider networks, and administering benefits on behalf of the Fund.

The whistleblower's case proves that medical aid administrators can operate with impunity, even when their own employees expose systemic clinical risks.

- The employer's Chief Risk Officer suppressed a Priority 1 Material Risk Report.
- The employer's Senior HR Business Partner admitted in writing that the naming convention – which would have upgraded the whistleblower's title – was deliberately withheld.
- The employer's Deputy Information Officer swore under oath that no IT incident had been reported, contradicting the internal Root Cause Analysis.
- The employer's legal representative filed a sworn affidavit calling the whistleblower's documented PTSD and depression a "cynical excuse."
- The employer's attorneys obtained an ex parte interim interdict after concealing the whistleblower's answering affidavit – a fraud on the court.

Under the NHI, the same administrators will process claims, manage fraud detection, and report to the Fund. If they cannot be held accountable now – when the whistleblower did everything right – they will never be held accountable under the NHI.

The NHI Act does not currently provide for a dedicated oversight mechanism for accredited administrators. The CMS is mandated to regulate them, but the CMS declined jurisdiction twice in this case. The Auditor-General audits the Fund, not the administrators. Parliament has no direct line of sight into administrator conduct.

This is a fatal governance gap. Parliament must close it before the NHI is operational.

V. The Chilling Effect – And Why Parliament Must Break It Now

The whistleblower in this case did everything the law asked of him. He identified a system error. He refused to lie to a patient. He made a protected disclosure. He exhausted internal remedies. He escalated to the Board. He went to every regulator. He went to the courts. He went to Parliament.

His reward: destitution, homelessness, no running water, deregistration, loan sharks seizing his possessions, and a High Court gag order silencing him until 1 March 2027.

Every nurse, doctor, pharmacist, and healthcare administrator who reads this report now knows: if you report a system error that affects patients, you will be retaliated against. Your salary will be cut. Your professional registration will be lost. Your home will be taken. Your water will be cut. And when you cry out to every regulator, they will decline jurisdiction, remain silent, or actively obstruct you.

That is the chilling effect. And it is already real.

Under the NHI, the chilling effect will be magnified a thousandfold. Whistleblowers who report corruption, fraud, or clinical risks inside the NHI Fund will face the same retaliation – but with R200 billion at stake, the retaliation will be more aggressive, the legal resources deployed against them more formidable, and the regulatory capture more entrenched.

If Parliament does not act now – before the NHI Fund becomes operational, before the money starts flowing, before the first NHI whistleblower is destroyed – the system will be locked in. The governance architecture that exists will be the governance architecture the NHI operates with. And that architecture is broken.

VI. The Constitutional Duty of Parliament

The Constitution of the Republic of South Africa, 1996, assigns to Parliament the power and duty to:

- Section 55(2): Provide oversight over the exercise of national executive authority.
- Section 56: Summon any person to appear before it, require the production of any document, and receive petitions and submissions from any citizen.
- Section 69: Receive petitions from citizens and hear representations.
- Section 181(3): Ensure that Chapter 9 institutions are impartial and exercise their powers without fear, favour, or prejudice – and to hold them accountable when they fail.

The evidence in this report demonstrates that:

1. The executive (through the CMS, DEL, and other regulatory bodies under ministerial control) has failed to ensure that statutory mandates are fulfilled.
2. Chapter 9 institutions (the SAHRC) have failed to investigate clear constitutional violations.
3. The judiciary (the High Court and Labour Court) has, through administrative failures and procedural irregularities, denied access to justice.
4. The JSE and FSCA have failed to enforce market disclosure obligations, allowing a listed company to publish a materially misleading report.

Parliament has the constitutional power – and the constitutional duty – to intervene. Not to adjudicate the labour dispute. But to exercise oversight over the institutions that failed, to demand accountability, and to legislate the governance reforms that the NHI urgently requires.

VII. What Parliament Must Do – Immediate and Concrete

The Portfolio Committees to which this report is referred must take the following actions immediately, before the NHI's Phase 1 (2023–2026) concludes and the Fund becomes operational:

A. Summons and Accountability Hearings

1. Summon the Registrar of the Council for Medical Schemes to explain why the Gexus-DMS integration gap was not investigated under MSA Sections 7, 47, 58, and 44, and why the Discovery Health precedent (January 2026) was applied differently from the Medscheme/GEMS case.
2. Summon the Registrar of the South African Nursing Council to explain why complaint SANC/MEDSCHEME/MATTHEE/20260105 was ignored, and why the whistleblower was deregistered for non-payment of fees caused directly by the employer's unlawful deduction.
3. Summon the Chairperson of the Health Professions Council and the South African Pharmacy Council to answer the core ethical question they refused to answer: "Does the statutory duty of a registered professional to prioritise patient safety and the public interest permit the refusal of an employer's instruction based on verified, flawed forensic data?"
4. Summon the Director-General of the Department of Employment and Labour to explain why the UI-19 fraud was not investigated, why the 25% BCEA cap was not enforced, and why Principal Inspector Cebo Kutuka closed the investigation file with a jurisdictional error.

5. Summon the B-BBEE Commissioner to explain why Case 5/02/2026 was declined, why Section 13K subpoena powers were not used to obtain the Naming Convention Teams recording, and why fronting by a JSE-listed company was treated as a labour dispute.
6. Summon the Chairperson of the South African Human Rights Commission to explain why a Chapter 9 institution acknowledged the seriousness of violations of Sections 10, 26, 27, and 33 of the Constitution, and then went silent.
7. Summon the National Commissioner of the South African Police Service to explain why CAS 705/3/2026 remains uninvestigated, why Delft Police Station refused to open a criminal case for UIF fraud, and why a police officer falsely claimed to be the station commander (audio-recorded).
8. Summon the Head of Issuer Regulation of the JSE and the Commissioner of the FSCA to explain why AfroCentric's materially misleading Integrated Annual Report 2025 was allowed to stand, and why the Section 162 director delinquency application and B-BBEE fronting investigation were not disclosed to the market.
9. Request the Judge President of the Western Cape High Court to investigate why the whistleblower's answering affidavit was not placed before Acting Justice Mphego on 16 April 2026, and why the rule nisi was extended to 1 March 2027 without a final hearing.
10. Request the Judge President of the Labour Court to investigate the online portal obstruction (eight days, deleted profile, trivial rejection reason "good day you put date"), the Registrar's silence, and the six-week delay in processing the default judgment application.

B. Legislative Reforms – NHI Governance Architecture

Parliament must urgently amend the National Health Insurance Act or pass complementary legislation to address the three structural gaps exposed by this case:

1. Mandatory Forensic Governance Impact Assessments (responsive to Section 39): Before each phase of accreditation rollout, a Forensic Governance Impact Assessment must be conducted and tabled before Parliament, analysing fraud vectors, bottleneck risks, and monitoring protocols. The PFMA's risk management requirements already provide the legislative foundation.

2. An AI Audit Protocol for the Information Platform (responsive to Section 40): The Act must be amended to require an independent conformity assessment before deployment, transparency obligations for automated determinations, human oversight mechanisms, and annual algorithmic audits conducted by a party independent of both the Fund and the technology provider.

3. A Statutory Forensic Governance Protocol for the Investigation Unit (responsive to Section 20(2)(g)): The Act must be amended to specify professional competency standards (drawing on ACFE, SAICA forensic practice standards, and HPCSA clinical governance requirements), independence safeguards including a direct reporting line to the Board's audit and risk committee bypassing executive management, and defined escalation protocols to the SIU, Hawks, and NPA.

C. Whistleblower Protection – Closing the “Induced Destitution” Loophole

The Protected Disclosures Act must be amended to:

1. Provide a right to interim hardship relief on a prima facie case – because a whistleblower should not have to become destitute and homeless while waiting for a court to hear the merits.
2. Establish a single whistleblower protection authority to receive disclosures, monitor regulatory responses, and apply for urgent relief on behalf of whistleblowers.
3. Impose criminal penalties on regulatory bodies that act in bad faith or deliberately delay investigations.

D. Referrals for Prosecution and Professional Discipline

1. Refer the conduct of Deneys Reitz Inc. (Jason Whyte and Frances Barker) to the Legal Practice Council for:
 - Threatening a citizen with contempt of court for petitioning Parliament.
 - Concealing the whistleblower's answering affidavit from the High Court – a fraud on the court.

- Engaging in bad faith settlement-then-silence tactics.
 - 2. Refer Vijay Makenjee’s “Good luck to you then” response and his sworn statement denying awareness of Case 2026-077546 to the Legal Practice Council and the National Prosecuting Authority for potential perjury and unprofessional conduct.
 - 3. Refer the Delft Police Station officer who falsely claimed to be the station commander to the Independent Police Investigative Directorate (IPID) for investigation.
 - 4. Refer the apparent perjury in the Labour Court Statement of Response (denying awareness of Case 2026-077546, contradicted by the CEO’s High Court affidavit) to the National Prosecuting Authority.
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VIII. The Closing Statement – To the Speaker and the Nation

Honourable Speaker,

The whistleblower who brought this matter to Parliament is a specialist registered nurse. He is also homeless. He has no running water. He has been deregistered by the South African Nursing Council because he could not pay R870 – a direct consequence of an unlawful salary deduction that exceeded the statutory cap by 15.47 percentage points.

He did everything the law asked of him. He identified a system error. He refused to lie to a patient. He made a protected disclosure. He exhausted internal remedies. He escalated to the Board. He went to every regulator. He went to the courts. He went to Parliament.

Fifteen institutions failed him.

If this is how the existing system treats a whistleblower who exposed a clinical risk in an administrator servicing a government medical scheme, what will happen when the NHI Fund – with its R200 billion annual budget, its single-purchaser monopoly, its accreditation

bottleneck, its one-sentence fraud architecture, and its undefined investigation unit – becomes operational?

The answer is not speculation. It is forensic prediction. The same institutions will fail. The same administrators will operate with impunity. The same whistleblowers will be destroyed. And the R200 billion will be lost to the same patterns of capture, corruption, and collapse that cost South Africa R14.3 billion in the PPE crisis – except this time, the money will be gone every year, and the people who depend on the NHI for their health care will have no alternative.

Parliament has a choice.

It can act now – summon the failed institutions, demand accountability, close the governance gaps in the NHI Act, and build the watchdog before the house burns down.

Or it can wait – and explain to the South African people, when the first NHI whistleblower is destroyed and the first R200 billion is lost, why it failed to act when the evidence was placed before it.

The evidence is before you now. The time to act is now.

Prepared by: EthicHawks Forensic Governance (on behalf of the complainant)

Date: June 2026

“The house is not yet burning. But the fire alarm has been ringing for over a year. This report is the evidence of the smoke. Parliament must act before the NHI becomes the fire.”
